

with this disorder often have impaired relationships with same-sex friends because their sexually provocative interpersonal style may seem a threat to their friends' relationships. These individuals may also alienate friends with demands for constant attention. They often become depressed and

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upset when they are not the center of attention. They may crave novelty, stimulation, and excitement and have a tendency to become bored with their usual routine. These individuals are often intolerant of, or frustrated by, situations that involve delayed gratification, and their actions are often directed at obtaining immediate satisfaction. Although they often initiate a job or project with great enthusiasm, their interest may lag quickly. Longer-term relationships may be neglected to make way for the excitement of new relationships.

Prevalence

The estimated prevalence of histrionic personality disorder based on a probability subsample from Part II of the National Comorbidity Survey Replication was 0.0%. The prevalence of histrionic personality disorder in the National Epidemiologic Survey on Alcohol and Related Conditions data was 1.8%. A review of five epidemiological studies (four in the United States) found a median prevalence of 0.9%.

Culture-Related Diagnostic Issues

Norms for interpersonal behavior, personal appearance, and emotional expressiveness vary widely across cultures, genders, and age groups. Before considering the various traits (e.g., emotionality, seductiveness, dramatic interpersonal style, novelty seeking, sociability, charm, impressionability, a tendency to somatization) to be evidence of histrionic personality disorder, it is important to evaluate whether they cause clinically significant impairment or distress. The presence of histrionic personality disorder should be distinguished from reactive and contextual expression of these traits, arising in response to socialization pressures in competitive peer groups, including the "need to be liked," that do not represent pervasive and enduring traits consistent with a personality disorder.

Sex- and Gender-Related Diagnostic Issues

In clinical settings, this disorder has been diagnosed more frequently in females; however, the gender ratio is not significantly different from the gender ratio of females within the respective clinical setting. In contrast, some studies using structured assessments report similar prevalence rates among males and females.

Association With Suicidal Thoughts or Behavior

The actual risk of suicide is not known, but clinical experience suggests that individuals with this disorder may be at increased risk for suicidal gestures and threats.

Differential Diagnosis

Other personality disorders and personality traits. Other personality disorders may be confused with histrionic personality disorder because they have certain features in common. It is therefore important to distinguish among these disorders based on differences in their characteristic features. However, if an individual has personality features that meet criteria for one or more personality disorders in addition to histrionic personality disorder, all can be diagnosed. Although borderline personality disorder can also be characterized by attention seeking, manipulative behavior, and rapidly shifting emotions, it is distinguished by self-destructiveness, angry disruptions in close relationships, and chronic feelings of deep emptiness and identity disturbance. Individuals with antisocial personality disorder and histrionic personality disorder share a tendency to be impulsive, superficial, excitement seeking, reckless, seductive, and manipulative, but persons with histrionic personality disorder tend to be more exaggerated in their emotions and do not characteristically engage in antisocial behaviors. Individuals with histrionic personality disorder are manipulative to gain nurturance, whereas those with antisocial personality disorder are manipulative.

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lative to gain profit, power, or some other material gratification. Although individuals with narcissistic personality disorder also crave attention from others, they usually want praise for their “superiority,” whereas individuals with histrionic personality disorder are willing to be viewed as fragile or dependent if this is instrumental in getting attention. Individuals with narcissistic personality disorder may exaggerate the intimacy of their relationships with other people, but they are more apt to emphasize the “VIP” status or wealth of their friends. In dependent personality disorder, the individual is excessively dependent on others for praise and guidance, but is without the flamboyant, exaggerated, emotional features of individuals with histrionic personality disorder.

Many individuals may display histrionic personality traits. Only when these traits are inflexible, maladaptive, and persisting and cause significant functional impairment or subjective distress do they constitute histrionic personality disorder.

Personality change due to another medical condition. Histrionic personality disorder must be distinguished from personality change due to another medical condition, in which the traits that emerge are a direct physiological consequence of another medical condition.

Substance use disorders. The disorder must also be distinguished from symptoms that may develop in association with persistent substance use.

Comorbidity

Histrionic personality disorder has been associated with higher rates of borderline, narcissistic, paranoid, dependent, and antisocial personality disorders; alcohol and other substance use and misuse; as well as aggression and violence. Histrionic personality disorder is also thought to be related to somatic symptom disorder, functional neurological symptom disorder (conversion disorder), and major depressive disorder.

Narcissistic Personality Disorder

Diagnostic Criteria

F60.81

A pervasive pattern of grandiosity (in fantasy or behavior), need for admiration, and lack of empathy, beginning by early adulthood and present in a variety of contexts, as indicated by five (or more) of the following:

1. Has a grandiose sense of self-importance (e.g., exaggerates achievements and talents, expects to be recognized as superior without commensurate achievements).
2. Is preoccupied with fantasies of unlimited success, power, brilliance, beauty, or ideal love.
3. Believes that he or she is “special” and unique and can only be understood by, or should associate with, other special or high-status people (or institutions).
4. Requires excessive admiration.
5. Has a sense of entitlement (i.e., unreasonable expectations of especially favorable treatment or automatic compliance with his or her expectations).
6. Is interpersonally exploitative (i.e., takes advantage of others to achieve his or her own ends).
7. Lacks empathy: is unwilling to recognize or identify with the feelings and needs of others.
8. Is often envious of others or believes that others are envious of him or her.
9. Shows arrogant, haughty behaviors or attitudes.

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Diagnostic Features

The essential feature of narcissistic personality disorder is a pervasive pattern of grandiosity, need for admiration, and lack of empathy that begins by early adulthood and is present in a variety of contexts.

Individuals with this disorder have a grandiose sense of self-importance, which may be manifest as an exaggerated or unrealistic sense of superiority, value, or capability (Criterion 1). They tend to overestimate their abilities and amplify their accomplishments, often appearing boastful and pretentious. They may blithely assume that others attribute the same value to their efforts and may be surprised when the praise they expect and feel they deserve is not forthcoming. Often implicit in the inflated judgments of their own accomplishments is an underestimation or devaluation of the contributions of others. Individuals with narcissistic personality disorder are often preoccupied with fantasies of unlimited success, power, brilliance, beauty, or ideal love (Criterion 2). They may ruminate about “long overdue” admiration and privilege and compare themselves favorably with famous or privileged people.

Individuals with narcissistic personality disorder believe that they are special or unique and expect others to recognize them as such (Criterion 3). They can be surprised or even devastated when the recognition of acclaim they expect and feel they deserve from others is not forthcoming. They may feel that they can only be understood by, and should only associate with, people of high status and may attribute “unique,” “perfect,” or “gifted” qualities to those with

whom they associate. Individuals with this disorder believe that their needs are special and beyond the ken of ordinary people. Their own self-esteem is enhanced (i.e., “mirrored”) by the idealized value that they assign to those with whom they associate. They are likely to insist on having only the “top” person (doctor, lawyer, hairdresser, instructor) or being affiliated with the “best” institutions but may devalue the credentials of those who disappoint them.

Individuals with this disorder generally require excessive admiration (Criterion 4). Their self-esteem is almost invariably very fragile, and their struggle with severe internal self-doubt, self-criticism, and emptiness results in their need to actively seek others’ admiration. They may be preoccupied with how well they are doing and how favorably they are regarded by others. They may expect their arrival to be greeted with great fanfare and are astonished if others do not covet their possessions. They may constantly fish for compliments, often with great charm.

A sense of entitlement, which is rooted in their distorted sense of self-worth, is evident in these individuals’ unreasonable expectation of especially favorable treatment (Criterion 5). They expect to be catered to and are puzzled or furious when this does not happen. For example, they may assume that they do not have to wait in line and that their priorities are so important that others should defer to them, and then get irritated when others fail to assist “in their very important work.” They expect to be given whatever they want or feel they need, no matter what it might mean to others. For example, these individuals may expect great dedication from others and may overwork them without regard for the impact on their lives. This sense of entitlement, combined with a lack of understanding and sensitivity to the wants and needs of others, may result in the conscious or unwitting exploitation of others (Criterion 6). They tend to form friendships or romantic relationships only if the other person seems likely to advance their purposes or otherwise enhance their self-esteem. They often usurp special privileges and extra resources that they believe they deserve. Some individuals with narcissistic personality disorder intentionally and purposefully take advantage of others emotionally, socially, intellectually, or financially for their own purposes and gains.

Individuals with narcissistic personality disorder generally have a lack of empathy and are unwilling to recognize or identify with the desires, subjective experiences, and feelings of others (Criterion 7). They tend to have some degree of cognitive empathy

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(understanding another person’s perspective on an intellectual level) but lack emotional empathy (directly feeling the emotions that another person is feeling). These individuals may be oblivious to the hurt their remarks may inflict (e.g., exuberantly telling a former lover that “I am now in the relationship of a lifetime!”; boasting of health in front of someone who is sick). When recognized, the needs, desires, or feelings of others are likely to be viewed disparagingly as signs of weakness or vulnerability. Those who relate to individuals with narcissistic personality disorder typically find an emotional coldness and lack of reciprocal interest.

These individuals are often envious of others or believe that others are envious of them (Criterion 8). They may begrudge others their successes or possessions, feeling that they better deserve those achievements, admiration, or privileges. They may harshly devalue the contributions of others, particularly when those individuals have received acknowledgment or praise for their accomplishments. Arrogant, haughty behaviors characterize these individuals; they often display snobbish, disdainful, or patronizing attitudes (Criterion 9).

Associated Features

Vulnerability in self-esteem makes individuals with narcissistic personality disorder very sensitive to criticism or defeat. Although they may not show it outwardly, such experiences may leave them feeling ashamed, humiliated, degraded, hollow, and empty. They may react with disdain, rage, or defiant counterattack. However, such experiences can also lead to social withdrawal or an appearance of humility that may mask and protect the grandiosity. Interpersonal relations are typically impaired because of problems related to self-preoccupation, entitlement, need for admiration, and relative disregard for the sensitivities of others.

Individuals with narcissistic personality disorder can be competent and high functioning with professional and social success, while others can have various levels of functional impairment. Professional capability combined with self-control, stoicism, and interpersonal distancing with minimal self-disclosure can support sustained life engagement and even enable marriage and social affiliations. Sometimes ambition and temporary confidence lead to high achievements, but performance can be disrupted because of fluctuating self-confidence and intolerance of criticism or defeat. Some individuals with narcissistic personality disorder have very low vocational functioning, reflecting an unwillingness to take a risk in competitive or other situations in which failure or defeat can be possible.

Low self-esteem with inferiority, vulnerability, and sustained feelings of shame, envy, and humiliation accompanied by self-criticism and insecurity can make individuals with narcissistic personality disorder susceptible to social withdrawal, emptiness, and depressed mood. High perfectionist standards are often associated with significant fear of exposure to imperfection, failure, and overwhelming emotions.

Prevalence

The estimated prevalence of narcissistic personality disorder based on a probability subsample from Part II of the National Comorbidity Survey Replication was 0.0%. The prevalence of narcissistic personality disorder in the National Epidemiologic Survey on Alcohol and Related Conditions data was 6.2%. A review of five epidemiological studies (four in the United States) found a median prevalence of 1.6%.

Development and Course

Narcissistic traits may be particularly common in adolescents but do not necessarily indicate that the individual will develop narcissistic personality disorder in adulthood. Predominant narcissistic traits or manifestations of the full disorder may first come to clinical attention or be exacerbated in the context of unexpected or extremely challenging life

experiences or crises, such as bankruptcies, demotions or loss of work, or divorces. In addition, individuals with narcissistic personality disorder may have specific difficulties adjusting to the onset of physical and occupational limitations that are inherent in the aging process. However, life experiences, such as new durable relationships, real successful achievements, and tolerable disappointments and setbacks, can all be corrective and contribute to changes and improvements in individuals with this disorder.

Culture-Related Diagnostic Issues

Narcissistic traits may be elevated in sociocultural contexts that emphasize individualism and personal autonomy over collectivistic goals. Compared with collectivistic contexts, in individualistic contexts, narcissistic traits may warrant less clinical attention or less frequently lead to social impairment.

Sex- and Gender-Related Diagnostic Issues

Among adults age 18 and older diagnosed with narcissistic personality disorder, 50%–75% are men. Gender differences in adults with this disorder include stronger reactivity in response to stress and compromised empathic processing in men as opposed to self-focus and withdrawal in women. Culturally based gender patterns and expectations may also contribute to gender differences in narcissistic personality disorder traits and patterns.

Association With Suicidal Thoughts or Behavior

In the context of severe stress, and given the perfectionism often associated with narcissistic personality disorder, exposure to imperfection, failure, and overwhelming emotions can evoke suicidal ideation. Suicide attempts in individuals with narcissistic personality disorder tend to be less impulsive and are characterized by higher lethality compared with suicide attempts by individuals with other personality disorders.

Differential Diagnosis

Other personality disorders and personality traits. Other personality disorders may be confused with narcissistic personality disorder because they have certain features in common. It is, therefore, important to distinguish among these disorders based on differences in their characteristic features. However, if an individual has personality features that meet criteria for one or more personality disorders in addition to narcissistic personality disorder, all can be diagnosed. The most useful feature in discriminating narcissistic personality disorder from histrionic, antisocial, and borderline personality disorders, in which the interactive styles are coquettish, callous, and needy, respectively, is the grandiosity characteristic of narcissistic personality disorder. The relative stability of self-image and self-control as well as the relative lack of self-destructiveness, impulsivity, separation insecurity, and emotional hyperreactivity also help distinguish narcissistic personality disorder from borderline personality disorder.

Excessive pride in achievements, a relative lack of emotional display, and ignorance of or disdain for others' sensitivities help distinguish narcissistic personality disorder from histrionic personality disorder. Although individuals with borderline, histrionic, and narcissistic personality disorders may require much attention, those with narcissistic personality disorder specifically need that attention to be admiring. Individuals with antisocial and narcissistic personality disorders share a tendency to be tough-minded, glib, superficial, exploitative, and unempathic. However, narcissistic personality disorder does not necessarily include characteristics of impulsive aggressivity and deceitfulness. In addition, individuals with antisocial personality disorder may be more indifferent and less sensitive to others' reactions or criticism, and individuals with narcissistic personality disorder usually lack the history of conduct disorder in childhood or criminal behavior in adulthood.

In both narcissistic personality disorder and obsessive-compulsive personality disorder, the individual may profess a commitment to perfectionism and believe that others cannot do things as well. However, while those with obsessive-compulsive personality disorder tend to be more immersed in perfectionism related to order and rigidity, individuals with narcissistic personality disorder tend to set high perfectionistic standards, especially for appearance and performance, and to be critically concerned if they are not measuring up.

Suspiciousness and social withdrawal usually distinguish those with schizotypal, avoidant, or paranoid personality disorder from those with narcissistic personality disorder. When these qualities are present in individuals with narcissistic personality disorder, they derive primarily from shame and fear of failure, or fear of having imperfections or flaws revealed.

Many highly successful individuals display personality traits that might be considered narcissistic. Only when these traits are inflexible, maladaptive, and persisting and cause significant functional impairment or subjective distress do they constitute narcissistic personality disorder.

Mania or hypomania. Grandiosity may emerge as part of manic or hypomanic episodes, but the association with mood change or functional impairments helps distinguish these episodes from narcissistic personality disorder.

Substance use disorders. Narcissistic personality disorder must also be distinguished from symptoms that may develop in association with persistent substance use.

Persistent depressive disorder. Experiences that threaten self-esteem can evoke a deep sense of inferiority and sustained feelings of shame, envy, self-criticism, and insecurity in individuals with narcissistic personality disorder that can result in persistent negative feelings resembling those seen in persistent depressive disorder. If criteria are also met for persistent depressive disorder, both conditions can be diagnosed.

Comorbidity

Narcissistic personality disorder is associated with depressive disorders (persistent depressive disorder and major depressive disorder), anorexia nervosa, and substance use disorders (especially related to cocaine). Histrionic, borderline, antisocial, and paranoid personality disorders may also be associated with narcissistic personality disorder.

Cluster C Personality Disorders

Avoidant Personality Disorder

Diagnostic Criteria

F60.6

A pervasive pattern of social inhibition, feelings of inadequacy, and hypersensitivity

to negative evaluation, beginning by early adulthood and present in a variety of contexts, as indicated by four (or more) of the following:

1. Avoids occupational activities that involve significant interpersonal contact because of fears of criticism, disapproval, or rejection.
2. Is unwilling to get involved with people unless certain of being liked.

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3. Shows restraint within intimate relationships because of the fear of being shamed or ridiculed.
4. Is preoccupied with being criticized or rejected in social situations.
5. Is inhibited in new interpersonal situations because of feelings of inadequacy.
6. Views self as socially inept, personally unappealing, or inferior to others.
7. Is unusually reluctant to take personal risks or to engage in any new activities because they may prove embarrassing.

Diagnostic Features

The essential feature of avoidant personality disorder is a pervasive pattern of social inhibition, feelings of inadequacy, and hypersensitivity to negative evaluation that begins by early adulthood and is present in a variety of contexts.

Individuals with avoidant personality disorder avoid work activities that involve significant interpersonal contact because of fears of criticism, disapproval, or rejection (Criterion 1). Offers of job promotions may be declined because failure to manage the new responsibilities might result in criticism from coworkers. These individuals avoid making new friends unless they are certain they will be liked and accepted without criticism (Criterion 2). Until they pass stringent tests proving the contrary, other people are assumed to be critical and disapproving. Individuals with this disorder are highly avoidant of group activities. Interpersonal intimacy is often difficult for these individuals, although they are able to establish intimate relationships when there is assurance of uncritical acceptance. They may act with restraint, be reluctant to talk about themselves, and withhold intimate feelings for fear of being exposed, ridiculed, or shamed (Criterion 3).

Because individuals with this disorder are preoccupied with being criticized or rejected in social situations, they may have a markedly low threshold for detecting such reactions (Criterion 4). If someone is even slightly disapproving or critical, they may feel extremely hurt. They tend to be shy, quiet, inhibited, and “invisible” because of the fear that any attention would be critical or rejecting. They expect that no matter what they say, others will see it as “wrong,” and so they may say nothing at all. They react strongly to subtle cues that are suggestive of mockery or derision, and may misinterpret a neutral gesture or statement as critical or rejecting. Despite their longing to be active participants in social life, they fear placing their psychological welfare in the hands of others. Individuals with avoidant personality disorder are inhibited in new interpersonal situations because they feel inadequate and have low self-esteem (Criterion 5). These individuals believe themselves to be socially inept, personally unappealing, or inferior to others (Criterion 6). Doubts concerning social competence and personal appeal may be most intense for some individuals in settings involving interactions with strangers. But many others report more

difficulties with repeated interaction, when sharing of personal information would normally occur, thus, in the individual's perception, increasing the chances that their inferiority would be revealed and that they would be rejected. When commencing a new ongoing social or occupational commitment requiring repeated interpersonal interaction, individuals may over weeks or months develop a growing conviction that others or colleagues view them as inferior or lacking worth, resulting in intolerable distress or anxiety that prompts resignation. Thus, a history of repeated job changes may be present. Individuals with this disorder are unusually reluctant to take personal risks or to engage in any new activities because these may prove embarrassing (Criterion 7). They are prone to exaggerate the potential dangers of ordinary situations, and a restricted lifestyle may result from their need for certainty and security.

Associated Features

Individuals with avoidant personality disorder often vigilantly appraise the movements and expressions of those with whom they come into contact. They are likely to

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misinterpret social responses as critical, which in turn confirms their self-doubts. They are described by others as being “shy,” “timid,” “lonely,” and “isolated.” The major problems associated with this disorder occur in social and occupational functioning. The low self-esteem and hypersensitivity to rejection are associated with restricted interpersonal contacts. These individuals may become relatively isolated and usually do not have a large social support network that can help them weather crises. They desire affection and acceptance and may fantasize about idealized relationships with others. Avoidant behaviors can also adversely affect occupational functioning because these individuals try to avoid the types of social situations that may be important for meeting the basic demands of the job or for advancement.

Individuals with avoidant personality disorder have been reported as having insecure attachment styles characterized by a desire for emotional attachment (which may include a preoccupation with previous and current relationships), but their fears that others may not value them or may hurt them may lead them to respond with passivity, anger, or fear. These attachment patterns have been referred to variously as “preoccupied” or “fearful” depending on the model employed by researchers.

Prevalence

The estimated prevalence of avoidant personality disorder based on a probability subsample from Part II of the National Comorbidity Survey Replication was 5.2%. The prevalence of avoidant personality disorder in the National Epidemiologic Survey on Alcohol and Related Conditions was 2.4%. A review of six epidemiological studies (four in the United States) found a median prevalence of 2.1%.

Development and Course

The avoidant behavior often starts in infancy or childhood with shyness, isolation, and fear of strangers and new situations. Although shyness in childhood is a common precursor of avoidant personality disorder, in most individuals it tends to gradually dissipate as they get older. In

contrast, individuals who go on to develop avoidant personality disorder may become increasingly shy and avoidant during adolescence and early adulthood, when social relationships with new people become especially important. There is some evidence that in adults, avoidant personality disorder tends to become less evident or to remit with age; the prevalence in adults older than 65 years has been estimated at 0.8%. This diagnosis should be used with great caution in children and adolescents, for whom shy and avoidant behavior may be developmentally appropriate.

Culture-Related Diagnostic Issues

There may be variation in the degree to which different cultural and ethnic groups regard diffidence and avoidance as appropriate. Moreover, avoidant behavior may be the result of problems in acculturation following migration. In some sociocultural contexts, marked avoidance might occur following social embarrassment (“loss of face”) or failure to meet major life goals rather than temperamental shyness. In these settings, the goal of avoidance includes deliberate minimization of social interactions in order to preserve social harmony or prevent public offense.

Sex- and Gender-Related Diagnostic Issues

Avoidant personality disorder appears to be more common in women than in men in community surveys. This gender difference in prevalence is small but consistently found in large population-based samples.

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Differential Diagnosis

Social anxiety disorder. There appears to be a great deal of overlap between avoidant personality disorder and social anxiety disorder. It has been suggested that they may represent different manifestations of similar underlying problems, or avoidant personality disorder may be a more severe form of social anxiety disorder. However, differences have also been described, especially in relation to self-concept (such as self-esteem and the sense of inferiority in avoidant personality disorder); the latter is indirect evidence as it shows that negative self-concept in social anxiety disorder may be unstable and thus less pervasive and entrenched than in avoidant personality disorder. Additionally, studies have shown that avoidant personality disorder frequently occurs in the absence of social anxiety disorder, and some separate risk factors have been identified, providing support for retaining two separate diagnostic categories.

Agoraphobia. Avoidance characterizes both avoidant personality disorder and agoraphobia, and they often co-occur. They can be distinguished by the motivation for the avoidance (e.g., fear of panic or physical harm in agoraphobia).

Other personality disorders and personality traits. Other personality disorders may be confused with avoidant personality disorder because they have certain features in common. It is, therefore, important to distinguish among these disorders based on differences in their characteristic features. However, if an individual has personality features that meet criteria for one or more personality disorders in addition to avoidant personality disorder, all can be diagnosed. Both avoidant personality disorder and dependent personality disorder are characterized by feelings of

inadequacy, hypersensitivity to criticism, and a need for reassurance. Similar behaviors (e.g., unassertiveness) and attributes (e.g., low self-esteem and low self-confidence) may be observed in both dependent personality disorder and avoidant personality disorder, although other behaviors are notably divergent, such as avoidance of social proximity in avoidant personality disorder but proximity-seeking in dependent personality disorder. The motivations behind similar behaviors may be quite different. For example, the unassertiveness in avoidant personality disorder is described as more closely related to fears of being rejected or humiliated, whereas in dependent personality disorder it is motivated by the desire to avoid being left to fend for oneself. However, avoidant personality disorder and dependent personality disorder may be particularly likely to co-occur. Like avoidant personality disorder, schizoid personality disorder and schizotypal personality disorder are characterized by social isolation. However, individuals with avoidant personality disorder want to have relationships with others and feel their loneliness deeply, whereas those with schizoid or schizotypal personality disorder may be content with and even prefer their social isolation. Paranoid personality disorder and avoidant personality disorder are both characterized by a reluctance to confide in others. However, in avoidant personality disorder, this reluctance is attributable more to a fear of humiliation or being found inadequate than to a fear of others' malicious intent.

Many individuals display avoidant personality traits. Only when these traits are inflexible, maladaptive, and persisting and cause significant functional impairment or subjective distress do they constitute avoidant personality disorder.

Personality change due to another medical condition. Avoidant personality disorder must be distinguished from personality change due to another medical condition, in which the traits that emerge are a direct physiological consequence of another medical condition.

Substance use disorders. Avoidant personality disorder must also be distinguished from symptoms that may develop in association with persistent substance use.

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Comorbidity

Other disorders that are commonly diagnosed with avoidant personality disorder include depressive disorders and anxiety disorders, especially social anxiety disorder. Avoidant personality disorder also tends to be diagnosed with schizoid personality disorder. Avoidant personality disorder is associated with increased rates of substance use disorders at a similar rate to the generalized form of social anxiety disorder.

Dependent Personality Disorder

Diagnostic Criteria

F60.7

A pervasive and excessive need to be taken care of that leads to submissive and clinging behavior and fears of separation, beginning by early adulthood and present in a variety of contexts, as indicated by five (or more) of the following:

1. Has difficulty making everyday decisions without an excessive amount of advice and reassurance from others.
 2. Needs others to assume responsibility for most major areas of his or her life.
 3. Has difficulty expressing disagreement with others because of fear of loss of support or approval. (**Note:** Do not include realistic fears of retribution.)
 4. Has difficulty initiating projects or doing things on his or her own (because of a lack of self-confidence in judgment or abilities rather than a lack of motivation or energy).
 5. Goes to excessive lengths to obtain nurturance and support from others, to the point of volunteering to do things that are unpleasant.
 6. Feels uncomfortable or helpless when alone because of exaggerated fears of being unable to care for himself or herself.
 7. Urgently seeks another relationship as a source of care and support when a close relationship ends.
 8. Is unrealistically preoccupied with fears of being left to take care of himself or herself.
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Diagnostic Features

The essential feature of dependent personality disorder is a pervasive and excessive need to be taken care of that leads to submissive and clinging behavior and fears of separation. This pattern begins by early adulthood and is present in a variety of contexts. The dependent and submissive behaviors are designed to elicit caregiving and arise from a self-perception of being unable to function adequately without the help of others.

Individuals with dependent personality disorder have great difficulty making everyday decisions (e.g., what color shirt to wear to work or whether to carry an umbrella) without an excessive amount of advice and reassurance from others (Criterion 1). These individuals tend to be passive and to allow other people (often a single other person) to take the initiative and assume responsibility for most major areas of their lives (Criterion 2). Adults with this disorder typically depend on a parent or spouse to decide where they should live, what kind of job they should have, and which neighbors to befriend. Adolescents with this disorder may allow their parent(s) to decide what they should wear, with whom they should associate, how they should spend their free time, and what school or college they should attend. This need for others to assume responsibility goes beyond age-appropriate and situation-appropriate requests for assistance from others (e.g., the specific needs of children, elderly persons, and handicapped persons). Dependent personality disorder may occur in an individual who has a serious medical condition or disability, but in such cases the difficulty in taking responsibility must go beyond what would normally be associated with that condition or disability.

Because they fear losing support or approval, individuals with dependent personality disorder often have difficulty expressing disagreement with other individuals, especially those on whom they are dependent (Criterion 3). These individuals feel so unable to function alone that they will agree with things that they feel are wrong rather than risk losing the help of those to whom they

look for guidance. They do not express anger toward others whose support and nurturance they need for fear of alienating them. If the individual's concerns regarding the consequences of expressing disagreement are realistic (e.g., realistic fears of retribution from an abusive spouse), the behavior should not be considered to be evidence of dependent personality disorder.

Individuals with this disorder have difficulty initiating projects or doing things independently (Criterion 4). They lack self-confidence and believe that they need help to begin and carry through tasks. They will wait for others to start things because they believe that as a rule others can do them better. These individuals are convinced that they are incapable of functioning independently and present themselves as inept and requiring constant assistance. They are, however, likely to function adequately if given the assurance that someone else is supervising and approving. There may be a fear of becoming or appearing to be more competent, because they may believe that this will lead to loss of support. Because they rely on others to handle their problems, they often do not learn the skills of independent living, thus perpetuating dependency.

Individuals with dependent personality disorder may go to excessive lengths to obtain nurturance and support from others, even to the point of volunteering for unpleasant tasks if such behavior will bring the care they need (Criterion 5). They are willing to submit to what others want, even if the demands are unreasonable. Their need to maintain an important bond will often result in imbalanced or distorted relationships. They may make extraordinary self-sacrifices or tolerate verbal, physical, or sexual abuse. (It should be noted that this behavior should be considered evidence of dependent personality disorder only when it can clearly be established that other options are available to the individual.) Individuals with this disorder feel uncomfortable or helpless when alone because of their exaggerated fears of being unable to care for themselves (Criterion 6).

When a close relationship ends (e.g., a breakup with a lover; the death of a caregiver), individuals with dependent personality disorder may urgently seek another relationship to provide the care and support they need (Criterion 7). Their belief that they are unable to function in the absence of a close relationship motivates these individuals to become quickly and indiscriminately attached to another individual. Individuals with this disorder are often preoccupied with fears of being left to care for themselves (Criterion 8). They see themselves as so totally dependent on the advice and help of an important other person that they worry about losing the support of that person when there are no grounds to justify such fears. To be considered as evidence of this criterion, the fears must be excessive and unrealistic. For example, an elderly man with cancer who moves into his son's household for care is exhibiting dependent behavior that is appropriate given this person's life circumstances.

Associated Features

Individuals with dependent personality disorder are often characterized by pessimism and self-doubt and tend to belittle their abilities and assets. They take criticism and disapproval as proof of their worthlessness and lose faith in themselves. They may seek overprotection and dominance from others. Occupational functioning may be impaired if independent initiative is required. They may avoid positions of responsibility and become anxious when faced with decisions.

Prevalence

The estimated prevalence of dependent personality disorder based on a probability subsample from Part II of the National Comorbidity Survey Replication was 0.6%. The

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prevalence of dependent personality disorder in the National Epidemiologic Survey on Alcohol and Related Conditions was 0.5%. A review of six epidemiological studies (four in the United States) found a median prevalence of 0.4%.

Development and Course

This diagnosis should be used with great caution, if at all, in children and adolescents, for whom dependent behavior may be developmentally appropriate.

Culture-Related Diagnostic Issues

The degree to which dependent behaviors are considered to be appropriate varies substantially across different age and sociocultural groups. Age and cultural factors need to be considered in evaluating the diagnostic threshold of each criterion. Dependent behavior should be considered characteristic of the disorder only when it is clearly in excess of the individual's cultural norms or reflects unrealistic concerns. An emphasis on passivity, politeness, and deferential treatment is characteristic of some societies and may be misinterpreted as traits of dependent personality disorder. Similarly, societies may differentially foster and discourage dependent behavior in males and females. Individuals with dependent personality disorder exhibit a pervasive inability to make decisions, continuous feelings of subjugation, lack of initiative, silence, and social distancing that are far in excess of usual cultural norms of politeness and purposeful passivity.

Sex- and Gender-Related Diagnostic Issues

In clinical and community settings, dependent personality disorder has been diagnosed more frequently in women compared with men.

Differential Diagnosis

Separation anxiety disorder in adults. Adults with separation anxiety disorder are typically overconcerned about their offspring, spouses, parents, and pets, and experience marked discomfort when separated from them. In contrast, individuals with dependent personality disorder feel uncomfortable or helpless when alone because of exaggerated fears of being unable to take care of themselves.

Other mental disorders and medical conditions. Dependent personality disorder must be distinguished from dependency arising as a consequence of other mental disorders (e.g., depressive disorders, panic disorder, agoraphobia) and as a result of other medical conditions.

Other personality disorders and personality traits. Other personality disorders may be confused with dependent personality disorder because they have certain features in common. It is therefore important to distinguish among these disorders based on differences in their characteristic features. However, if an individual has personality features that meet criteria for one or more personality disorders in addition to dependent personality disorder, all can be diagnosed.

Although many personality disorders are characterized by dependent features, dependent personality disorder can be distinguished by its predominantly submissive and clinging behavior and by the person's self-perception of not being able to function adequately without the help and support of others. Both dependent personality disorder and borderline personality disorder are characterized by fear of abandonment; however, the individual with borderline personality disorder reacts to abandonment with feelings of emotional emptiness, rage, and demands, whereas the individual with dependent personality disorder reacts with increasing appeasement and submissiveness and urgently seeks a replacement relationship to provide caregiving and support. Borderline personality disorder can further be distinguished from dependent

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personality disorder by a typical pattern of unstable and intense relationships. Individuals with histrionic personality disorder, like those with dependent personality disorder, have a strong need for reassurance and approval and may appear childlike and clinging. However, unlike dependent personality disorder, which is characterized by self-effacing and docile behavior, histrionic personality disorder is characterized by gregarious flamboyance with active demands for attention. Moreover, individuals with histrionic personality disorder typically have less insight regarding their underlying dependency needs than do people with dependent personality disorder. Both dependent personality disorder and avoidant personality disorder are characterized by feelings of inadequacy, hypersensitivity to criticism, and a need for reassurance; however, individuals with avoidant personality disorder have such a strong fear of humiliation and rejection that they withdraw until they are certain they will be accepted. In contrast, individuals with dependent personality disorder have a pattern of seeking and maintaining connections to important others, rather than avoiding and withdrawing from relationships.

Many individuals display dependent personality traits. Only when these traits are inflexible, maladaptive, and persisting and cause significant functional impairment or subjective distress do they constitute dependent personality disorder.

Personality change due to another medical condition. Dependent personality disorder must be distinguished from personality change due to another medical condition, in which the traits that emerge are a direct physiological consequence of another medical condition.

Substance use disorders. Dependent personality disorder must also be distinguished from symptoms that may develop in association with persistent substance use.

Comorbidity

There may be an increased risk of depressive disorders, anxiety disorders, and adjustment disorders. Dependent personality disorder often co-occurs with other personality disorders, especially borderline, avoidant, and histrionic personality disorders. Chronic physical illness or persistent separation anxiety disorder in childhood or adolescence may predispose the individual to the development of this disorder.

Obsessive-Compulsive Personality Disorder

A pervasive pattern of preoccupation with orderliness, perfectionism, and mental and interpersonal control, at the expense of flexibility, openness, and efficiency, beginning by early adulthood and present in a variety of contexts, as indicated by four (or more) of the following:

1. Is preoccupied with details, rules, lists, order, organization, or schedules to the extent that the major point of the activity is lost.
2. Shows perfectionism that interferes with task completion (e.g., is unable to complete a project because his or her own overly strict standards are not met).
3. Is excessively devoted to work and productivity to the exclusion of leisure activities and friendships (not accounted for by obvious economic necessity).
4. Is overconscientious, scrupulous, and inflexible about matters of morality, ethics, or values (not accounted for by cultural or religious identification).

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5. Is unable to discard worn-out or worthless objects even when they have no sentimental value.
6. Is reluctant to delegate tasks or to work with others unless they submit to exactly his or her way of doing things.
7. Adopts a miserly spending style toward both self and others; money is viewed as something to be hoarded for future catastrophes.
8. Shows rigidity and stubbornness.

Diagnostic Features

The essential feature of obsessive-compulsive personality disorder is a preoccupation with orderliness, perfectionism, and mental and interpersonal control, at the expense of flexibility, openness, and efficiency. This pattern begins by early adulthood and is present in a variety of contexts.

Individuals with obsessive-compulsive personality disorder attempt to maintain a sense of control through painstaking attention to rules, trivial details, procedures, lists, schedules, or form to the extent that the major point of the activity is lost (Criterion 1). They are excessively careful and prone to repetition, paying extraordinary attention to detail and repeatedly checking for possible mistakes, losing track of time in the process. For example, when such individuals misplace a list of things to be done, they will spend an inordinate amount of time looking for the list rather than spending a few moments trying their best to recreate it from memory and proceeding to accomplish the tasks. They dismiss the fact that other people tend to become very annoyed at the delays and inconveniences that result from this behavior because they preferentially respond to either their anxiety about making a mistake or their insistence on how things should be done. Time is poorly allocated, and the most important tasks are left to the last moment. The perfectionism and self-imposed high standards of performance cause significant dysfunction and distress in these individuals. They may become so involved in making every

detail of a project absolutely perfect that the project is never finished (Criterion 2). For example, the completion of a written report is delayed by numerous time-consuming rewrites that all come up short of “perfection.” Deadlines are routinely missed or the individual has a pattern of exerting extraordinary effort (e.g., working through the night, skipping meals) in order to make the deadline at the last moment, and aspects of the individual’s life that are not the current focus of activity may fall into disarray.

Individuals with obsessive-compulsive personality disorder display excessive devotion to work and productivity to the exclusion or devaluing of leisure activities and friendships (Criterion 3). This behavior is not accounted for by economic necessity. They often feel that they do not have time to take an evening or a weekend day off to go on an outing or to just relax. They may keep postponing a pleasurable activity, such as a vacation, so that it may never occur. When they reluctantly take time for leisure activities or vacations, they are very uncomfortable unless they have taken along something to work on so they do not “waste time.” There may be a great concentration on household chores (e.g., repeated excessive cleaning so that “one could eat off the floor”). If they spend time with friends, it is likely to be in some kind of formally organized activity (e.g., sports). Hobbies or recreational activities are approached as serious tasks or with methodical intensity, requiring careful organization and hard work to master. The emphasis is on perfect performance. These individuals turn play into a structured work-like task (e.g., correcting an infant for not putting rings on the post in the right order; telling a toddler to ride their tricycle in a straight line; turning a baseball game into a harsh “lesson”).

Individuals with obsessive-compulsive personality disorder may be excessively conscientious, scrupulous, and inflexible about matters of morality, ethics, or values (Criterion 4). They may force themselves and others to follow rigid moral principles and very strict standards of performance. They may also be mercilessly self-critical about their own

mistakes or harshly judgmental of others’ moral or ethical missteps. Individuals with this disorder are rigidly deferential to authority and rules and insist on quite literal compliance, with no rule bending for extenuating circumstances. For example, the individual will not lend a dollar to a friend who is short of the fare needed to get on a bus because “neither a borrower nor a lender be” or because it would be “bad” for the friend’s character. These qualities should not be accounted for by the individual’s cultural or religious identification.

Individuals with this disorder may be unable to discard worn-out or worthless objects, even when they have no sentimental value (Criterion 5). Often these individuals will admit to being “pack rats.” They regard discarding objects as wasteful because “you never know when you might need something.” The clutter may also result from an accumulation of partially read learning material or unfinished projects that the individual intends to get to someday but that have been sidelined because of procrastination and/or a meticulous yet slow work style. These individuals will become upset if someone tries to get rid of the things they have saved. Their spouses or roommates may complain about the amount of space taken up by old parts, piles of reading material, broken appliances, and so on.

Individuals with obsessive-compulsive personality disorder are reluctant to delegate tasks or to work with others (Criterion 6). They stubbornly and unreasonably insist that everything be done their way and that people conform to their way of doing things. They often give very

detailed instructions about how things should be done (e.g., there is one and only one way to mow the lawn, wash the dishes, load the dishwasher, build a doghouse), even to the point of micromanaging others, and are surprised and irritated if others suggest creative alternatives. At other times they may reject offers of help even when behind schedule because they believe no one else can do it right.

Individuals with this disorder may be miserly and stingy (having difficulty spending money on both themselves and others) and maintain a standard of living far below what they can afford, believing that spending must be tightly controlled to provide for future catastrophes (Criterion 7). Obsessive-compulsive personality disorder is characterized by rigidity and stubbornness (Criterion 8). Individuals with this disorder are so concerned about having things done the one “correct” way that they have trouble going along with anyone else’s ideas. These individuals plan ahead in meticulous detail and are unwilling to consider changes to these plans or their usual routines. Totally wrapped up in their own perspective, they have difficulty acknowledging the viewpoints of others. Friends and colleagues may become frustrated by this constant rigidity. Even when individuals with obsessive-compulsive personality disorder recognize that it may be in their interest to compromise, they may stubbornly refuse to do so, arguing that it is “the principle of the thing.”

Associated Features

When rules and established procedures do not dictate the correct answer, decision-making may become a time-consuming, often painful process (e.g., exhaustively researching options before making a purchase). Individuals with obsessive-compulsive personality disorder may have such difficulty deciding which tasks take priority or what is the best way of doing some particular task that they may never get started on anything. They are prone to become upset or angry in situations in which they are not able to maintain control of their physical or interpersonal environment, although the anger is typically not expressed directly. For example, an individual may be angry when service in a restaurant is poor, but instead of complaining to the management, the individual ruminates about how much to leave as a tip. On other occasions, anger may be expressed with righteous indignation over a seemingly minor matter. Individuals with this disorder may be especially attentive to their relative status in dominance-submission relationships and may display excessive deference to an authority they respect and excessive resistance to authority they do not respect.

Individuals with this disorder have difficulty relating to and sharing emotions. For example, they may express affection in a highly controlled or stilted fashion and may be very uncomfortable in the presence of others who are emotionally expressive. Their everyday relationships have a formal and serious quality, and they may be stiff in situations in which others would smile and be happy (e.g., greeting a lover at the airport). They carefully hold themselves back until they are sure that whatever they say will be perfect. They may be preoccupied with logic and intellect and intolerant of displays of emotion in others. They often have difficulty expressing tender feelings, rarely paying compliments. Individuals with this disorder may experience occupational difficulties and distress, particularly when confronted with

new situations that demand flexibility and compromise.

Prevalence

The estimated prevalence of obsessive-compulsive personality disorder based on a probability subsample from Part II of the National Comorbidity Survey Replication was 2.4%. The prevalence of obsessive-compulsive personality disorder in the National Epidemiologic Survey on Alcohol and Related Conditions was 7.9%. A review of five epidemiological studies (three in the United States) found a median prevalence of 4.7%.

Culture-Related Diagnostic Issues

In assessing an individual for obsessive-compulsive personality disorder, the clinician should not include those behaviors that reflect habits, customs, or interpersonal styles that are culturally sanctioned by the individual's reference group. Certain cultural communities place substantial emphasis on work and productivity, and some members of sociocultural groups (e.g., certain religious groups, professions, migrants) may at times rigidly embrace codes of conduct; work demands; restrictive social environments; rules of behavior; or standards that emphasize overconscientiousness, moral scrupulosity, and striving for perfectionism that may be reinforced by norms of the cultural group. Such behaviors should not on their own be considered indications of obsessive-compulsive personality disorder.

Sex- and Gender-Related Diagnostic Issues

In large population-based studies, obsessive-compulsive personality disorder appears to be equally prevalent in men and women.

Differential Diagnosis

Obsessive-compulsive disorder (OCD). Despite the similarity in names, OCD is usually easily distinguished from obsessive-compulsive personality disorder by the presence of true obsessions and compulsions in OCD. When criteria for both obsessive-compulsive personality disorder and OCD are met, both diagnoses should be recorded.

Hoarding disorder. A diagnosis of hoarding disorder should be considered especially when hoarding is extreme (e.g., accumulated stacks of worthless objects present a fire hazard and make it difficult for others to walk through the house). When criteria for both obsessive-compulsive personality disorder and hoarding disorder are met, both diagnoses should be recorded.

Other personality disorders and personality traits. Other personality disorders may be confused with obsessive-compulsive personality disorder because they have certain features in common. It is, therefore, important to distinguish among these disorders based on differences in their characteristic features. However, if an individual has personality

features that meet criteria for one or more personality disorders in addition to obsessive-compulsive personality disorder, all can be diagnosed. Individuals with narcissistic personality disorder may also profess a commitment to perfectionism and believe that others cannot do

things as well, but these individuals are more likely to believe that they have achieved perfection, whereas those with obsessive-compulsive personality disorder are usually self-critical. Individuals with narcissistic or antisocial personality disorder lack generosity but will indulge themselves, whereas those with obsessive-compulsive personality disorder adopt a miserly spending style toward both self and others. Both schizoid personality disorder and obsessive-compulsive personality disorder may be characterized by an apparent formality and social detachment. In obsessive-compulsive personality disorder, this stems from discomfort with emotions and excessive devotion to work, whereas in schizoid personality disorder there is a fundamental lack of capacity for intimacy.

Obsessive-compulsive personality traits in moderation may be especially adaptive, particularly in situations that reward high performance. Only when these traits are inflexible, maladaptive, and persisting and cause significant functional impairment or subjective distress do they constitute obsessive-compulsive personality disorder.

Personality change due to another medical condition. Obsessive-compulsive personality disorder must be distinguished from personality change due to another medical condition, in which the traits are a direct physiological consequence of another medical condition.

Substance use disorders. Obsessive-compulsive personality disorder must also be distinguished from symptoms that may develop in association with persistent substance use.

Comorbidity

Individuals with anxiety disorders (e.g., generalized anxiety disorder, separation anxiety disorder, social anxiety disorder, specific phobias) and OCD have an increased likelihood of having a personality disturbance that meets criteria for obsessive-compulsive personality disorder. Even so, it appears that the majority of individuals with OCD do not have a pattern of behavior that meets criteria for this personality disorder. Many of the features of obsessive-compulsive personality disorder overlap with “type A” personality characteristics (e.g., preoccupation with work, competitiveness, time urgency), and these features may be present in individuals at risk for myocardial infarction. There may be an association between obsessive-compulsive personality disorder and depressive and bipolar disorders and eating disorders.

Other Personality Disorders

Personality Change Due to Another Medical Condition

Diagnostic Criteria	F07.0
A. A persistent personality disturbance that represents a change from the individual's previous characteristic personality pattern.	
Note: In children, the disturbance involves a marked deviation from normal development or a significant change in the child's usual behavior patterns, lasting	

at least 1 year.

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- B. There is evidence from the history, physical examination, or laboratory findings that the disturbance is the direct pathophysiological consequence of another medical condition.
- C. The disturbance is not better explained by another mental disorder (including another mental disorder due to another medical condition).
- D. The disturbance does not occur exclusively during the course of a delirium.
- E. The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

Specify whether:

Labile type: If the predominant feature is affective lability.

Disinhibited type: If the predominant feature is poor impulse control as evidenced by sexual indiscretions, etc.

Aggressive type: If the predominant feature is aggressive behavior.

Apathetic type: If the predominant feature is marked apathy and indifference.

Paranoid type: If the predominant feature is suspiciousness or paranoid ideation.

Other type: If the presentation is not characterized by any of the above subtypes.

Combined type: If more than one feature predominates in the clinical picture.

Unspecified type

Coding note: Include the name of the other medical condition (e.g., F07.0 personality change due to temporal lobe epilepsy). The other medical condition should be coded and listed separately immediately before the personality change due to another medical condition (e.g., G40.209 temporal lobe epilepsy; F07.0 personality change due to temporal lobe epilepsy).

Subtypes

The particular personality change can be specified by indicating the symptom presentation that predominates in the clinical presentation.

Diagnostic Features

The essential feature of a personality change due to another medical condition is a persistent personality disturbance that is judged to be a physiological consequence of another medical condition. The personality disturbance represents a change from the individual's previous characteristic personality pattern. In children, this condition may be manifested as a marked deviation from normal development rather than as a change in a stable personality pattern (Criterion A). There must be evidence from the history, physical examination, or laboratory

findings that the personality change is the direct physiological consequence of another medical condition (Criterion B). The diagnosis is not given if the disturbance is better explained by another mental disorder (Criterion C). The diagnosis is not given if the disturbance occurs exclusively during the course of a delirium (Criterion D). The disturbance must also cause clinically significant distress or impairment in social, occupational, or other important areas of functioning (Criterion E).

Common manifestations of the personality change include affective instability, poor impulse control, outbursts of aggression or rage grossly out of proportion to any precipitating psychosocial stressor, marked apathy, suspiciousness, or paranoid ideation. The phenomenology of the change is indicated using the subtypes listed in the criteria set. An individual with the disorder is often characterized by others as “not himself [or herself].” Although it shares the term “personality” with the other personality disorders, this diagnosis is distinct by virtue of its specific etiology, different phenomenology, and more variable onset and course.

The clinical presentation in a given individual may depend on the nature and localization of the pathological process. For example, injury to the frontal lobes may yield

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symptoms such as lack of judgment or foresight, facetiousness, disinhibition, and euphoria. In this example, the diagnosis of personality change due to frontal lobe injury would be made if a persistent personality disturbance is a deviation from the individual’s previous characteristic personality pattern prior to the injury (Criterion A). Right hemisphere strokes have often been shown to evoke personality changes in association with unilateral spatial neglect, anosognosia (i.e., inability of the individual to recognize a bodily or functional deficit, such as the existence of hemiparesis), motor impersistence, and other neurological deficits.

Associated Features

A variety of neurological and other medical conditions may cause personality changes, including central nervous system neoplasms, head trauma, cerebrovascular disease, Huntington’s disease, epilepsy, infectious conditions with central nervous system involvement (e.g., HIV), endocrine conditions (e.g., hypothyroidism, hypo- and hyperadrenocorticism), and autoimmune conditions with central nervous system involvement (e.g., systemic lupus erythematosus). The associated physical examination findings, laboratory findings, and patterns of prevalence and onset reflect those of the neurological or other medical condition involved.

Differential Diagnosis

Chronic medical conditions associated with pain and disability. Chronic medical conditions associated with pain and disability can also be associated with changes in personality. The diagnosis of personality change due to another medical condition is given only if a direct pathophysiological mechanism can be established. This diagnosis is not given if the change is due to a behavioral or psychological adjustment or response to another medical condition (e.g., dependent behaviors that result from a need for the assistance of others following a severe head trauma, cardiovascular disease, or dementia).

Delirium or major neurocognitive disorder. Personality change is a frequently associated feature of a

delirium or major neurocognitive disorder. A separate diagnosis of personality change due to another medical condition is not given if the change occurs exclusively during the course of a delirium. However, the diagnosis of personality change due to another medical condition may be given in addition to the diagnosis of major neurocognitive disorder if the personality change is judged to be a physiological consequence of the pathological process causing the neurocognitive disorder and if the personality change is a prominent part of the clinical presentation.

Another mental disorder due to another medical condition. The diagnosis of personality change due to another medical condition is not given if the disturbance is better explained by another mental disorder due to another medical condition (e.g., depressive disorder due to brain tumor).

Substance use disorders. Personality changes may also occur in the context of substance use disorders, especially if the disorder is long-standing. The clinician should inquire carefully about the nature and extent of substance use. If the clinician wishes to indicate an etiological relationship between the personality change and substance use, the other specified category for the specific substance can be used (e.g., other specified stimulant-related disorder with personality change).

Other mental disorders. Marked personality changes may also be an associated feature of other mental disorders (e.g., schizophrenia; delusional disorder; depressive and bipolar disorders; other specified and unspecified disruptive behavior, impulse-control, and conduct disorders; panic disorder). However, in these disorders, no specific physiological factor is judged to be etiologically related to the personality change.

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Other personality disorders. Personality change due to another medical condition can be distinguished from a personality disorder by the requirement for a clinically significant change from baseline personality functioning and the presence of a specific etiological medical condition.

Other Specified Personality Disorder

F60.89

This category applies to presentations in which symptoms characteristic of a personality disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the personality disorders diagnostic class. The other specified personality disorder category is used in situations in which the clinician chooses to communicate the specific reason that the presentation does not meet the criteria for any specific personality disorder. This is done by recording "other specified personality disorder" followed by the specific reason (e.g., "mixed personality features").

Unspecified Personality Disorder

F60.9

This category applies to presentations in which symptoms characteristic of a personality disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the personality disorders diagnostic class. The unspecified personality disorder category is used in situations in which the clinician chooses *not* to specify the reason that the criteria are not met for a specific personality disorder and includes presentations in which there is insufficient information to make a more specific diagnosis.

Paraphilic Disorders

Paraphilic disorders included in this manual are voyeuristic disorder (spying on others in private activities), exhibitionistic disorder (exposing the genitals), frotteuristic disorder (touching or rubbing against a nonconsenting person), sexual masochism disorder (undergoing humiliation, bondage, or suffering), sexual sadism disorder (inflicting humiliation, bondage, or suffering), pedophilic disorder (sexual focus on children), fetishistic disorder (using nonliving objects or having a highly specific focus on nongenital body parts), and transvestic disorder (engaging in sexually arousing cross-dressing). These disorders have traditionally been selected for specific listing and assignment of explicit diagnostic criteria in DSM for two main reasons: they are relatively common, in relation to other paraphilic disorders, and some of them entail actions for their satisfaction that, because of their noxiousness or potential harm to others, are classed as criminal offenses. The eight listed disorders do not exhaust the list of possible paraphilic disorders. Many dozens of distinct paraphilias have been identified and named, and almost any of them could, by virtue of its negative consequences for the individual or for others, rise to the level of a paraphilic disorder.

In this chapter, the order of presentation of the listed paraphilic disorders generally corresponds to common classification schemes for these conditions. The first group of disorders is based on *anomalous activity preferences*. These disorders are subdivided into *courtship disorders*, which resemble distorted components of human courtship behavior (voyeuristic disorder, exhibitionistic disorder, and frotteuristic disorder), and *algolagnic disorders*, which involve pain and suffering (sexual masochism disorder and sexual sadism disorder). The second group of disorders is based on *anomalous target preferences*. These disorders include one directed at other humans (pedophilic disorder) and two directed elsewhere (fetishistic disorder and transvestic disorder).

The term *paraphilia* denotes any intense and persistent sexual interest other than sexual interest in genital stimulation or preparatory fondling with phenotypically normal, physically mature, consenting human partners. In some circumstances, the criteria “intense and persistent” may be difficult to apply, such as in the assessment of persons who are very old or medically ill and who may not have “intense” sexual interests of any kind. In such circumstances, the term *paraphilia* may be defined as any sexual interest greater than or equal to nonparaphilic sexual interests. There are also specific paraphilias that are generally better described as *preferential* sexual interests than as intense sexual interests.

Some paraphilias primarily concern the individual’s erotic activities, and others primarily concern the individual’s erotic targets. Examples of the former would include intense and persistent interests in spanking, whipping, cutting, binding, or strangulating another person, or an interest in these activities that equals or exceeds the individual’s interest in copulation or equivalent interaction with another person. Examples of the latter would include intense or preferential sexual interest in children, corpses, or amputees (as a class), as well as intense or

preferential interest in nonhuman animals, such as horses or dogs, or in inanimate objects, such as shoes or articles made of rubber. An individual's pattern of paraphilic interests is often reflected in his or her choice of pornography.

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A *paraphilic disorder* is a paraphilia that is currently causing distress or impairment to the individual or a paraphilia whose satisfaction has entailed personal harm, or risk of harm, to others. A paraphilia is a necessary but not a sufficient condition for having a paraphilic disorder, and a paraphilia by itself does not necessarily justify or require clinical intervention.

In the diagnostic criteria set for each of the listed paraphilic disorders, Criterion A specifies the qualitative nature of the paraphilia (e.g., an erotic focus on children or on exposing the genitals to strangers), and Criterion B specifies the negative consequences of the paraphilia (i.e., distress, impairment, or harm to others). In keeping with the distinction between paraphilias and paraphilic disorders, the term *diagnosis* should be reserved for individuals whose paraphilic interests or behaviors meet both Criteria A and B (i.e., individuals who have a paraphilic disorder). If an individual's paraphilic interests or behaviors meet Criterion A but not Criterion B for a particular paraphilia—a circumstance that might arise when a benign paraphilia is discovered during the clinical investigation of some other condition—then the individual may be said to have that paraphilia but not a paraphilic disorder.

It is not rare for an individual to manifest two or more paraphilias. In some cases, the paraphilic foci are closely related and the connection between the paraphilias is intuitively comprehensible (e.g., foot fetishism and shoe fetishism). In other cases, the connection between the paraphilias is not obvious, and the presence of multiple paraphilias may be coincidental or else related to some generalized vulnerability to anomalies of psychosexual development. In any event, comorbid diagnoses of separate paraphilic disorders may be warranted if more than one paraphilia is causing suffering to the individual or harm to others.

Because of the two-pronged nature of diagnosing paraphilic disorders, clinician-rated or self-rated measures and severity assessments could address either the strength of the paraphilia itself or the seriousness of its consequences. Although the distress and impairment stipulated in the Criterion B are special in being the immediate or ultimate result of the paraphilia and not primarily the result of some other factor, the phenomena of reactive depression, anxiety, guilt, poor work history, impaired social relations, and so on are not unique in themselves and may be quantified with multipurpose measures of psychosocial functioning or quality of life.

Voyeuristic Disorder

Diagnostic Criteria

F65.3

- A. Over a period of at least 6 months, recurrent and intense sexual arousal from observing an unsuspecting person who is naked, in the process of disrobing, or engaging in sexual activity, as manifested by fantasies, urges, or behaviors.
- B. The individual has acted on these sexual urges with a nonconsenting person, or

the sexual urges or fantasies cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

- C. The individual experiencing the arousal and/or acting on the urges is at least 18 years of age.

Specify if:

In a controlled environment: This specifier is primarily applicable to individuals living in institutional or other settings where opportunities to engage in voyeuristic behavior are restricted.

In full remission: The individual has not acted on the urges with a nonconsenting person, and there has been no distress or impairment in social, occupational, or other areas of functioning, for at least 5 years while in an uncontrolled environment.

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Specifiers

The “in full remission” specifier does not address the continued presence or absence of voyeurism per se, which may still be present after behaviors and distress have remitted.

Diagnostic Features

The diagnostic criteria for voyeuristic disorder can apply both to individuals who more or less freely disclose this paraphilic interest and to those who categorically deny any sexual arousal from observing an unsuspecting person who is naked, disrobing, or engaged in sexual activity despite substantial objective evidence to the contrary. If disclosing individuals also report distress or psychosocial problems because of their voyeuristic sexual preferences, they could be diagnosed with voyeuristic disorder. On the other hand, if they declare no distress, demonstrated by lack of anxiety, obsessions, guilt, or shame, about these paraphilic impulses and are not impaired in other important areas of functioning because of this sexual interest, and their psychiatric or legal histories indicate that they do not act on it, they could be ascertained as having voyeuristic sexual interest but should *not* be diagnosed with voyeuristic disorder.

Nondisclosing individuals include, for example, individuals known to have been spying repeatedly on unsuspecting persons who are naked or engaging in sexual activity on separate occasions but who deny any urges or fantasies concerning such sexual behavior, and who may report that known episodes of watching unsuspecting naked or sexually active persons were all accidental and nonsexual. Others may disclose past episodes of observing unsuspecting naked or sexually active persons but contest any significant or sustained sexual interest in this behavior. Since these individuals deny having fantasies or impulses about watching others nude or involved in sexual activity, it follows that they would also reject feeling subjectively distressed or socially impaired by such impulses. Despite their nondisclosing stance, such individuals may be diagnosed with voyeuristic disorder. Recurrent voyeuristic behavior constitutes sufficient support for voyeurism (by fulfilling Criterion A) and simultaneously demonstrates that this paraphilically motivated behavior is causing harm to others (by fulfilling Criterion B).

“Recurrent” spying on unsuspecting persons who are naked or engaging in sexual activity

may be interpreted as requiring multiple victims, each on a separate occasion; this requirement for multiple victims on separate occasions is relevant because it increases the confidence in the clinical inference that the individual is motivated by voyeuristic disorder. Fewer victims can be interpreted as satisfying this criterion if there were multiple occasions of watching the same victim or if there is corroborating evidence of a distinct or preferential interest in secret watching of naked or sexually active unsuspecting persons. Note that multiple victims, as suggested earlier, are a sufficient but not a necessary condition for diagnosis; the criteria may also be met if the individual acknowledges intense voyeuristic sexual interest.

Adolescence and puberty generally increase sexual curiosity and activity. To reduce the risk of pathologizing normative sexual interest and behavior during pubertal adolescence, the minimum age for the diagnosis of voyeuristic disorder is 18 years (Criterion C).

Prevalence

The population prevalence of individuals whose presentations meet the full criteria for voyeuristic disorder is unknown. Voyeuristic acts, however, are the most common of potentially law-breaking sexual behaviors. For example, in a Quebec Internet and phone survey sample, the lifetime prevalence of voyeuristic behaviors was reported to be as high as 34.5% (50.3% in men, 21.2% in women). Because this same study found that an “intense desire” and “persistent behavior” occur with much less frequency (9.6% and 2.1%, respectively), the prevalence of voyeuristic disorder is likely much lower. The ratio of voyeuristic behavior in men to women was approximately 2:1 in the Quebec sample and 3:1 in a Swedish general population sample. In a study determining which particular disorders were prevalent

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in individuals incarcerated for sexual offenses, a study of 1,346 incarcerated sex offenders from Austria found a prevalence of voyeuristic disorder of 3.7%.

Development and Course

Adult men with voyeuristic disorder often first become aware of their sexual interest in secretly watching unsuspecting persons during adolescence. However, the minimum age for a diagnosis of voyeuristic disorder is 18 years because there is substantial difficulty in differentiating it from age-appropriate puberty-related sexual curiosity and activity. The persistence of voyeurism over time is unclear. With or without treatment of voyeuristic disorder, the subjective distress (e.g., guilt, shame, intense sexual frustration, loneliness) or impairment from the disorder may change over time, as may a number of factors that may potentially affect the course of the disorder, such as psychiatric morbidity, hypersexuality, and sexual impulsivity. Thus, the severity and course may vary over time. As with other sexual preferences, advancing age may be associated with decreasing voyeuristic sexual preferences and behavior.

Risk and Prognostic Factors

Temperamental. Because voyeurism is a necessary precondition for voyeuristic disorder, risk factors for voyeurism should also increase the risk of voyeuristic disorder.

Environmental. Childhood sexual abuse, substance misuse, and sexual

preoccupation/hypersexuality have been suggested as risk factors, although the causal relationship to voyeuristic behavior is uncertain and the specificity unclear.

Sex- and Gender-Related Diagnostic Issues

Voyeuristic disorder is very uncommon among women in clinical settings, whereas the ratio in men to women for single sexually arousing voyeuristic acts is less extreme and may be 2:1–3:1.

Differential Diagnosis

Voyeurism. Individuals with voyeurism experience recurrent, intense sexual arousal from the act of observing an unsuspecting person who is naked, in the process of disrobing, or engaging in sexual activity. Unless the individual acts on these urges with an unsuspecting person (e.g., surreptitiously peeping through a neighbor's window) or unless there is accompanying clinically significant distress or impairment in social, occupational, or other important areas of functioning, a diagnosis of voyeuristic disorder is not warranted.

Manic episode, major neurocognitive disorder, intellectual developmental disorder, personality change due to another medical condition, substance intoxication, and schizophrenia.

Individuals with a major neurocognitive disorder, intellectual developmental disorder, personality change due to another medical condition, or schizophrenia, or who are in a manic episode or experiencing substance intoxication, may become sexually disinhibited or have impaired judgment or impulse control and engage in voyeuristic behavior. Unless that behavior occurs at times other than in the context of one of these disorders, a diagnosis of voyeuristic disorder should not be made.

Conduct disorder and antisocial personality disorder. Conduct disorder in adolescents and antisocial personality disorder would be characterized by additional norm-breaking and antisocial behaviors, and the specific sexual interest in secretly watching unsuspecting others who are naked or engaging in sexual activity will usually be lacking.

Comorbidity

Known comorbidities in voyeuristic disorder are largely based on research with males suspected of or convicted for acts involving the secret watching of unsuspecting nude or

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sexually active persons. Hence, these comorbidities might not apply to all individuals with voyeuristic disorder. Conditions that occur comorbidly with voyeuristic disorder include hypersexuality and other paraphilic disorders, particularly exhibitionistic disorder. Depressive, bipolar, anxiety, and substance use disorders; attention-deficit/hyperactivity disorder; and conduct disorder and antisocial personality disorder are also frequent comorbid conditions.

Exhibitionistic Disorder

- A. Over a period of at least 6 months, recurrent and intense sexual arousal from the exposure of one's genitals to an unsuspecting person, as manifested by fantasies, urges, or behaviors.
- B. The individual has acted on these sexual urges with a nonconsenting person, or the sexual urges or fantasies cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

Specify whether:

Sexually aroused by exposing genitals to prepubertal children

Sexually aroused by exposing genitals to physically mature individuals

Sexually aroused by exposing genitals to prepubertal children and to physically mature individuals

Specify if:

In a controlled environment: This specifier is primarily applicable to individuals living in institutional or other settings where opportunities to expose one's genitals are restricted.

In full remission: The individual has not acted on the urges with a nonconsenting person, and there has been no distress or impairment in social, occupational, or other areas of functioning, for at least 5 years while in an uncontrolled environment.

Subtypes

The subtypes for exhibitionistic disorder are based on the age or physical maturity of the nonconsenting persons to whom the individual prefers to expose his or her genitals. The nonconsenting persons could be prepubescent children, adults, or both. This specifier should help draw adequate attention to characteristics of victims of individuals with exhibitionistic disorder to prevent co-occurring pedophilic disorder from being overlooked. However, indications that the individual with exhibitionistic disorder is sexually attracted to exposing his or her genitals to children should not preclude a diagnosis of pedophilic disorder.

Specifiers

The "in full remission" specifier does not address the continued presence or absence of exhibitionism per se, which may still be present after behaviors and distress have remitted.

Diagnostic Features

The diagnostic criteria for exhibitionistic disorder can apply both to individuals who more or less freely disclose this paraphilia and to those who categorically deny any sexual arousal from exposing their genitals to unsuspecting persons despite substantial objective evidence to the contrary. If disclosing individuals also report psychosocial difficulties because of their sexual attractions or preferences for exposing, they may be diagnosed with exhibitionistic

disorder. In contrast, if they declare no distress (exemplified by absence of anxiety, obsessions, and guilt or shame about these paraphilic impulses) and are not impaired by this sexual interest in other important areas of functioning, and their self-reported, psychiatric, or legal histories indicate that they do not act on them, they could be ascertained as having exhibitionistic sexual interest but *not* be diagnosed with exhibitionistic disorder.

Examples of nondisclosing individuals include those who have exposed themselves repeatedly to unsuspecting persons on separate occasions but who deny any urges or fantasies about such sexual behavior and who report that known episodes of exposure were all accidental and nonsexual. Others may disclose past episodes of sexual behavior involving genital exposure but refute any significant or sustained sexual interest in such behavior. Since these individuals deny having urges or fantasies involving genital exposure, it follows that they would also deny feeling subjectively distressed or socially impaired by such impulses. Such individuals may be diagnosed with exhibitionistic disorder despite their negative self-report. Recurrent exhibitionistic behavior constitutes sufficient support for exhibitionism (Criterion A) and simultaneously demonstrates that this paraphilically motivated behavior is causing harm to others (Criterion B).

“Recurrent” genital exposure to unsuspecting others may be interpreted as requiring multiple victims, each on a separate occasion; this requirement for multiple victims on separate occasions is relevant because it increases the confidence in the clinical inference that the individual is motivated by exhibitionistic disorder. Fewer victims can be interpreted as satisfying this criterion if there were multiple occasions of exposure to the same victim, or if there is corroborating evidence of a strong or preferential interest in genital exposure to unsuspecting persons. Note that multiple victims, as suggested earlier, are a sufficient but not a necessary condition for diagnosis, as criteria may be met by an individual’s acknowledging intense exhibitionistic sexual interest with distress or impairment.

Prevalence

The population prevalence of individuals whose presentations meet the full criteria for exhibitionistic disorder is unknown, although the disorder is highly unusual in women. Exhibitionistic acts, however, are not uncommon, and single sexually arousing exhibitionistic acts occur up to half as often among women compared with men. In a Quebec Internet and phone survey sample, lifetime prevalence of exhibitionistic behaviors was reported to be 30.9% (32.6% in men, 29.4% in women). Because this same study found that an “intense desire” and “persistent behavior” occur with much less frequency (4.8% and 0.8%, respectively), the prevalence of exhibitionistic disorder is likely much lower. For example, a Swedish study suggested that the lifetime prevalence of exhibitionistic disorder in the general population was 4.1% in men and 2.1% in women.

Development and Course

Adult men with exhibitionistic disorder often report that they first became aware of sexual interest in exposing their genitals to unsuspecting persons during adolescence, at a somewhat later time than the typical development of normative sexual interest in women or men. Although there is no minimum age requirement for the diagnosis of exhibitionistic disorder, it may be difficult to differentiate exhibitionistic behaviors from age-appropriate sexual curiosity in

adolescents. Whereas exhibitionistic impulses appear to emerge in adolescence or early adulthood, very little is known about persistence over time. With or without treatment of exhibitionistic disorder, the subjective distress (e.g., guilt, shame, intense sexual frustration, loneliness) or impairment from the disorder may change over time, as may a number of factors that may potentially affect the course of the disorder, such as psychiatric morbidity, hypersexuality, and sexual impulsivity. Thus, the severity and course may vary over time. As with other sexual preferences, advancing age may be associated with decreasing exhibitionistic sexual preferences and behavior.

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Risk and Prognostic Factors

Temperamental. Because exhibitionism is a necessary precondition for exhibitionistic disorder, risk factors for exhibitionism should also increase the risk of exhibitionistic disorder. Antisocial history, antisocial personality disorder, alcohol misuse, and pedophilic sexual preference might increase risk of sexual recidivism in exhibitionistic offenders. Hence, antisocial personality disorder, alcohol use disorder, and pedophilic interest may be considered risk factors for exhibitionistic disorder in men with exhibitionistic sexual preferences.

Environmental. Childhood sexual and emotional abuse and sexual preoccupation/hypersexuality have been suggested as risk factors for exhibitionism, although the causal relationship to exhibitionism is uncertain and the specificity unclear.

Differential Diagnosis

Exhibitionism. Individuals with exhibitionism experience recurrent, intense sexual arousal from the act of exposing their genitals to an unsuspecting person. Unless the individual acts on these urges with an unsuspecting person (e.g., exposing his genitals to riders on a train) or unless there is accompanying clinically significant distress or impairment in social, occupational, or other important areas of functioning, a diagnosis of exhibitionistic disorder is not warranted.

Manic episode, major neurocognitive disorder, intellectual developmental disorder, personality change due to another medical condition, substance intoxication, and schizophrenia.

Individuals with a major neurocognitive disorder, intellectual developmental disorder, personality change due to another medical condition, or schizophrenia, or who are in a manic episode or experiencing substance intoxication, may become sexually disinhibited or have impaired judgment or impulse control and engage in exhibitionistic behavior. Unless that behavior occurs at times other than in the context of one of these disorders, a diagnosis of exhibitionistic disorder should not be made.

Conduct disorder and antisocial personality disorder. Conduct disorder in adolescents and antisocial personality disorder would be characterized by additional norm-breaking and antisocial behaviors, and the specific sexual interest in exposing the genitals will usually be lacking.

Comorbidity

Known comorbidities in exhibitionistic disorder are largely based on research with individuals (almost all men) convicted for criminal acts involving genital exposure to nonconsenting persons. Hence, these comorbidities might not apply to all individuals who qualify for a

diagnosis of exhibitionistic disorder. Conditions that occur comorbidly with exhibitionistic disorder at high rates include depressive, bipolar, anxiety, and substance use disorders; hypersexuality; attention-deficit/hyperactivity disorder; other paraphilic disorders; and antisocial personality disorder.

Frotteuristic Disorder

Diagnostic Criteria	F65.81
A. Over a period of at least 6 months, recurrent and intense sexual arousal from touching or rubbing against a nonconsenting person, as manifested by fantasies, urges, or behaviors. B. The individual has acted on these sexual urges with a nonconsenting person, or the sexual urges or fantasies cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.	
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<i>Specify if:</i> In a controlled environment: This specifier is primarily applicable to individuals living in institutional or other settings where opportunities to touch or rub against a nonconsenting person are restricted. In full remission: The individual has not acted on the urges with a nonconsenting person, and there has been no distress or impairment in social, occupational, or other areas of functioning, for at least 5 years while in an uncontrolled environment.	

Specifiers

The “in remission” specifier does not address the continued presence or absence of frotteurism per se, which may still be present after behaviors and distress have remitted.

Diagnostic Features

The diagnostic criteria for frotteuristic disorder can apply both to individuals who relatively freely disclose this paraphilia and to those who firmly deny any sexual arousal from touching or rubbing against a nonconsenting person regardless of considerable objective evidence to the contrary. If disclosing individuals also report psychosocial impairment because of their sexual preferences for touching or rubbing against a nonconsenting person, they could be diagnosed with frotteuristic disorder. In contrast, if they declare no distress (demonstrated by lack of anxiety, obsessions, guilt, or shame) about these paraphilic impulses and are not impaired in other important areas of functioning because of this sexual interest, and their psychiatric or legal histories indicate that they do not act on it, they could be ascertained as having frotteuristic

sexual interest but should *not* be diagnosed with frotteuristic disorder.

Nondisclosing individuals include, for instance, individuals known to have been touching or rubbing against nonconsenting persons on separate occasions but who contest any urges or fantasies concerning such sexual behavior. Such individuals may report that identified episodes of touching or rubbing against an unwilling individual were all unintentional and nonsexual. Others may disclose past episodes of touching or rubbing against nonconsenting persons but contest any major or persistent sexual interest in this. Since these individuals deny having fantasies or impulses about touching or rubbing, they would consequently reject feeling distressed or psychosocially impaired by such impulses. Despite their nondisclosing position, such individuals may be diagnosed with frotteuristic disorder. Recurrent frotteuristic behavior constitutes satisfactory support for frotteurism (by fulfilling Criterion A) and concurrently demonstrates that this paraphilically motivated behavior is causing harm to others (by fulfilling Criterion B).

“Recurrent” touching or rubbing against a nonconsenting person may be interpreted as requiring multiple victims, each on a separate occasion; this requirement for multiple victims on separate occasions is relevant because it increases the confidence in the clinical inference that the individual is motivated by frotteuristic disorder. Fewer victims can be interpreted as satisfying this criterion if there were multiple occasions of touching or rubbing against the same unwilling individual, or corroborating evidence of a strong or preferential interest in touching or rubbing against nonconsenting persons. Note that multiple victims are a sufficient but not a necessary condition for diagnosis; criteria may also be met if the individual acknowledges intense frotteuristic sexual interest with clinically significant distress and/or impairment.

Prevalence

The population prevalence of individuals whose presentations meet the full criteria for frotteuristic disorder is unknown, but frotteuristic acts, including the uninvited sexual touching of or rubbing against another individual, may occur in up to 30% of adult men in the U.S. and Canadian general population. Prevalence of frotteuristic disorder is certainly

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much lower, considering the finding that “intense desire” and “persistent behavior” were reported infrequently (3.8% and 0.7%, respectively). In outpatient settings for men with paraphilic disorders and hypersexuality, approximately 10%–14% have a presentation that meets diagnostic criteria for frotteuristic disorder. Prevalence among women is likely lower.

Development and Course

Adult men with frotteuristic disorder often report first becoming aware of their sexual interest in surreptitiously touching unsuspecting persons during late adolescence or emerging adulthood. However, children and adolescents may also touch or rub against unwilling others in the absence of a diagnosis of frotteuristic disorder. Although there is no minimum age for the diagnosis, frotteuristic disorder can be difficult to differentiate from conduct-disordered behavior without sexual motivation in individuals at younger ages. The persistence of frotteurism over time is unclear. With or without treatment of frotteuristic disorder, the subjective distress (e.g., guilt,

shame, intense sexual frustration, loneliness) or impairment from the disorder may change over time, as may a number of factors that may potentially affect the course of the disorder, such as psychiatric morbidity, hypersexuality, and sexual impulsivity. Thus, the severity and course may vary over time. As with other sexual preferences, advancing age may be associated with decreasing frotteuristic sexual preferences and behavior.

Risk and Prognostic Factors

Temperamental. Nonsexual antisocial behavior and sexual preoccupation/hypersexuality might be nonspecific risk factors, although the causal relationship to frotteurism is uncertain and the specificity unclear. However, because frotteurism is a necessary precondition for frotteuristic disorder, risk factors for frotteurism should also increase the risk of frotteuristic disorder.

Differential Diagnosis

Frotteurism. Individuals with frotteurism experience recurrent intense sexual arousal from the act of touching or rubbing against a nonconsenting person. Unless the individual acts on these urges with a nonconsenting person (e.g., rubbing his genitals against a passenger on a crowded subway car) or unless there is accompanying clinically significant distress or impairment in social, occupational, or other important areas of functioning, a diagnosis of frotteuristic disorder is not warranted.

Manic episode, major neurocognitive disorder, intellectual developmental disorder, personality change due to another medical condition, substance intoxication, and schizophrenia.

Individuals with a major neurocognitive disorder, intellectual developmental disorder, personality change due to another medical condition, or schizophrenia, or who are in a manic episode or experiencing substance intoxication, may become sexually disinhibited or have impaired judgment or impulse control and engage in frotteuristic behavior. Unless that behavior occurs at times other than in the context of one of these disorders, a diagnosis of frotteuristic disorder should not be made.

Conduct disorder and antisocial personality disorder. Conduct disorder in adolescents and antisocial personality disorder would be characterized by additional norm-breaking and antisocial behaviors, and the specific sexual interest in touching or rubbing against a nonconsenting person will usually be lacking.

Comorbidity

Known comorbidities in frotteuristic disorder are largely based on research with men suspected of or convicted for criminal acts involving sexually motivated touching of or

rubbing against a nonconsenting person. Hence, these comorbidities might not apply to other individuals with a diagnosis of frotteuristic disorder based on subjective distress over their sexual interest. Conditions that occur comorbidly with frotteuristic disorder include hypersexuality and other paraphilic disorders, particularly exhibitionistic disorder and voyeuristic disorder. Conduct disorder, antisocial personality disorder, depressive disorders, bipolar disorders, anxiety disorders, and substance use disorders also co-occur.

Sexual Masochism Disorder

Diagnostic Criteria	F65.51
A. Over a period of at least 6 months, recurrent and intense sexual arousal from the act of being humiliated, beaten, bound, or otherwise made to suffer, as manifested by fantasies, urges, or behaviors. B. The fantasies, sexual urges, or behaviors cause clinically significant distress or impairment in social, occupational, or other important areas of functioning. <i>Specify if:</i> With asphyxiophilia: If the individual engages in the practice of achieving sexual arousal related to restriction of breathing. <i>Specify if:</i> In a controlled environment: This specifier is primarily applicable to individuals living in institutional or other settings where opportunities to engage in masochistic sexual behaviors are restricted. In full remission: There has been no distress or impairment in social, occupational, or other areas of functioning for at least 5 years while in an uncontrolled environment.	

Diagnostic Features

The diagnostic criteria for sexual masochism disorder are intended to apply to individuals who freely admit to having such paraphilic interests. Such individuals openly acknowledge intense sexual arousal from the act of being humiliated, beaten, bound, or otherwise made to suffer, as manifested by fantasies, urges, or behaviors. If these individuals also report psychosocial difficulties because of their sexual attractions or preferences for being humiliated, beaten, bound, or otherwise made to suffer, they may be diagnosed with sexual masochism disorder. In contrast, if they declare no distress, exemplified by anxiety, obsessions, guilt, or shame, about these paraphilic impulses, and are not hampered by them in pursuing other personal goals, they could be ascertained as having masochistic sexual interest but should *not* be diagnosed with sexual masochism disorder.

The term *bondage-domination-sadism-masochism* (BDSM) is broadly used to refer to a wide range of behaviors that individuals with sexual masochism and/or sexual sadism (as well as other individuals with similar sexual interests) engage in, such as restraints or restriction, discipline, spanking, slapping, sensory deprivation (e.g., using blindfolds), and dominance-submission role-play involving themes such as master/enslaved person, owner/pet, or kidnapper/victim.

Associated Features

The extensive use of pornography involving the act of being humiliated, beaten, bound, or otherwise made to suffer is sometimes an associated feature of sexual masochism disorder.

Those who engage in sadomasochistic sexual behavior may experience a hyposensitivity to pain, although it is unknown whether this finding applies to those with sexual masochism disorder. Additionally, although it is often assumed that individuals with

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masochistic sexual interest have a history of childhood sexual abuse experiences, there is insufficient evidence to support this association.

Prevalence

The population prevalence of individuals whose presentations meet the full criteria for sexual masochism disorder is unknown. In Australia, it has been estimated that 2.2% of men and 1.3% of women had been involved in BDSM behavior in the past 12 months.

Development and Course

Individuals with paraphilias living in the community have reported a mean age at onset for masochism of 19.3 years, although earlier ages, including puberty and childhood, have also been reported for the onset of masochistic fantasies. Very little is known about persistence over time. With or without treatment of sexual masochism disorder, the subjective distress (e.g., guilt, shame, intense sexual frustration, loneliness) or impairment from the disorder may change over time, as may a number of factors that may potentially affect the course of the disorder, such as psychiatric morbidity, hypersexuality, and sexual impulsivity. Thus, the severity and course may vary over time. As with other sexual preferences, advancing age may be associated with decreasing sexual masochistic preferences and behavior.

Culture-Related Diagnostic Issues

It is important to distinguish self-harming behaviors that occur during collectively accepted religious and spiritual practices from sadomasochistic behavior conducted for sexual arousal. For example, collective rituals in various religions and societies include suspension from hooks, self-flagellation, self-mortification, and other painful ordeals. The role of sexual arousal or pleasure in these practices remains unknown.

Association With Suicidal Thoughts or Behavior

Association of sexual masochism disorder with suicidal thoughts or behavior is unknown. However, a study of 321 adults who endorsed BDSM involvement found an association of stigma-related shame and guilt with suicidal ideation.

Functional Consequences of Sexual Masochism Disorder

The functional consequences of sexual masochism disorder are unknown. Individuals reporting sexual interest in asphyxiophilia seem to experience more sexual distress and psychological maladjustment than the general population. Individuals engaging in masochistic behavior are at risk for accidental death while practicing asphyxiophilia or other autoerotic procedures. However, the proportion of these decedents whose sexual interests and behavior fulfill diagnostic

criteria for sexual masochism is unknown.

Differential Diagnosis

Sexual masochism. Individuals with sexual masochism experience recurrent, intense sexual arousal from the act of being humiliated, beaten, bound, or otherwise made to suffer. Unless the sexual urges, fantasies, or behaviors involving being humiliated or made to suffer are accompanied by clinically significant distress or impairment in social, occupational, or other important areas of functioning, a diagnosis of sexual masochism disorder is not warranted.

Comorbidity

Known comorbidities with sexual masochism disorder are largely based on individuals in treatment. Disorders that occur comorbidly with sexual masochism disorder typically

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include other paraphilic disorders, such as transvestic fetishism. There is some indication of an association of sexual masochism disorder with borderline personality disorder (based on data from a small clinical sample of women with and without borderline personality disorder).

Sexual Sadism Disorder

Diagnostic Criteria	F65.52
A. Over a period of at least 6 months, recurrent and intense sexual arousal from the physical or psychological suffering of another person, as manifested by fantasies, urges, or behaviors. B. The individual has acted on these sexual urges with a nonconsenting person, or the sexual urges or fantasies cause clinically significant distress or impairment in social, occupational, or other important areas of functioning. <i>Specify if:</i> In a controlled environment: This specifier is primarily applicable to individuals living in institutional or other settings where opportunities to engage in sadistic sexual behaviors are restricted. In full remission: The individual has not acted on the urges with a nonconsenting person, and there has been no distress or impairment in social, occupational, or other areas of functioning, for at least 5 years while in an uncontrolled environment.	

Diagnostic Features

The diagnostic criteria for sexual sadism disorder are intended to apply both to individuals who freely admit to having such paraphilic interests and to those who deny any sexual interest in the

physical or psychological suffering of another individual despite substantial objective evidence to the contrary. Individuals who openly acknowledge intense sexual interest in the physical or psychological suffering of others are referred to as “admitting individuals.” If these individuals also report psychosocial difficulties because of their sexual attractions or preferences for the physical or psychological suffering of another individual, they may be diagnosed with sexual sadism disorder. In contrast, if admitting individuals declare no distress, exemplified by anxiety, obsessions, guilt, or shame, about these paraphilic impulses, and are not hampered by them in pursuing other goals, and their self-reported, psychiatric, or legal histories indicate that they do not act on them with nonconsenting persons, then they could be ascertained as having sadistic sexual interest but their presentation would *not* meet criteria for sexual sadism disorder.

Examples of individuals who deny any interest in the physical or psychological suffering of another individual include individuals known to have inflicted pain or suffering on multiple victims on separate occasions but who deny any urges or fantasies about such sexual behavior and who may further claim that known episodes of sexual assault were either unintentional or nonsexual. Others may admit past episodes of sexual behavior involving the infliction of pain or suffering on a nonconsenting person but do not report any significant or sustained sexual interest in the physical or psychological suffering of another person. Since these individuals deny having urges or fantasies involving sexual arousal to pain and suffering, it follows that they would also deny feeling subjectively distressed or socially impaired by such impulses. Such individuals may be diagnosed with sexual sadism disorder despite their negative self-report. Their recurrent behavior constitutes clinical support for the presence of the paraphilia of sexual sadism (by satisfying Criterion A) and simultaneously demonstrates that their paraphilically motivated behavior is causing clinically significant distress, harm, or risk of harm to others (satisfying Criterion B).

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“Recurrent” sexual sadism involving nonconsenting others may be interpreted as requiring multiple victims, each on a separate occasion; this requirement for multiple victims on separate occasions is relevant because it increases the confidence in the clinical inference that the individual is motivated by sexual sadism disorder. Fewer victims can be interpreted as satisfying this criterion, if there are multiple instances of infliction of pain and suffering to the same victim, or if there is corroborating evidence of a strong or preferential interest in pain and suffering involving multiple victims. Note that multiple victims, as suggested earlier, are a sufficient but not a necessary condition for diagnosis, as the criteria may be met if the individual acknowledges intense sadistic sexual interest.

The term *bondage-domination-sadism-masochism* (BDSM) is broadly used to refer to a wide range of behaviors that individuals with sexual masochism and/or sexual sadism (as well as other individuals with similar sexual interests) engage in, such as restraints or restriction, discipline, spanking, slapping, sensory deprivation (e.g., using blindfolds), and dominance-submission role-play involving themes such as master/enslaved person, owner/pet, or kidnapper/victim.

Associated Features

The extensive use of pornography involving the infliction of pain and suffering is sometimes an

associated feature of sexual sadism disorder.

Prevalence

The population prevalence of individuals whose presentations meet the full criteria for sexual sadism disorder is unknown and is largely based on individuals in forensic settings. Among civilly committed sexual offenders in the United States, less than 10% have sexual sadism disorder. Among individuals who have committed sexually motivated homicides, the proportion of sexually sadistic behavior is about one-third.

Individuals with sexual sadism disorder in forensic samples are almost exclusively men, but a representative sample of the population in Australia reported that 2.2% of men and 1.3% of women said that they had been involved in BDSM behavior in the previous year. In a population-based sample in Finland, the lifetime prevalence for sexually sadistic behavior was 2.7% among men and 2.3% among women.

Development and Course

Information on the development and course of sexual sadism disorder is extremely limited. Whereas sexually sadistic preferences per se are probably a lifelong characteristic, sexual sadism disorder may fluctuate according to the individual's subjective distress or his or her propensity to harm nonconsenting others. As with other sexual preferences, advancing age may be associated with decreasing sexually sadistic preferences and behavior. Regarding sexually sadistic preference, many individuals who engage in BDSM behavior became aware of their corresponding interest in their teenage years.

Culture-Related Diagnostic Issues

The legal status of sexually sadistic behavior ranges across countries and societies, suggesting the potential for variation in distress (because of variation in cultural acceptance) and functional impairment (because of legal status).

Association With Suicidal Thoughts or Behavior

Association of sexual sadism disorder with suicidal thoughts or behavior is unknown. However, a study of 321 adults who endorsed BDSM involvement found an association of stigma-related shame and guilt with suicidal ideation.

Differential Diagnosis

Sexual sadism. Individuals with sexual sadism experience recurrent, intense sexual arousal from the physical or psychological suffering of another person. Unless the sexual urges to make another person suffer physically or psychologically are acted on with a non-consenting person, or unless there is accompanying clinically significant distress or impairment in social, occupational, or other important areas of functioning, a diagnosis of sexual sadism disorder is not warranted. The majority of individuals who are active in community networks that practice sadistic and masochistic behaviors do not express any dissatisfaction with their sexual interests, and their

behavior would not meet DSM-5 criteria for sexual sadism disorder.

Infliction of physical or psychological suffering during the commission of a sex crime. Individuals who commit rape or other sexual assaults might inflict pain on their victims as a result of the act of rape, or in the course of subduing victims or restraining them to commit the sexual assault. Such instrumental infliction of pain should not be considered to be indicative of sexual sadism disorder unless there is evidence that the individual is deriving pleasure from the infliction of pain and the resulting suffering of the victim (e.g., admission of specifically being aroused by the pain, evidence of a preference for pornography involving themes of sexual sadism, excessive use of pain-inducing violence that goes beyond what might be necessary in the course of committing the sexual assault).

Conduct disorder and antisocial personality disorder. Individuals with conduct disorder and antisocial personality disorder may be physically cruel to people and force others to engage in sexual activity. Coercive or sadistic sexual behaviors that occur in the context of conduct disorder or antisocial personality disorder but that do not reflect an underlying pattern of sexual arousal from the physical or psychological suffering of another person should not be used as a basis for diagnosing sexual sadism disorder. In cases in which the diagnostic criteria are met for both sexual sadism disorder and conduct disorder/antisocial personality disorder, both disorders may be diagnosed.

Comorbidity

Known comorbidities with sexual sadism disorder are largely based on individuals (almost all men) convicted for criminal acts involving sadistic acts against nonconsenting victims. Hence, these comorbidities might not apply to all individuals who never engaged in sadistic activity with a nonconsenting victim but who qualify for a diagnosis of sexual sadism disorder based on subjective distress over their sexual interest. Disorders that are commonly comorbid with sexual sadism disorder include other paraphilic disorders. According to a population-based study in Finland, individuals who had engaged in sexually sadistic behavior had also engaged in other types of paraphilic behavior, namely (in descending order of co-occurrence) masochism (68.8%), voyeurism (33.3%), transvestic fetishism (9.2%), and exhibitionism (6.4%).

Pedophilic Disorder

Diagnostic Criteria

F65.4

- A. Over a period of at least 6 months, recurrent, intense sexually arousing fantasies, sexual urges, or behaviors involving sexual activity with a prepubescent child or children (generally age 13 years or younger).
- B. The individual has acted on these sexual urges, or the sexual urges or fantasies cause marked distress or interpersonal difficulty.

C. The individual is at least age 16 years and at least 5 years older than the child or children in Criterion A.

Note: Do not include an individual in late adolescence involved in an ongoing sexual relationship with a 12- or 13-year-old.

Specify whether:

Exclusive type (attracted only to children)

Nonexclusive type

Specify if:

Sexually attracted to males

Sexually attracted to females

Sexually attracted to both

Specify if:

Limited to incest

Diagnostic Features

The diagnostic criteria for pedophilic disorder are intended to apply both to individuals who freely disclose this paraphilia and to individuals who deny any sexual attraction to prepubertal children (generally age 13 years or younger), despite substantial objective evidence to the contrary. The age guideline of 13 or younger is approximate only, because the onset of puberty varies from person to person, and there is good evidence the average age at onset of puberty has been declining over time and differs across ethnicities and cultures. Examples of disclosing this paraphilia include candidly acknowledging an intense sexual interest in children and indicating that sexual interest in children is greater than or equal to sexual interest in physically mature persons. If individuals also complain that their sexual attractions or preferences for children are causing marked distress or psychosocial difficulties, they may be diagnosed with pedophilic disorder. However, if they report an absence of feelings of guilt, shame, or anxiety about these impulses and are not functionally limited by their paraphilic impulses (according to self-report, objective assessment, or both), and their self-reported and legally recorded histories indicate that they have never acted on their impulses, then these individuals have a pedophilic sexual interest but not pedophilic disorder. When trying to differentiate child offenders with pedophilic disorder from child offenders without pedophilic disorder, factors that suggest a diagnosis of pedophilic disorder in the offender include self-reported interest in children, use of child pornography, a history of multiple child victims, boy victims, and unrelated child victims.

Examples of individuals who deny attraction to children include individuals who are known to have sexually approached multiple children on separate occasions but who deny any urges or fantasies about sexual behavior involving children, and who may further claim that the known episodes of physical contact were all unintentional and nonsexual. Other individuals may acknowledge past episodes of sexual behavior involving children but deny any significant or sustained sexual interest in children. Because these individuals may deny experiences, impulses, or fantasies involving children, they may also deny feeling subjectively distressed. Such individuals may still be diagnosed with pedophilic disorder despite the absence of self-reported

distress, provided that there is evidence of recurrent behaviors persisting for 6 months (Criterion A) and evidence that the individual has acted on sexual urges or experienced interpersonal difficulties as a consequence of the disorder (Criterion B). Behaviors include sexual interactions with children, whether or not they involve physical contact (e.g., some pedophilic individuals expose themselves to children). Although the use of sexually explicit content depicting prepubescent children is typical of individuals with pedophilic sexual interests and thus might contribute important information relevant to the evaluation of Criterion A, such behavior in the absence of the individual's sexual interactions with children (i.e., acting on these sexual urges in person) is insufficient to conclude that Criterion B is met.

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Presence of multiple victims, as discussed above, is sufficient but not necessary for diagnosis; that is, the individual can still meet Criterion A by merely acknowledging intense or preferential sexual interest in children.

Associated Features

Individuals with pedophilic disorder may experience an emotional and cognitive affinity with children, sometimes referred to as *emotional congruence* with children. Emotional congruence with children can manifest in different ways, including preferring social interactions with children over adults, feeling like one has more in common with children than with adults, and choosing occupations or volunteer roles in order to be around children more often. Studies show that emotional congruence with children is related to both pedophilic sexual interest and the likelihood of sexually reoffending among individuals who have sexually offended.

Prevalence

The population prevalence of individuals whose presentations meet the full criteria for pedophilic disorder is unknown but is likely less than 3% among men in international studies. The population prevalence of pedophilic disorder in women is even more uncertain, but it is likely a small fraction of the prevalence in men.

Development and Course

Adult men with pedophilic disorder may indicate that they became aware of strong or preferential sexual interest in children around the time of puberty—the same time frame in which men who later prefer physically mature partners became aware of their sexual interest in women or men. Attempting to diagnose pedophilic disorder at the age at which it first manifests is problematic because of the difficulty during adolescent development in differentiating it from age-appropriate sexual interest in peers or from sexual curiosity. Hence, Criterion C requires for diagnosis a minimum age of 16 years and at least 5 years older than the child or children in Criterion A.

Pedophilia per se appears to be a lifelong condition. Pedophilic disorder, however, necessarily includes other elements that may change over time with or without treatment: subjective distress (e.g., guilt, shame, intense sexual frustration, or feelings of isolation) or psychosocial impairment, or the propensity to act out sexually with children, or both. Therefore,

the course of pedophilic disorder may fluctuate, or the intensity might increase or decrease with age.

Adults with pedophilic disorder may report an awareness of sexual interest in children that preceded engaging in sexual behavior involving children or self-identification as an individual with pedophilia. Advanced age is as likely to similarly diminish the frequency of sexual behavior involving children as it does other paraphilically motivated and nonparaphilic sexual behavior.

Risk and Prognostic Factors

Temperamental. There appears to be an interaction between pedophilia and antisocial personality traits such as callousness, impulsivity, and a willingness to take risks without adequate regard for the consequences. Men with pedophilic interest and antisocial personality traits are more likely to act out sexually with children and thus qualify for a diagnosis of pedophilic disorder. Thus, antisocial personality disorder may be considered a risk factor for pedophilic disorder in males with pedophilia.

Environmental. Adult men with pedophilia sometimes report that they were sexually abused as children. It is unclear, however, whether this correlation reflects a causal influence of childhood sexual abuse on adult pedophilia.

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Genetic and physiological. Since pedophilia is a necessary condition for pedophilic disorder, any factor that increases the probability of pedophilia also increases the risk of pedophilic disorder. There is some evidence that neurodevelopmental perturbation in utero increases the probability of development of a pedophilic interest.

Sex- and Gender-Related Diagnostic Issues

Laboratory measures of sexual interest, in terms of psychophysiological responses to sexual stimuli depicting children, which are sometimes useful in diagnosing pedophilic disorder in men, are not necessarily useful in diagnosing this disorder in women because there has been very limited research on the assessment of pedophilic sexual interest in women.

Diagnostic Markers

Psychophysiological measures of sexual interest may sometimes be useful when an individual's history suggests the possible presence of pedophilic disorder but the individual denies strong or preferential attraction to children. The most thoroughly researched and longest used of such measures is *penile plethysmography*, although the sensitivity and specificity of diagnosis may vary across sites, which frequently use different stimuli, procedures, and scoring. *Viewing time*, using photographs of nude or minimally clothed persons as visual stimuli, is also used to diagnose pedophilic disorder, especially in combination with self-report measures. U.S. clinicians, however, should be aware that possession of visual sexual stimuli depicting children, even for diagnostic purposes, may violate American law regarding possession of child pornography and leave the clinician susceptible to criminal prosecution. The option exists to use audio stimuli describing sexual interactions in penile plethysmography. Across psychophysiological methods, the diagnostic marker is relative sexual response to stimuli

depicting children compared with stimuli depicting adults, rather than absolute response to child stimuli.

Differential Diagnosis

Pedophilia. Individuals with pedophilia experience recurrent, intense, sexually arousing fantasies or sexual urges involving sexual activity with a prepubescent child or children. Unless the individual has acted on these sexual urges with a prepubescent child or unless the sexual urges or fantasies cause marked distress or interpersonal difficulty, a diagnosis of pedophilic disorder is not warranted.

Other paraphilic disorders. Sometimes individuals present with a different paraphilic disorder but are referred for an evaluation regarding possible pedophilic disorder (e.g., when an individual with a diagnosis of exhibitionistic disorder exposes himself to children as well as adults). In some cases, both diagnoses may apply, whereas in others, it may be the case that one paraphilic disorder diagnosis is sufficient. For example, an individual who exposes himself exclusively to prepubescent children may have both exhibitionistic disorder and pedophilic disorder, whereas another individual who exposes himself to victims, irrespective of the victims' age, may be considered to have only exhibitionistic disorder.

Antisocial personality disorder. Some individuals with antisocial personality disorder sexually abuse children, reflecting the fact that the presence of antisocial personality disorder increases the likelihood that an individual who is primarily attracted to mature persons will approach a child sexually, on the basis of relative access to the child. An additional diagnosis of pedophilic disorder should only be considered if there is evidence that over a period of at least 6 months, the individual has also had recurrent, intense, sexually arousing fantasies, sexual urges, or behaviors involving sexual activity with a prepubescent child.

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Substance intoxication. The disinhibiting effects of substance intoxication may also increase the likelihood that an individual who is primarily attracted to mature persons will sexually approach a child.

Obsessive-compulsive disorder. There are occasional individuals who complain about ego-dystonic thoughts and worries about possible attraction to children. Clinical interviewing usually reveals an absence of positive feelings about these thoughts, no connection between these thoughts and sexual behavior (e.g., masturbating to these thoughts), and sometimes additional ego-dystonic, intrusive sexual ideas (e.g., concerns about homosexuality).

Comorbidity

Psychiatric comorbidity of pedophilic disorder includes substance use disorders; depressive, bipolar, and anxiety disorders; antisocial personality disorder; and other paraphilic disorders. However, findings on comorbid disorders are largely among individuals convicted for sexual offenses involving children (almost all males) and may not be generalizable to other individuals with pedophilic disorder (e.g., individuals who have never approached a child sexually but who qualify for the diagnosis of pedophilic disorder on the basis of subjective distress).

Fetishistic Disorder

Diagnostic Criteria	F65.0
A. Over a period of at least 6 months, recurrent and intense sexual arousal from either the use of nonliving objects or a highly specific focus on nongenital body part(s), as manifested by fantasies, urges, or behaviors. B. The fantasies, sexual urges, or behaviors cause clinically significant distress or impairment in social, occupational, or other important areas of functioning. C. The fetish objects are not limited to articles of clothing used in cross-dressing (as in transvestic disorder) or devices specifically designed for the purpose of tactile genital stimulation (e.g., vibrator). <i>Specify:</i> Body part(s) Nonliving object(s) Other <i>Specify if:</i> In a controlled environment: This specifier is primarily applicable to individuals living in institutional or other settings where opportunities to engage in fetishistic behaviors are restricted. In full remission: There has been no distress or impairment in social, occupational, or other areas of functioning for at least 5 years while in an uncontrolled environment.	

Specifiers

Although individuals with fetishistic disorder may report intense and recurrent sexual arousal to inanimate objects or a specific body part, it is not unusual for non-mutually exclusive combinations of fetishistic sexual interests to occur. Thus, an individual may have fetishistic disorder associated with an inanimate object (e.g., female undergarments) or an exclusive focus on an intensely eroticized body part (e.g., feet, hair), or his or her fetishistic interest may meet criteria for various combinations of these specifiers (e.g., socks, shoes, and feet).

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Diagnostic Features

The paraphilic focus of fetishistic disorder involves the persistent and repetitive use of or dependence on nonliving objects or a highly specific focus on a (typically nongenital) body part as a primary element associated with sexual arousal (Criterion A). A diagnosis of fetishistic disorder must include clinically significant personal distress or impairment in social, occupational, or other important areas of functioning (Criterion B). Common fetish objects

include women's undergarments, men's or women's footwear, rubber articles, leather clothing, diapers, or other wearing apparel. Highly eroticized body parts associated with fetishistic disorder include feet, toes, and hair. It is not uncommon for sexualized fetishes to include both inanimate objects and body parts (e.g., dirty socks and feet), and for this reason the definition of fetishistic disorder now re-incorporates *partialism* (i.e., an exclusive focus on a body part) into its boundaries. Partialism, previously considered in DSM-IV-TR to be a paraphilia not otherwise specified, had historically been subsumed in fetishism prior to DSM-III.

Many individuals who self-identify as fetishist practitioners do not necessarily report clinical impairment in association with their fetish-associated behaviors. Such individuals could be considered as having a fetishistic sexual interest (i.e., a recurrent and intense sexual arousal from either the use of nonliving objects or a highly specific focus on a nongenital body part, as manifested by fantasies, urges, or behaviors), but not fetishistic disorder. A diagnosis of fetishistic disorder requires concurrent fulfillment of both the behaviors in Criterion A and the clinically significant distress or impairment in functioning noted in Criterion B.

Associated Features

Fetishistic disorder can be a multisensory experience, including holding, tasting, rubbing, inserting, or smelling the fetish object while masturbating, or preferring that a sexual partner wear or utilize a fetish object during sexual encounters. It should be noted that many individuals with fetishistic sexual interests also enjoy sexual experiences with their partner(s) without using their fetish object. However, it should also be noted that individuals with a fetishistic sexual interest often find that sexual experiences that involve their fetish object are more sexually satisfying than sexual experience without it. And for a minority of people with a fetishistic sexual interest, their fetish object is obligatory to becoming sexually aroused and/or satisfied. Some individuals may acquire extensive collections of highly desired fetish objects.

Development and Course

Usually paraphilias have an onset during puberty, but fetishistic sexual interests can develop prior to adolescence. Once established, fetishistic disorder tends to have a continuous course that fluctuates in intensity and frequency of urges or behavior.

Culture-Related Diagnostic Issues

Knowledge of and appropriate consideration for normative aspects of sexual behavior are important factors to explore to establish a clinical diagnosis of fetishistic disorder and to distinguish a clinical diagnosis from a socially acceptable sexual behavior.

Sex- and Gender-Related Diagnostic Issues

Fetishistic behaviors have been reported more in men, but also occur in women. This gender difference is smaller for fetishistic fantasy than for actual fetishistic behavior. In clinical samples, fetishistic disorder is nearly exclusively reported in men.

Functional Consequences of Fetishistic Disorder

Typical impairments associated with fetishistic disorder include sexual dysfunction during

romantic reciprocal relationships when the preferred fetish object or body part is

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unavailable during foreplay or coitus. Some individuals with fetishistic disorder may prefer solitary sexual activity associated with their fetishistic preference(s) even while involved in a meaningful reciprocal and affectionate relationship.

Differential Diagnosis

Transvestic disorder. The nearest diagnostic neighbor of fetishistic disorder is transvestic disorder. As noted in the diagnostic criteria, fetishistic disorder is not diagnosed when fetish objects are limited to articles of clothing exclusively worn during cross-dressing (as in transvestic disorder), or when the object is genitally stimulating because it has been designed for that purpose (e.g., a vibrator).

Sexual masochism disorder or other paraphilic disorders. Fetishistic disorder can co-occur with other paraphilic disorders, especially sadomasochistic behavior or interests and transvestic disorder. When an individual fantasizes about or engages in “forced cross-dressing” and is primarily sexually aroused by the domination or humiliation associated with such fantasy or repetitive activity, and experiences distress or functional impairment, the diagnosis of sexual masochism disorder should be made.

Fetishism. Use of a fetish object for sexual arousal (fetishism) without any associated distress or psychosocial role impairment or other adverse consequence would not meet criteria for fetishistic disorder, as the threshold required by Criterion B would not be met. For example, an individual whose sexual partner either shares or can successfully incorporate his interest in caressing, smelling, or licking feet or toes as an important element of foreplay would not be diagnosed with fetishistic disorder; nor would an individual who prefers, and is not distressed or impaired by, solitary sexual behavior associated with wearing rubber garments or leather boots.

Comorbidity

Fetishistic disorder may co-occur with other paraphilic disorders as well as hypersexuality. Rarely, fetishistic disorder may be associated with neurological conditions.

Transvestic Disorder

Diagnostic Criteria	F65.1
A. Over a period of at least 6 months, recurrent and intense sexual arousal from cross-dressing, as manifested by fantasies, urges, or behaviors.	
B. The fantasies, sexual urges, or behaviors cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.	
<i>Specify if:</i>	

With fetishism: If sexually aroused by fabrics, materials, or garments.

With autogynephilia: If sexually aroused by thoughts or images of self as a woman.

Specify if:

In a controlled environment: This specifier is primarily applicable to individuals living in institutional or other settings where opportunities to cross-dress are restricted.

In full remission: There has been no distress or impairment in social, occupational, or other areas of functioning for at least 5 years while in an uncontrolled environment.

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Specifiers

The presence of fetishism decreases the likelihood of gender dysphoria in men with transvestic disorder. The presence of autogynephilia increases the likelihood of gender dysphoria in men with transvestic disorder.

Diagnostic Features

The diagnosis of transvestic disorder does not apply to all individuals who dress as the opposite sex, even those who do so habitually. It applies to individuals whose cross-dressing or thoughts of cross-dressing are always or often accompanied by sexual excitement (Criterion A) and who are emotionally distressed by this pattern or for whom it impairs their social or interpersonal functioning (Criterion B). The cross-dressing may involve only one or two articles of clothing (e.g., for men, it may pertain only to women's undergarments), or it may involve dressing completely in the inner and outer garments of the other sex and (in men) may include the use of women's wigs and makeup. Sexual arousal, in its most obvious form of penile erection, may co-occur with cross-dressing in various ways. In younger men, cross-dressing often leads to masturbation, following which any women's clothing is removed. Older men often learn to avoid masturbating or doing anything to stimulate the penis so that the avoidance of ejaculation allows them to prolong their cross-dressing session. Men and women sometimes complete a cross-dressing session by having intercourse with their partners, and some have difficulty maintaining sufficient sexual arousal for sexual activity without cross-dressing (or having private fantasies of cross-dressing).

Clinical assessment of distress or impairment, like clinical assessment of transvestic sexual arousal, is usually dependent on the individual's self-report. The pattern of behavior "purging and acquisition" often signifies the presence of distress in individuals with transvestic disorder. During this behavioral pattern, an individual (usually a man) who has spent a great deal of money on women's clothes and other apparel (e.g., shoes, wigs) discards the items (i.e., purges them) in an effort to overcome urges to cross-dress, and then begins acquiring a woman's wardrobe all over again.

Associated Features

Transvestic disorder in men is often accompanied by *autogynephilia* (i.e., a man's paraphilic tendency to be sexually aroused by the thought or image of himself as a woman). Autogynephilic fantasies and behaviors may focus on the idea of exhibiting female physiological functions (e.g., lactation, menstruation), engaging in stereotypically feminine behavior (e.g., knitting), or possessing female anatomy (e.g., breasts).

Prevalence

The prevalence of transvestic disorder is unknown; however, it appears to be much more prevalent in men than in women. Fewer than 3% of Swedish men report having ever been sexually aroused by dressing in women's attire. The percentage of individuals who have cross-dressed with sexual arousal more than once or a few times in their lifetimes would be even lower.

Development and Course

In men, the first signs of transvestic disorder may begin in childhood, in the form of strong fascination with a particular item of women's attire. Prior to puberty, cross-dressing produces generalized feelings of pleasurable excitement. With the arrival of puberty, dressing in women's clothes begins to elicit penile erection and, in some cases, leads directly to first ejaculation. In many cases, cross-dressing elicits less and less sexual excitement as the individual grows older; eventually it may produce no discernible penile response at all. The desire to cross-dress, at the same time, remains the same or grows even stronger.

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Individuals who report such a diminution of sexual response typically report that the sexual excitement of cross-dressing has been replaced by feelings of comfort or well-being.

In some cases, the course of transvestic disorder is continuous, and in others it is episodic. It is not rare for men with transvestic disorder to lose interest in cross-dressing when they first fall in love with a woman and begin a relationship, but such abatement usually proves temporary. When the desire to cross-dress returns, so does the associated distress.

Some cases of transvestic disorder progress to gender dysphoria. The men in these cases, who may be indistinguishable from others with transvestic disorder in adolescence or early childhood, gradually develop desires to remain in the woman's role for longer periods and to feminize their anatomy. The development of gender dysphoria is usually accompanied by a (self-reported) reduction or elimination of sexual arousal in association with cross-dressing.

The manifestation of transvestism in penile erection and stimulation, like the manifestation of other paraphilic as well as nonparaphilic sexual interests, is most intense in adolescence and early adulthood. The severity of transvestic disorder is highest in adulthood, when the transvestic drives are most likely to conflict with performance in heterosexual intercourse and desires to marry and start a family. Middle-age and older men with a history of transvestism are less likely to present with transvestic disorder than with gender dysphoria.

Functional Consequences of Transvestic Disorder

Engaging in transvestic behaviors can interfere with, or detract from, heterosexual relationships. This can be a source of distress to men who wish to maintain conventional marriages or romantic partnerships with women.

Differential Diagnosis

Transvestism. Individuals with transvestism experience recurrent and intense sexual arousal from cross-dressing. Unless the fantasies, sexual urges, or behaviors involving cross-dressing are accompanied by clinically significant distress or impairment in social, occupational, or other important areas of functioning, a diagnosis of transvestic disorder is not warranted.

Fetishistic disorder. This disorder may resemble transvestic disorder, in particular, in men with fetishism who put on women's undergarments while masturbating with them. Distinguishing transvestic disorder depends on the individual's specific thoughts during such activity (e.g., are there any ideas of being a woman, being like a woman, or being dressed as a woman?) and on the presence of other fetishes (e.g., soft, silky fabrics, whether these are used for garments or for something else).

Gender dysphoria. Individuals with transvestic disorder do not report an incongruence between their experienced gender and their assigned gender or a desire to be of the other gender; and they typically do not have a history of childhood cross-gender behaviors, which would be present in individuals with gender dysphoria. Individuals with a presentation that meets full criteria for transvestic disorder as well as gender dysphoria should be given both diagnoses.

Comorbidity

Transvestic disorder is often found in association with other paraphilias. The most frequently co-occurring paraphilias are fetishistic sexual interests or behavior and masochistic sexual interests or behavior. One particularly dangerous form of masochistic sexual interests or behavior, *autoerotic asphyxia*, is associated with transvestic sexual interests or behavior in a substantial proportion of fatal cases.

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Other Specified Paraphilic Disorder

F65.89

This category applies to presentations in which symptoms characteristic of a paraphilic disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the paraphilic disorders diagnostic class. The other specified paraphilic disorder category is used in situations in which the clinician chooses to communicate the specific reason that the presentation does not meet the criteria for any specific paraphilic disorder. This is done by recording "other specified paraphilic disorder" followed by the specific reason (e.g., "zoophilia").

Examples of presentations that can be specified using the “other specified” designation include, but are not limited to, recurrent and intense sexual arousal involving *telephone scatologia* (obscene phone calls), *necrophilia* (corpses), *zoophilia* (animals), *coprophilia* (feces), *klismaphilia* (enemas), or *urophilia* (urine) that has been present for at least 6 months and causes marked distress or impairment in social, occupational, or other important areas of functioning. Other specified paraphilic disorder can be specified as in remission and/or as occurring in a controlled environment.

Unspecified Paraphilic Disorder

F65.9

This category applies to presentations in which symptoms characteristic of a paraphilic disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the paraphilic disorders diagnostic class. The unspecified paraphilic disorder category is used in situations in which the clinician chooses *not* to specify the reason that the criteria are not met for a specific paraphilic disorder, and includes presentations in which there is insufficient information to make a more specific diagnosis.

Other Mental Disorders and Additional Codes

This chapter provides diagnostic codes for psychiatric presentations that are mental disorders (i.e., symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning) but that do not meet diagnostic requirements for any of the mental disorders in the prior Section II chapters. These codes allow for the documentation and coding of these otherwise unclassified mental disorders. This chapter also includes an additional code, “No diagnosis or condition,” for situations in which the individual has been evaluated and it is determined that no mental disorder or condition is present.

The categories 1) other specified mental disorder due to another medical condition and 2) unspecified mental disorder due to another medical condition are for presentations for which it has been determined that the psychiatric symptoms (e.g., dissociative symptoms) are a direct physiological consequence of another medical condition but do not otherwise meet diagnostic criteria for any of the prior Section II mental disorders due to another medical condition. For the diagnosis of other specified or unspecified mental disorder due to another medical condition, it is necessary to code and list the medical condition first (e.g., B20 HIV disease), followed by the applicable code for either other specified or unspecified mental disorder due to another medical condition.

The categories 1) other specified mental disorder and 2) unspecified mental disorder are residual categories used when all of the following considerations are met: the psychiatric presentation is a mental disorder (i.e., the symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning); the presentation does not meet diagnostic criteria for any of the specific mental disorders in Section II; neither does the presentation meet the definitional requirements of any of the other specified and unspecified mental disorder categories presented in Section II; and no other mental disorder diagnosis applies.

As is the case with other specified and unspecified categories throughout DSM-5, the other specified category is used when the clinician chooses to specify the specific reason that the presentation does not meet the criteria for any of the existing categories (e.g., other specified mental disorder due to complex partial seizures, with dissociative symptoms), and the unspecified category is used when the clinician chooses not to specify the reason.

Other Specified Mental Disorder Due to Another Medical Condition

F06.8

This category applies to presentations in which symptoms characteristic of a mental

disorder due to another medical condition that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any specific mental disorder attributable to another medical

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condition. The other specified mental disorder due to another medical condition category is used in situations in which the clinician chooses to communicate the specific reason that the presentation does not meet the criteria for any specific mental disorder attributable to another medical condition. This is done by recording the name of the disorder, with the specific etiological medical condition inserted in place of “another medical condition,” followed by the specific symptomatic manifestation that does not meet the criteria for any specific mental disorder due to another medical condition. Furthermore, the diagnostic code for the specific medical condition must be listed immediately before the code for the other specified mental disorder due to another medical condition. For example, dissociative symptoms due to complex partial seizures would be coded and recorded as G40.209 complex partial seizures, F06.8 other specified mental disorder due to complex partial seizures, dissociative symptoms.

An example of a presentation that can be specified using the “other specified” designation is the following:

Dissociative symptoms: This includes symptoms occurring, for example, in the context of complex partial seizures.

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Unspecified Mental Disorder Due to Another Medical Condition

F09

This category applies to presentations in which symptoms characteristic of a mental disorder due to another medical condition that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any specific mental disorder due to another medical condition. The unspecified mental disorder due to another medical condition category is used in situations in which the clinician chooses *not* to specify the reason that the criteria are not met for a specific mental disorder due to another medical condition, and includes presentations for which there is insufficient information to make a more specific diagnosis (e.g., in emergency room settings). This is done by recording the name of the disorder, with the specific etiological medical condition inserted in place of “another medical condition.” Furthermore, the

diagnostic code for the specific medical condition must be listed immediately before the code for the unspecified mental disorder due to another medical condition. For example, dissociative symptoms due to complex partial seizures would be coded and recorded as G40.209 complex partial seizures, F09 unspecified mental disorder due to complex partial seizures.

Other Specified Mental Disorder

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This category applies to presentations in which symptoms characteristic of a mental disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any specific mental disorder. The other specified mental disorder category is used in situations in which the clinician chooses to communicate the specific reason that the presentation does not meet the criteria for any specific mental disorder. This is done by recording “other specified mental disorder” followed by the specific reason.

Unspecified Mental Disorder

F99

This category applies to presentations in which symptoms characteristic of a mental disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any mental disorder. The unspecified mental disorder category is used in situations in which the clinician chooses *not* to specify the reason that the criteria are not met for a specific mental disorder, and includes presentations for which there is insufficient information to make a more specific diagnosis (e.g., in emergency room settings).

Additional Codes

Z03.89 No Diagnosis or Condition

This code applies to situations in which the person has been evaluated and it is determined that no mental disorder or condition is present.

Medication-Induced Movement Disorders and Other Adverse Effects of Medication

Medication-induced movement disorders are included in Section II because of their frequent importance in 1) the management by medication of mental disorders or other medical conditions and 2) the differential diagnosis of mental disorders (e.g., anxiety disorder vs. medication-induced akathisia; malignant catatonia [a particularly severe and potentially life-threatening form of catatonia] vs. neuroleptic malignant syndrome; tardive dyskinesia vs. chorea). Although these movement disorders are labeled “medication induced,” it is often difficult to establish the causal relationship between medication exposure and the development of the movement disorder, especially because some of these movement disorders also occur in the absence of medication exposure. The conditions and problems listed in this chapter are not mental disorders.

The term *neuroleptic* is becoming outdated because it highlights the propensity of antipsychotic medications to cause abnormal movements, and it is being replaced with the term *antipsychotic medications and other dopamine receptor blocking agents* in many contexts. Although newer antipsychotic medications may be less likely to cause some medication-induced movement disorders, those disorders still occur. Antipsychotic medications and other dopamine receptor blocking agents include so-called conventional, “typical,” or first-generation antipsychotic agents (e.g., chlorpromazine, haloperidol, fluphenazine); “atypical” or second-generation antipsychotic agents (e.g., clozapine, risperidone, olanzapine, quetiapine); certain dopamine receptor blocking drugs used in the treatment of symptoms such as nausea and gastroparesis (e.g., prochlorperazine, promethazine, trimethobenzamide, thiethylperazine, metoclopramide); and amoxapine, which is indicated for the treatment of depression.

Medication-Induced Parkinsonism

G21.11 Antipsychotic Medication– and Other Dopamine Receptor Blocking Agent–Induced Parkinsonism

G21.19 Other Medication-Induced Parkinsonism

Medication-induced parkinsonism (MIP), the second most common cause of parkinsonism after Parkinson’s disease, is associated with significant morbidity, disability, and treatment nonadherence, particularly in individuals with psychiatric disorders. Because early recognition is important, any new case of parkinsonism should prompt a thorough medication history, which is essential for diagnosis of MIP. A temporal relationship between medication initiation and onset of parkinsonism should be evident. A host of agents that may be prescribed in individuals with psychiatric disorders may also induce parkinsonism, but MIP is most often seen upon exposure

to antipsychotic medications that block dopamine D₂ receptors. MIP occurs at higher rates with antipsychotics that have higher potency for the dopamine D₂ receptor, such as haloperidol, fluphenazine, and risperidone, but there are no

differences in the clinical features of parkinsonism between first- and second-generation antipsychotics.

Other medications that can cause MIP include calcium channel antagonists (e.g., flunarizine, cinnarizine), dopamine depleters (e.g., reserpine, tetrabenazine), antiepileptics (e.g., phenytoin, valproate, levetiracetam), antidepressants (e.g., selective serotonin reuptake inhibitors, monoamine oxidase inhibitors), lithium, chemotherapeutic drugs (e.g., cytosine arabinoside, cyclophosphamide, vincristine, doxorubicin, paclitaxel, etoposide), and immunosuppressants (e.g., cyclosporine, tacrolimus). Toxins (e.g., 1-methyl-4-phenyl-1,2,3,6-tetrahydropyridine [MPTP], organophosphate pesticides, manganese, methanol, cyanide, carbon monoxide, and carbon disulfide) may also cause MIP.

The time course for development of MIP varies. Usually, MIP develops a few weeks after starting or raising the dose of a medication known to cause parkinsonism or after reducing an antiparkinsonian medication (e.g., an anticholinergic agent) that is being used to treat or prevent medication-induced dystonia or parkinsonian symptoms. However, MIP may also develop rapidly after starting or raising the dose of a medication or have an insidious onset after many months of exposure. With antipsychotic medications or other dopamine receptor blocking agents, MIP typically develops 2–4 weeks after starting the medication and usually by 3 months. Mainly with calcium channel blockers, a second peak of symptom onset is reported after about 1 year.

Reported rates of MIP are affected by absence of standard diagnostic criteria, incorrect diagnosis or misattribution of MIP signs to Lewy body disease (e.g., Parkinson's disease), or a psychiatric condition, and overall lack of recognition, especially in milder cases. It is estimated that at least 50% of outpatients receiving long-term antipsychotic treatment with typical agents develop parkinsonian signs or symptoms at some point in their course of treatment.

There are no clinical characteristics that distinguish MIP reliably from Parkinson's disease. Because motor signs and symptoms in Parkinson's disease begin unilaterally and progress asymmetrically, the subacute onset of bilateral parkinsonism within weeks of starting an antipsychotic or other MIP-causing agent is highly suggestive for MIP. Parkinsonian signs are often symmetric in MIP, but asymmetric patterns are not uncommon and should not exclude a diagnosis of MIP. In addition, the course and presentation of parkinsonism should not be better accounted for by psychiatric phenomena, such as catatonia, negative symptoms of schizophrenia, or psychomotor retardation in a major depressive episode; other nonparkinsonian medication-induced movement disorders; another neurological or medical condition (e.g., Parkinson's disease, Wilson's disease); or antipsychotic-exacerbated Parkinson's disease.

In MIP, rigidity and bradykinesia are more often present, whereas tremor is somewhat less common and may be absent. Parkinsonian tremor, also referred to as a "pill-rolling tremor," is a steady, rhythmic oscillatory movement (3–6 cycles per second) that is apparent at rest and is typically slower than other tremors. It may be intermittent, unilateral or bilateral, or dependent on limb position (i.e., positional tremor). The tremor may involve the limbs, head, jaw, mouth, lip ("rabbit syndrome"), or tongue. As it is present at rest, the tremor can be suppressed, especially

when the individual attempts to perform a task with the tremulous limb. Individuals may describe the tremor as “shaking” and report that it may worsen with anxiety, stress, or fatigue.

Parkinsonian rigidity is experienced as an involuntary stiffness and inflexibility of the muscles of the limbs, shoulders, neck, or trunk. Rigidity is evaluated by assessing muscle tone, or the amount of resistance present when the examiner moves a limb (and stretches the muscles) passively around a joint. In lead-pipe rigidity, increased tone is constant throughout range of motion (in contrast to the clasp-knife rigidity spasticity). Cogwheel rigidity is thought to represent a tremor superimposed on rigidity. Most common in the wrists and elbows, it is experienced as a rhythmic, ratchet-like resistance (cogwheeling) when the muscles are passively moved around a joint. Individuals with parkinsonian rigidity may

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complain of generalized muscle tenderness or stiffness, tightness in their limbs, muscle or joint pain, body aching, or lack of coordination.

Bradykinesia and akinesia are observable states of decreased or absent spontaneous motor activity, respectively. There is global slowing as well as slowness in initiating and executing movements. Everyday behaviors (e.g., grooming) can be difficult to perform normally and may be reduced. Individuals may complain of listlessness, lack of spontaneity and drive, or fatigue. Parkinsonian rigidity and bradykinesia manifest as gait abnormalities, including decreased stride length, arm swing, or overall spontaneity of walking. Other signs include a hunched posture with bent-over neck and stooped shoulders, a staring facial expression, and small shuffling steps. Drooling can arise as a result of reduced pharyngeal motor activity and swallowing, but because of anticholinergic properties of these medications, it may be less common in antipsychotic-induced parkinsonism as compared with other medications that cause MIP.

MIP is associated with increased gait dysfunction, falls, and nursing home placement. As such, MIP is a serious iatrogenic movement disorder in older individuals that warrants recognition and early diagnosis. Associated behavioral symptoms may include depression and worsening of negative signs of schizophrenia. Other parkinsonian signs and symptoms include small handwriting (micrographia), reduced motor dexterity, hypophonia, decreased gag reflex, dysphagia, postural instability, reduced facial expression and blinking, and seborrhea. When parkinsonism is associated with severe decreased motor activity, medical complications of parkinsonism include contractures, bedsores, pulmonary emboli, urinary incontinence, aspiration pneumonia, weight loss, and hip fractures.

Consistent risk factors are female gender, older age, cognitive impairment, other concurrent neurological conditions, HIV infection, family history of Parkinson’s disease, and severe psychiatric disease. MIP secondary to antipsychotic use is also reported in children. The risk of MIP is reduced if individuals are taking anticholinergic medications.

Differential Diagnosis

Parkinson’s disease and Parkinson’s-plus conditions such as multiple system atrophy, progressive supranuclear palsy, and Wilson’s disease are distinguished from MIP by their other signs and symptoms that accompany parkinsonism. For example, Parkinson’s disease is suggested by evidence of three or more cardinal features of Parkinson’s disease (e.g., resting

tremor, rigidity, bradykinesia, postural instability), hyposmia, sleep disturbances such as rapid eye movement (REM) sleep behavior disorder, and urinary and other autonomic symptoms common to Parkinson's disease. These features are less likely to be present in MIP. Individuals with primary neurological causes of parkinsonism are also susceptible to worsening symptoms if treated with medications causing MIP.

Nonparkinsonian tremors tend to be finer (e.g., smaller amplitude) and faster (10 cycles per second) and worsen on intention (e.g., when reaching out to grab an object). With substance withdrawal, there is usually associated hyperreflexia and increased autonomic signs. In cerebellar disease, tremor worsens on intention and may be associated with nystagmus, ataxia, or scanning speech. Choreiform movements associated with tardive dyskinesia lack the steady rhythmicity of a parkinsonian tremor. Strokes and other central nervous system lesions can cause focal neurological signs or immobility from flaccid or spastic paralysis, which is characterized by decreased muscle strength and increased tone on passive movement that gives way with further pressure (i.e., clasp-knife rigidity). This contrasts with the lead-pipe rigidity and normal muscle strength in MIP.

Diagnostic alternatives to MIP are also suggested by a family history of an inherited neurological condition, rapidly progressive parkinsonism not accounted for by recent psychopharmacological changes, or presence of focal neurological signs (e.g., frontal release signs, cranial nerve abnormalities, a positive Babinski sign). Neuroleptic malignant syndrome involves severe akinesia and rigidity, but also characteristic physical and laboratory findings (e.g., fever, increased creatine phosphokinase).

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Psychomotor slowing, inactivity, and apathy seen in major depressive disorder can be indistinguishable from the motor slowness or akinesia of MIP, but major depressive disorder is more likely to include vegetative signs (e.g., early-morning awakening), hopelessness, and despair. Negative symptoms of schizophrenia, catatonia associated with schizophrenia, or mood disorders with catatonic features may also be difficult to distinguish from medication-induced akinesia. Rigidity may also manifest in psychotic disorders, delirium, major neurocognitive disorder, anxiety disorders, and functional neurological symptom disorder (conversion disorder). In parkinsonian rigidity, resistance to passive motion is constant through the full range of motion, whereas it is inconsistent in psychiatric disorders or other neurological conditions presenting with rigidity. In general, the constellation of associated physical signs on examination and symptoms associated with the tremor, rigidity, and bradykinesia of parkinsonism helps distinguish MIP-related rigidity and bradykinesia from other primary psychiatric causes of rigidity and decreased movement.

Neuroleptic Malignant Syndrome

G21.0 Neuroleptic Malignant Syndrome

Individuals with neuroleptic malignant syndrome have generally been exposed to a dopamine antagonist within 72 hours prior to symptom development. Hyperthermia ($>100.4^{\circ}\text{F}$ or $>38.0^{\circ}\text{C}$ on at least two occasions, measured orally), associated with profuse diaphoresis, is a

distinguishing feature of neuroleptic malignant syndrome, setting it apart from other neurological side effects of antipsychotic medications and other dopamine receptor blocking agents. Extreme elevations in temperature, reflecting a breakdown in central thermoregulation, are more likely to support the diagnosis of neuroleptic malignant syndrome. Generalized rigidity, described as “lead pipe” in its most severe form and usually unresponsive to antiparkinsonian agents, is a cardinal feature of the disorder and may be associated with other neurological symptoms (e.g., tremor, sialorrhea, akinesia, dystonia, trismus, myoclonus, dysarthria, dysphagia, rhabdomyolysis). Creatine kinase elevation of at least four times the upper limit of normal is commonly seen. Changes in mental status, characterized by delirium or altered consciousness ranging from stupor to coma, are often an early sign of neuroleptic malignant syndrome. Affected individuals may appear alert but dazed and unresponsive, consistent with catatonic stupor. Autonomic activation and instability—manifested by tachycardia (rate > 25% above baseline), diaphoresis, blood pressure elevation (systolic or diastolic $\geq$ 25% above baseline) or fluctuation ($\geq$ 20 mmHg diastolic change or $\geq$ 25 mmHg systolic change within 24 hours), urinary incontinence, and pallor—may be seen at any time but provide an early clue to the diagnosis. Tachypnea (rate > 50% above baseline) is common, and respiratory distress—resulting from metabolic acidosis, hypermetabolism, chest wall restriction, aspiration pneumonia, or pulmonary emboli—can occur and lead to sudden respiratory arrest.

Although several laboratory abnormalities are associated with neuroleptic malignant syndrome, no single abnormality is specific to the diagnosis. Individuals with neuroleptic malignant syndrome may have leukocytosis, metabolic acidosis, hypoxia, decreased serum iron concentrations, and elevations in serum muscle enzymes and catecholamines. Findings from cerebrospinal fluid analysis and neuroimaging studies are generally normal, whereas electroencephalography shows generalized slowing. Autopsy findings in fatal cases have been nonspecific and variable, depending on complications.

Evidence from database studies suggests incidence rates for neuroleptic malignant syndrome of 0.01%–0.02% among individuals treated with antipsychotics. A population-based study conducted in Hong Kong found an incidence risk of 0.11% in individuals treated with antipsychotic medication.

The temporal progression of signs and symptoms provides important clues to the diagnosis and prognosis of neuroleptic malignant syndrome. Alteration in mental status and other neurological signs typically precede systemic signs. The onset of symptoms varies from hours to days after drug initiation. Some cases develop within 24 hours after drug initiation, most within the first week, and virtually all cases within 30 days. Once the syndrome is diagnosed and oral antipsychotic drugs and other dopamine receptor blocking agents are discontinued, neuroleptic malignant syndrome is self-limited in most cases. The mean recovery time after drug discontinuation is 7–10 days, with most individuals recovering within 1 week and nearly all within 30 days. The duration may be prolonged when long-acting antipsychotic medications are implicated. There have been reports of individuals in whom residual neurological signs persisted for weeks after the acute hypermetabolic symptoms resolved. Total resolution of symptoms can be obtained in most cases of neuroleptic malignant syndrome; however, fatality rates of 10%–20% have been reported when the disorder is not recognized. Although many individuals do not

experience a recurrence of neuroleptic malignant syndrome when rechallenged with antipsychotic medication, some do, especially when antipsychotic medications are reinstituted soon after an episode.

Neuroleptic malignant syndrome is a potential risk in any individual after administration of an antipsychotic medication or other dopamine receptor blocking agent. It is not specific to any neuropsychiatric diagnosis and may occur in persons without a diagnosable mental disorder who receive dopamine antagonists. Clinical, systemic, and metabolic factors associated with a heightened risk of neuroleptic malignant syndrome include agitation, exhaustion, dehydration, and iron deficiency. A prior episode associated with antipsychotic medication and other dopamine receptor blocking agents has been described in 15%–20% of index cases, suggesting underlying vulnerability in some individuals; however, genetic findings based on neurotransmitter receptor polymorphisms have not been replicated consistently.

Nearly all antipsychotic medication and other dopamine receptor blocking agents have been associated with neuroleptic malignant syndrome, although high-potency antipsychotics pose a greater risk compared with low-potency agents and atypical antipsychotics. Partial or milder forms may be associated with newer antipsychotics, but neuroleptic malignant syndrome varies in severity even with older drugs. Dopamine receptor blocking agents used in medical settings (e.g., metoclopramide, prochlorperazine) have also been implicated. Parenteral administration routes, rapid titration rates, and higher total drug dosages have been associated with increased risk; however, neuroleptic malignant syndrome usually occurs within the therapeutic dosage range of antipsychotic medications and other dopamine receptor blocking agents.

Differential Diagnosis

Neuroleptic malignant syndrome should be distinguished from other serious neurological or medical conditions, including central nervous system infections, inflammatory or autoimmune conditions, status epilepticus, subcortical structural lesions, and systemic conditions (e.g., pheochromocytoma, thyrotoxicosis, tetanus, heat stroke).

Neuroleptic malignant syndrome also should be distinguished from similar syndromes resulting from the use of other substances or medications, such as serotonin syndrome; parkinsonian hyperthermia syndrome following abrupt discontinuation of dopamine agonists; alcohol or sedative withdrawal; malignant hyperthermia occurring during anesthesia; hyperthermia associated with misuse of stimulants and hallucinogens; and atropine poisoning from anticholinergics.

In rare instances, individuals with schizophrenia or a mood disorder may present with malignant catatonia, which may be indistinguishable from neuroleptic malignant syndrome. Some investigators consider neuroleptic malignant syndrome to be a drug-induced form of malignant catatonia.

Medication-Induced Acute Dystonia

G24.02 Medication-Induced Acute Dystonia

The essential feature of medication-induced acute dystonia is sustained abnormal muscle

contractions (increased muscle tone) and postures that develop in association with use of a medication known to cause acute dystonia. Any medication that blocks dopamine D₂-like receptors can induce an acute dystonic reaction (ADR). Most commonly, ADRs occur after exposure to antipsychotics and antiemetic and promotility agents. A variety of other medication classes are also reported to have induced ADRs, including selective serotonin reuptake inhibitors, cholinesterase inhibitors, opioids, and methylphenidate.

Dystonic reactions vary greatly in severity and location and can be focal, segmented, or generalized. They most often affect head and neck muscles, but can extend to upper and lower limbs or trunk. A common presentation is acute oro-mandibular (jaw) dystonia involving the tongue and mouth with tongue protrusion, or gaping or grimacing postures that can impair speech (dysarthria) and swallowing (dysphagia) and may evolve into frank trismus (lockjaw). Involvement of ocular muscles (oculogyric crisis) manifests as involuntary forced and sustained conjugate deviations of eyes upward, downward, or sideways that can last minutes to hours. Blepharospasm can also occur. Cervical (neck) dystonia presents as abnormal forward, backward, lateral, or twisting positions of the head and neck in relation to the body (e.g., antecollis, retrocollis, laterocollis, and torticollis). Focal limb dystonia, generally more distal than proximal, Pisa syndrome (lateral bending of the trunk with a tendency to lean to one side), and back arching that may evolve into opisthotonos (backward arching of head, neck, and spine) can also occur. Acute laryngeal dystonia is life-threatening, causing airway obstruction, and manifests as a “clutching of the throat,” stridor, dysphonia, dysphagia, dyspnea, and respiratory distress from the medication effects on vocal cords and laryngeal muscles.

At least 50% of individuals develop ADR signs or symptoms within 24–48 hours of starting or rapidly raising the dose of antipsychotic medication or other dopamine receptor blocking agent or of reducing a medication being used to treat or prevent acute extrapyramidal symptoms (e.g., anticholinergic agents). Approximately 90% of affected individuals have onset of ADRs within 5 days. The symptoms must not be better accounted for by a mental disorder (e.g., catatonia) and must not be due to a primary neurological or other medical condition, or a tardive medication-induced movement disorder.

Fear and anxiety often accompany ADRs given their intense nature, inability of the individual to control or stop the movements, and, when present, difficulty breathing, speaking, or swallowing. Some individuals experience pain or cramps in affected muscles. Individuals who are unaware of the possibility of developing a medication-induced dystonia can be especially distressed, increasing the likelihood of subsequent medication nonadherence. Thought disorder, delusions, or mannerisms in an individual with psychosis may cause the affected individual or others to mistakenly regard his or her dystonic symptoms as a feature of the psychiatric condition, which could lead to increased doses of the causative medication. The risk of developing ADRs is greatest in children and in adults younger than age 40 with psychosis, with a greater incidence in males than females in both children and adults. Other risk factors for developing ADRs include prior dystonic reactions to antipsychotic medications or other dopamine receptor blocking agents and use of high-potency typical antipsychotic medications.

Differential Diagnosis

It is important to distinguish between medication-induced ADRs and other causes of dystonia, especially in individuals being treated with antipsychotic or other dopamine receptor blocking

medications. A primary neurological or other medical condition is evident

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based on the time course and evolution of the dystonic phenomena (e.g., dystonia precedes exposure to the antipsychotic medication or progresses in the absence of change in medication) and, possibly, other evidence of focal neurological signs. Idiopathic focal or segmental dystonias usually persist for several days or weeks independent of medication. A family history of dystonia may also be present. Tardive dystonia secondary to medication exposure, including antipsychotic medication or other dopamine receptor blocking agents, does not have acute onset and may become evident when the dose of an antipsychotic medication is lowered. Other neurological conditions (e.g., epileptic seizures, viral and bacterial infections, trauma, space-occupying lesions in the peripheral or central nervous system) and endocrinopathies (e.g., hypoparathyroidism) can also produce symptoms (e.g., tetany) that resemble a medication-induced acute dystonia. Other diagnoses that mimic an acute medication-induced dystonia include anaphylaxis, tardive laryngeal dystonia, and respiratory dyskinesia. Neuroleptic malignant syndrome can produce dystonia but differs in that it is also accompanied by fever and generalized rigidity.

Catatonia associated with a mood disorder or schizophrenia can be distinguished by the temporal relationship between the symptoms and the exposure to antipsychotic treatment (e.g., dystonia preceding exposure to antipsychotic medication) and response to pharmacological intervention (e.g., no improvement after lowering of dose of the antipsychotic medication or in response to anticholinergic administration). Furthermore, individuals with medication-induced acute dystonia are generally distressed about the dystonic reaction and usually seek intervention. In contrast, individuals with the retarded type of catatonia are typically mute and withdrawn and do not express subjective distress about their condition.

Medication-Induced Acute Akathisia

G25.71 Medication-Induced Acute Akathisia

The essential features of medication-induced acute akathisia are subjective complaints of restlessness and at least one of the following observed movements: fidgety movements or swinging of the legs while seated, rocking from foot to foot or “walking on the spot” while standing, pacing to relieve the restlessness, or an inability to sit or stand still for at least several minutes. Individuals experiencing the most severe form of medication-induced acute akathisia may be unable to maintain any position for more than a few seconds. The subjective complaints include a sense of inner restlessness, most often in the legs; a compulsion to move one’s legs; distress if one is asked not to move one’s legs; and dysphoria and anxiety. The symptoms typically occur within 4 weeks of initiating or increasing the dose of a medication that can cause akathisia, which includes antipsychotic medications and other dopamine receptor blocking agents, tricyclic antidepressants, selective serotonin reuptake inhibitors, dopamine agonists, and calcium channel blockers, and can occasionally follow the reduction of medication used to treat or prevent acute extrapyramidal symptoms (e.g., anticholinergic agents). The symptoms are not better explained by a mental disorder (e.g., schizophrenia, substance withdrawal, agitation from a

major depressive or manic episode, hyperactivity in attention-deficit/hyperactivity disorder) and are not due to a neurological or other medical condition (e.g., Parkinson's disease, iron-deficiency anemia).

The subjective distress resulting from akathisia is significant and can lead to noncompliance with antipsychotic or antidepressant treatment. Akathisia may be associated with dysphoria, irritability, aggression, or suicide attempts. Worsening of psychotic symptoms or behavioral dyscontrol may lead to an increase in medication dose, which may exacerbate the problem. Akathisia can develop very rapidly after initiating or increasing the causative medication. The development of akathisia appears to be dose dependent and to be more frequently associated with particular high-potency antipsychotic medications or drugs with higher affinity for central dopamine receptors. Acute akathisia tends to persist for

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as long as the causative medication is continued, although the intensity may fluctuate over time. The reported prevalence of akathisia among individuals receiving antipsychotic medication or other dopamine receptor blocking agents has varied widely (20%–75%). Variations in reported prevalence may be attributable to a lack of consistency in the definition, antipsychotic prescribing practices, study design, and the demographics of the population being studied.

Differential Diagnosis

Medication-induced acute akathisia may be clinically indistinguishable from syndromes of restlessness due to certain neurological or other medical conditions, and to agitation presenting as part of a mental disorder (e.g., a manic episode). The akathisia of Parkinson's disease and iron-deficiency anemia is phenomenologically similar to medication-induced acute akathisia. The frequently abrupt appearance of restlessness soon after initiation or increase in medication usually distinguishes medication-induced acute akathisia.

Serotonin-specific reuptake inhibitor antidepressant medications may produce akathisia that appears to be identical in phenomenology and treatment response to akathisia induced by antipsychotic medication or other dopamine receptor blocking agents. Tardive dyskinesia also often has a component of generalized restlessness that may coexist with akathisia in an individual receiving antipsychotic medications or other dopamine blocking agents. Antipsychotic medication and other dopamine blocking agent–induced acute akathisia is differentiated from antipsychotic medication and other dopamine blocking agent–induced tardive dyskinesia by the nature of the movements and their relationship to the initiation of medication. The time course of symptomatic presentation relative to medication dose changes may aid in this distinction. An increase in antipsychotic medication will often exacerbate akathisia, whereas it often temporarily relieves the symptoms of tardive dyskinesia.

Medication-induced acute akathisia should be distinguished from symptoms that are better accounted for by a mental disorder. Individuals with depressive episodes, manic episodes, generalized anxiety disorder, schizophrenia spectrum and other psychotic disorders, attention-deficit/hyperactivity disorder, major neurocognitive disorder, delirium, substance intoxication (e.g., with cocaine), or substance withdrawal (e.g., from an opioid) may also display agitation that is difficult to distinguish from akathisia. Some of these individuals are able to differentiate

akathisia from the anxiety, restlessness, and agitation characteristic of a mental disorder by their experience of akathisia as being different from previously experienced feelings. Other evidence that restlessness or agitation may be better accounted for by a mental disorder includes the onset of agitation prior to exposure to the causative medication, absence of increasing restlessness with increasing doses of the causative medication, and absence of relief with pharmacological interventions (e.g., no improvement after decreasing the dose of the causative medication or treatment with another medication intended to treat the akathisia).

Tardive Dyskinesia

G24.01 Tardive Dyskinesia

The essential features of tardive dyskinesia are abnormal, involuntary movements of the tongue, jaw, trunk, or extremities that develop in association with the use of medications that block postsynaptic dopamine receptors, such as first- and second-generation antipsychotic medications and other medications such as metoclopramide for gastrointestinal disorders. The movements are present over a period of at least 4 weeks and may be choreiform (rapid, jerky, nonrepetitive), athetoid (slow, sinuous, continual), or semirhythmic (e.g., stereotypies) in nature; however, the movements are distinctly different from the rhythmic

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(3-6 Hz) tremors commonly seen in medication-induced parkinsonism. Signs or symptoms of tardive dyskinesia develop during exposure to the antipsychotic medication or other dopamine blocking agent, or within 4 weeks of withdrawal from an oral agent (or within 8 weeks of withdrawal from a long-acting injectable agent). There must be a history of the use of the offending agent for at least 3 months (or 1 month in individuals age 60 years or older). Although a large number of epidemiological studies have established the etiological relationship between dopamine blocking drug use and tardive dyskinesia, any dyskinesia in an individual who is receiving antipsychotic medication is not necessarily tardive dyskinesia.

Abnormal orofacial movements are the most obvious manifestations of tardive dyskinesia and have been observed in most individuals afflicted with tardive dyskinesia; however, approximately one-half can have limb involvement, and up to one-quarter can have axial dyskinesia of the neck, shoulders, or trunk. Involvement of other muscle groups (e.g., pharyngeal, diaphragm, abdominal) may occur but is uncommon, especially in the absence of dyskinesia of the orofacial region, limbs, or trunk. Limb or truncal dyskinesia without orofacial involvement may be more common in younger individuals, whereas orofacial dyskinesias are typical in older individuals.

The symptoms of tardive dyskinesia tend to be worsened by stimulants, antipsychotic medication withdrawal, and anticholinergic medications (such as benztropine, commonly used to manage medication-induced parkinsonism) and may be transiently worsened by emotional arousal, stress, and distraction during voluntary movements in unaffected parts of the body. The abnormal movements of dyskinesia are transiently reduced by relaxation and by voluntary movements in affected parts of the body. They are generally absent during sleep. Dyskinesia may be suppressed, at least temporarily, by increased doses of antipsychotic medication.

The overall prevalence of tardive dyskinesia in individuals who have received long-term antipsychotic medication treatment ranges from 20% to 30%. The overall incidence among younger individuals ranges from 3% to 5% per year. Middle-age and elderly individuals appear to develop tardive dyskinesia more often, with prevalence figures reported up to 50% and an incidence of 25%–30% after an average of 1 year's cumulative exposure to antipsychotic medication. Prevalence also varies depending on setting, with tardive dyskinesia tending to be more common among chronically institutionalized individuals. Variations in reported prevalence may be attributable to a lack of consistency in the definition of tardive dyskinesia, antipsychotic prescribing practices, study design, and the demographics of the population being studied.

There is no obvious gender difference in the susceptibility to tardive dyskinesia, although the risk may be somewhat greater in postmenopausal women. Greater cumulative amounts of antipsychotic medications and early development of acute extrapyramidal side effects (such as medication-induced parkinsonism) are two of the most consistent risk factors for tardive dyskinesia. Mood disorders (especially major depressive disorder), neurological conditions, and alcohol use disorder have also been found to be risk factors in some groups of individuals. Second-generation antipsychotics are associated with a somewhat lower incidence of tardive dyskinesia compared with first-generation antipsychotics, but the difference is not as large as once thought, especially when the dose of the first-generation antipsychotic is taken into account; the most important risk factors are age and cumulative exposure.

Onset of tardive dyskinesia may occur at any age and is almost always insidious. The signs are typically minimal to mild at onset and escape notice except by a keen observer. In many cases, tardive dyskinesia is objectively mild but, although it has been thought of as a cosmetic problem, can be associated with significant distress and social avoidance. In severe cases, it may be associated with medical complications (e.g., ulcers in cheeks and tongue; loss of teeth; macroglossia; difficulty in walking, swallowing, or breathing; muffled speech; weight loss; depression; suicidal ideation). In older individuals there is a greater

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likelihood that tardive dyskinesia may become more severe or more generalized with continued antipsychotic medication use. When antipsychotic medications are discontinued, some individuals experience symptom improvement over time; however, for others tardive dyskinesia can be enduring.

Differential Diagnosis

It is imperative to distinguish medication-induced parkinsonism from tardive dyskinesia because the treatments commonly used to manage medication-induced parkinsonism (i.e., anticholinergic medications) may worsen the abnormal motor movements associated with tardive dyskinesia. Moreover, treatments used to manage tardive dyskinesia (i.e., VMAT2 inhibitors) may worsen the symptoms of medication-induced parkinsonism.

Dyskinesia that emerges during withdrawal from an antipsychotic medication or other dopamine receptor blocking agent may remit with continued withdrawal from the medication. If the dyskinesia persists for at least 4 weeks, a diagnosis of tardive dyskinesia may be warranted. Tardive dyskinesia must be distinguished from other causes of orofacial and body dyskinesia.

These conditions include Huntington's disease, Wilson's disease, Sydenham's (rheumatic) chorea, systemic lupus erythematosus, thyrotoxicosis, heavy metal poisoning, ill-fitting dentures, dyskinesia due to other medications such as L-dopa or bromocriptine, and spontaneous dyskinesias. Factors that may be helpful in making the distinction are evidence that the symptoms preceded the exposure to the antipsychotic medication or other dopamine receptor blocking agent or that other focal neurological signs are present. It should be noted that other movement disorders may coexist with tardive dyskinesia. Because spontaneous dyskinesia can occur in more than 5% of individuals and is also more common in elderly persons, it may be difficult to prove that antipsychotic medications produced tardive dyskinesia in a given individual. Tardive dyskinesia must be distinguished from symptoms that are due to a medication-induced acute movement disorder (e.g., medication-induced parkinsonism, acute dystonia, acute akathisia). Acute dystonia and acute akathisia can develop quickly within hours to days, and medication-induced parkinsonism develops within weeks of initiating or increasing the dose of an antipsychotic medication or other dopamine receptor blocking agent (or reducing the dose of a medication used to treat the acute extrapyramidal symptoms). Tardive dyskinesia, on the other hand, generally develops after more prolonged exposure to antipsychotic medication (months to years) and can appear after the withdrawal of antipsychotic medication; the minimum exposure history required for the diagnosis of tardive dyskinesia is antipsychotic medication use for at least 3 months (or 1 month in middle-age and elderly individuals).

Tardive Dystonia Tardive Akathisia

G24.09 Tardive Dystonia

G25.71 Tardive Akathisia

This category is for tardive syndromes involving other types of movement problems, such as dystonia or akathisia, which are distinguished by their late emergence in the course of treatment and their potential persistence for months to years, even in the face of discontinuation of an antipsychotic medication or other dopamine receptor blocking agent or dosage reduction.

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Medication-Induced Postural Tremor

G25.1 Medication-Induced Postural Tremor

The essential feature of this condition is a fine tremor occurring during attempts to maintain a posture, which develops in association with the use of medication. Medications with which such a tremor may be associated include lithium, β -adrenergic medications (e.g., isoproterenol), stimulants (e.g., amphetamine), dopaminergic medications, anticonvulsant medications (e.g., valproic acid), antidepressant medications, and methylxanthines (e.g., caffeine, theophylline). The tremor is a regular, rhythmic oscillation of the limbs (most commonly hands and fingers), head, mouth, or tongue, most commonly with a frequency of between 8 and 12 cycles per second. It is most easily observed when the affected body part is held in a sustained posture (e.g., hands outstretched, mouth held open). The tremor may worsen in severity when the affected

body part is moved intentionally (kinetic or action tremor). When an individual describes a tremor that is consistent with postural tremor but the clinician does not directly observe the tremor, it may be helpful to try to re-create the situation in which the tremor occurred (e.g., drinking from a cup and saucer).

Most available information concerns lithium-induced tremor. Lithium tremor is a common, usually benign, and well-tolerated side effect of therapeutic doses. However, it may cause social embarrassment, occupational difficulties, and noncompliance in some individuals. As serum lithium levels approach toxic levels, the tremor may become coarser and be accompanied by muscle twitching, fasciculations, or ataxia. Nontoxic lithium tremor may improve spontaneously over time. A variety of factors may increase the risk of lithium tremor (e.g., increasing age, high serum lithium levels, concurrent antidepressant or antipsychotic medication or another dopamine receptor blocking agent, excessive caffeine intake, personal or family history of tremor, presence of alcohol use disorder, and associated anxiety). The frequency of complaints about tremor appears to decrease with duration of lithium treatment. Factors that may exacerbate the tremor include anxiety, stress, fatigue, hypoglycemia, thyrotoxicosis, pheochromocytoma, hypothermia, and alcohol withdrawal. Tremor can also be an early feature of serotonin syndrome.

Differential Diagnosis

Medication-induced postural tremor should be distinguished from a preexisting tremor that is not caused by the effects of a medication. Factors that help to establish that the tremor was preexisting include its temporal relationship to the initiation of medication, lack of correlation with serum levels of the medication, and persistence after the medication is discontinued. If a preexisting, nonpharmacologically induced tremor is present (e.g., essential tremor) that worsens with medication, such a tremor would not be considered to be medication-induced postural tremor. The factors described above that may contribute to the severity of a medication-induced postural tremor (e.g., anxiety, stress, fatigue, hypoglycemia, thyrotoxicosis, pheochromocytoma, hypothermia, alcohol withdrawal) may also be a cause of tremor independent of the medication.

Medication-induced postural tremor is not diagnosed if the tremor is better accounted for by medication-induced parkinsonism. A medication-induced postural tremor is usually absent at rest and intensifies when the affected part is brought into action or held in a sustained position. In contrast, the tremor related to medication-induced parkinsonism is usually lower in frequency (3–6 Hz), worse at rest, and suppressed during intentional movement and usually occurs in association with other symptoms of medication-induced parkinsonism (e.g., akinesia, rigidity).

Other Medication-Induced Movement Disorder

G25.79 Other Medication-Induced Movement Disorder

This category is for medication-induced movement disorders not captured by any of the specific disorders listed earlier. Examples include 1) presentations resembling neuroleptic malignant syndrome that are associated with medications other than antipsychotic medications and other dopamine receptor blocking agents and 2) other medication-induced tardive conditions.

Antidepressant Discontinuation Syndrome

T43.205A Initial encounter

T43.205D Subsequent encounter

T43.205S Sequelae

Discontinuation symptoms may occur following treatment with all types of antidepressants. The incidence of this syndrome depends on the dosage and half-life of the medication being taken, as well as the rate at which the medication is tapered. Short half-life medications that are abruptly discontinued (or when the dose is significantly reduced) rather than tapered gradually may pose the greatest risk. The short-acting antidepressants paroxetine and venlafaxine are the agents most commonly associated with discontinuation symptoms. Antidepressant discontinuation syndrome may occur in the context of intermittent non-adherence to treatment and therefore may be irregularly present in some individuals who have not actually stopped taking the medication. This is especially true for very short half-life medications (e.g., venlafaxine). By contrast, long half-life medications like fluoxetine seldom produce significant discontinuation effects.

Unlike withdrawal syndromes associated with opioids, alcohol, and other substances, antidepressant discontinuation syndrome has no pathognomonic symptoms. Instead, the symptoms tend to be vague and variable. Symptoms typically begin 2–4 days after the last dose of the antidepressant. For selective serotonin reuptake inhibitors, symptoms such as dizziness, tinnitus, “electric shock”-like sensations, insomnia, and acute anxiety are described. The antidepressant use before discontinuation must not have incurred hypomania or mixed state (i.e., there should be confidence that the discontinuation syndrome is not the result of fluctuations in mood stability associated with the previous treatment). For the tricyclic antidepressants, sudden discontinuation has been associated with gastrointestinal symptoms (cramping—reflecting cholinergic overactivity after stopping an anticholinergic tricyclic antidepressant) as well as rebound hypomania.

The antidepressant discontinuation syndrome is based solely on pharmacological factors and is not related to the reinforcing effects of an antidepressant. Unlike the discontinuation of substances with reinforcing effects like opioids, drug craving does not occur. Also, when a stimulant is used to augment an antidepressant, abrupt cessation may result in stimulant withdrawal symptoms (see “Stimulant Withdrawal” in the chapter “Substance-Related and Addictive Disorders”) rather than the antidepressant discontinuation syndrome described here.

The prevalence of antidepressant discontinuation syndrome is unknown but is thought to vary according to any of the following factors: the dosage before discontinuation, the half-life (i.e., occurring more commonly with short half-life medications) and receptor-binding affinity of the medication (e.g., more likely to occur with serotonin reuptake inhibitors), and possibly the individual’s genetically influenced rate of metabolism for this

medication. Therefore, discontinuation reactions occur more frequently with short half-life medications, but may also be influenced by rapid or ultrarapid metabolizer status of cytochrome enzymes that metabolize the antidepressant.

Because longitudinal studies are lacking, little is known about the clinical course of antidepressant discontinuation syndrome. Symptoms appear to abate over time with very gradual dosage reductions. Symptoms are usually short-lived, lasting no more than 2 weeks, and are seldom present more than 3 weeks after discontinuation.

Differential Diagnosis

The differential diagnosis of antidepressant discontinuation syndrome includes a relapse of the disorder for which the medication was prescribed (e.g., depression or panic disorder), somatic symptom disorder, bipolar I or bipolar II disorder with mixed features, substance use disorders, migraine, or cerebrovascular accident. Discontinuation symptoms often resemble symptoms of a persistent anxiety disorder or a return of somatic symptoms of depression for which the medication was initially given. It is important not to confuse discontinuation syndrome with a relapse of the original depressive or anxiety disorder for which the medication was being prescribed. Antidepressant discontinuation syndrome differs from substance withdrawal in that antidepressants themselves have no reinforcing or euphoric effects. Individuals typically do not escalate the dose of medications on their own, and they generally do not engage in drug-seeking behavior to obtain additional medication. Criteria for a substance use disorder are not met.

Other Adverse Effect of Medication

T50.905A Initial encounter

T50.905D Subsequent encounter

T50.905S Sequelae

This category is available for optional use by clinicians to code side effects of medication (other than movement symptoms) when these adverse effects become a main focus of clinical attention. Examples include severe hypotension, cardiac arrhythmias, and priapism.

Other Conditions That May Be a Focus of Clinical Attention

This chapter includes conditions and psychosocial or environmental problems that may be a focus of clinical attention or otherwise affect the diagnosis, course, prognosis, or treatment of an individual's mental disorder. These conditions are presented with their corresponding codes from ICD-10-CM (usually Z codes). A condition or problem in this chapter may be coded 1) if it is a reason for the current visit; 2) if it helps to explain the need for a test, procedure, or treatment; 3) if it plays a role in the initiation or exacerbation of a mental disorder; or 4) if it constitutes a problem that should be considered in the overall management plan.

The conditions and problems listed in this chapter are not mental disorders. Their inclusion in DSM-5-TR is meant to draw attention to the scope of additional issues that may be encountered in routine clinical practice and to provide a systematic listing that may be useful to clinicians in documenting these issues.

For quick reference to all codes in this section, see the DSM-5-TR Classification. Conditions and problems that may be a focus of clinical attention are listed in the subsequent text as follows:

1. **Suicidal behavior** (potentially self-injurious behavior with at least some intent to die) **and nonsuicidal self-injury** (intentional self-inflicted damage to the body in the absence of suicidal intent).
2. **Abuse and neglect** (e.g., child and adult maltreatment and neglect problems, including physical abuse, sexual abuse, neglect, and psychological abuse).
3. **Relational problems** (e.g., parent-child relational problem, sibling relational problem, relationship distress with spouse or intimate partner, disruption by separation or divorce).
4. **Educational problems** (e.g., illiteracy or low-level literacy, schooling unavailable or unattainable, failed school examinations, underachievement in school).
5. **Occupational problems** (e.g., unemployment, change of job, threat of job loss, stressful work schedule, discord with boss and workmates).
6. **Housing problems** (e.g., homelessness; inadequate housing; discord with neighbor, lodger, or landlord).
7. **Economic problems** (e.g., lack of adequate food or safe drinking water, extreme poverty, low income).
8. **Problems related to the social environment** (e.g., problem related to living alone, acculturation difficulty, social exclusion or rejection).
9. **Problems related to interaction with the legal system** (e.g., conviction in criminal proceedings, imprisonment or other incarceration, problems related to release from prison, problems related to other legal circumstances).
0. **Problems related to other psychosocial, personal, and environmental circumstances** (e.g., problems related to unwanted pregnancy, victim of crime, victim of terrorism).

1. **Problems related to access to medical and other health care** (e.g., unavailability or inaccessibility of health care facilities).
2. **Circumstances of personal history** (e.g., personal history of psychological trauma, military deployment).

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3. **Other health service encounters for counseling and medical advice** (e.g., sex counseling, other counseling or consultation).
4. **Additional conditions or problems that may be a focus of clinical attention** (e.g., wandering associated with a mental disorder, uncomplicated bereavement, phase of life problem).

Suicidal Behavior and Nonsuicidal Self-Injury

Coding Note for ICD-10-CM Suicidal Behavior

For T codes only, the 6th character should be coded as follows:

A (initial encounter)—Use while the individual is receiving active treatment for the condition (e.g., emergency department encounter, evaluation and treatment by a new clinician); or

D (subsequent encounter)—Use for encounters after the individual has received active treatment for the condition and when he or she is receiving routine care for the condition during the healing or recovery phase (e.g., medication adjustment, other aftercare and follow-up visits).

Suicidal Behavior

This category may be used for individuals who have engaged in potentially self-injurious behavior with at least some intent to die as a result of the act. Evidence of intent to end one's life can be explicit or inferred from the behavior or circumstances. A suicide attempt may or may not result in actual self-injury. If the individual is dissuaded by another person or changes his or her mind before initiating the behavior, this category does not apply.

Current Suicidal Behavior

T14.91A Initial encounter: If suicidal behavior is part of the initial encounter with the clinical presentation

T14.91D Subsequent encounter: If suicidal behavior is part of subsequent encounters with the clinical presentation

Z91.51 History of Suicidal Behavior

If suicidal behavior has occurred during the individual's lifetime

Nonsuicidal Self-Injury

This category may be used for individuals who have engaged in intentional self-inflicted damage

to their body of a sort likely to induce bleeding, bruising, or pain (e.g., cutting, burning, stabbing, hitting, excessive rubbing) in the absence of suicidal intent.

R45.88 Current Nonsuicidal Self-Injury

If nonsuicidal self-injurious behavior is part of the clinical presentation

Z91.52 History of Nonsuicidal Self-Injury

If nonsuicidal self-injurious behavior has occurred during the individual's lifetime

Abuse and Neglect

Maltreatment by a family member (e.g., caregiver, intimate adult partner) or by a nonrelative can be the area of current clinical focus, or such maltreatment can be an important factor in the assessment and treatment of individuals with mental disorders or other medical conditions. Because of the legal implications of abuse and neglect, care should be used in assessing these conditions and assigning these codes. Having a past history of abuse or

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neglect can influence diagnosis and treatment response in a number of mental disorders, and may also be noted along with the diagnosis.

For the following categories, in addition to listings of the confirmed or suspected event of abuse or neglect, other codes are provided for use if the current clinical encounter is to provide mental health services to either the victim or the perpetrator of the abuse or neglect. A separate code is also provided for designating a past history of abuse or neglect.

Coding Note for ICD-10-CM Abuse and Neglect Conditions

For T codes only, the 7th character should be coded as follows:

A (initial encounter)—Use while the individual is receiving active treatment for the condition (e.g., surgical treatment, emergency department encounter, evaluation and treatment by a new clinician); or

D (subsequent encounter)—Use for encounters after the individual has received active treatment for the condition and when he or she is receiving routine care for the condition during the healing or recovery phase (e.g., cast change or removal, removal of external or internal fixation device, medication adjustment, other aftercare and follow-up visits).

Child Maltreatment and Neglect Problems

Child Physical Abuse

This category may be used when physical abuse of a child is a focus of clinical attention. Child physical abuse is nonaccidental physical injury to a child—ranging from minor bruises to severe fractures or death—occurring as a result of punching, beating, kicking, biting, shaking, throwing, stabbing, choking, hitting (with a hand, stick, strap, or other object), burning, or any other method that is inflicted by a parent, caregiver, or other individual who has responsibility for the child. Such injury is considered abuse regardless of whether the caregiver intended to hurt the child. Physical discipline, such as spanking or paddling, is not considered abuse as long as it is

reasonable and causes no bodily injury to the child.

Child Physical Abuse, Confirmed

T74.12XA Initial encounter

T74.12XD Subsequent encounter

Child Physical Abuse, Suspected

T76.12XA Initial encounter

T76.12XD Subsequent encounter

Other Circumstances Related to Child Physical Abuse

Z69.010 Encounter for mental health services for victim of child physical abuse by parent

Z69.020 Encounter for mental health services for victim of nonparental child physical abuse

Z62.810 Personal history (past history) of physical abuse in childhood

Z69.011 Encounter for mental health services for perpetrator of parental child physical abuse

Z69.021 Encounter for mental health services for perpetrator of nonparental child physical abuse

Child Sexual Abuse

This category may be used when sexual abuse of a child is a focus of clinical attention. Child sexual abuse encompasses any sexual act involving a child that is intended to provide sexual gratification to a parent, caregiver, or other individual who has responsibility

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for the child. Sexual abuse includes activities such as fondling a child's genitals, penetration, incest, rape, sodomy, and indecent exposure. Sexual abuse also includes noncontact exploitation of a child by a parent or caregiver—for example, forcing, tricking, enticing, threatening, or pressuring a child to participate in acts for the sexual gratification of others, without direct physical contact between child and abuser.

Child Sexual Abuse, Confirmed

T74.22XA Initial encounter

T74.22XD Subsequent encounter

Child Sexual Abuse, Suspected

T76.22XA Initial encounter

T76.22XD Subsequent encounter

Other Circumstances Related to Child Sexual Abuse

- Z69.010** Encounter for mental health services for victim of child sexual abuse by parent
- Z69.020** Encounter for mental health services for victim of nonparental child sexual abuse
- Z62.810** Personal history (past history) of sexual abuse in childhood
- Z69.011** Encounter for mental health services for perpetrator of parental child sexual abuse
- Z69.021** Encounter for mental health services for perpetrator of nonparental child sexual abuse

Child Neglect

This category may be used when child neglect is a focus of clinical attention. Child neglect is defined as any confirmed or suspected egregious act or omission by a child's parent or other caregiver that deprives the child of basic age-appropriate needs and thereby results, or has reasonable potential to result, in physical or psychological harm to the child. Child neglect encompasses abandonment; lack of appropriate supervision; failure to attend to necessary emotional or psychological needs; and failure to provide necessary education, medical care, nourishment, shelter, and/or clothing.

Child Neglect, Confirmed

- T74.02XA** Initial encounter
- T74.02XD** Subsequent encounter

Child Neglect, Suspected

- T76.02XA** Initial encounter
- T76.02XD** Subsequent encounter

Other Circumstances Related to Child Neglect

- Z69.010** Encounter for mental health services for victim of child neglect by parent
- Z69.020** Encounter for mental health services for victim of nonparental child neglect
- Z62.812** Personal history (past history) of neglect in childhood
- Z69.011** Encounter for mental health services for perpetrator of parental child neglect
- Z69.021** Encounter for mental health services for perpetrator of nonparental child neglect

Child Psychological Abuse

This category may be used when psychological abuse of a child is a focus of clinical attention. Child psychological abuse is nonaccidental verbal or symbolic acts by a child's parent or caregiver that result, or have reasonable potential to result, in significant psychological harm to the child. (Physical and sexual abusive acts are not included in this category.) Examples of psychological abuse of a child include berating, disparaging, or humiliating the child; threatening the child; harming/abandoning—or indicating that the alleged offender will harm/abandon—people or things that the child cares about; confining the child (as by tying a child's arms or legs

together or binding a child to furniture or another object, or confining a child to a small enclosed area [e.g., a closet]); egregious scapegoating of the child; coercing the child to inflict pain on himself or herself; and disciplining the child excessively (i.e., at an extremely high frequency or duration, even if not at a level of physical abuse) through physical or nonphysical means.

Child Psychological Abuse, Confirmed

T74.32XA Initial encounter

T74.32XD Subsequent encounter

Child Psychological Abuse, Suspected

T76.32XA Initial encounter

T76.32XD Subsequent encounter

Other Circumstances Related to Child Psychological Abuse

Z69.010 Encounter for mental health services for victim of child psychological abuse by parent

Z69.020 Encounter for mental health services for victim of nonparental child psychological abuse

Z62.811 Personal history (past history) of psychological abuse in childhood

Z69.011 Encounter for mental health services for perpetrator of parental child psychological abuse

Z69.021 Encounter for mental health services for perpetrator of nonparental child psychological abuse

Adult Maltreatment and Neglect Problems

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Spouse or Partner Violence, Physical

This category may be used when spouse or partner physical violence is a focus of clinical attention. Spouse or partner physical violence is nonaccidental acts of physical force that result, or have reasonable potential to result, in physical harm to an intimate partner or that evoke significant fear in the partner. Nonaccidental acts of physical force include shoving, slapping, hair pulling, pinching, restraining, shaking, throwing, biting, kicking, hitting with the fist or an object, burning, poisoning, applying force to the throat, cutting off the air supply, holding the head under water, and using a weapon. Acts for the purpose of physically protecting oneself or one's partner are excluded.

Spouse or Partner Violence, Physical, Confirmed

T74.11XA Initial encounter

T74.11XD Subsequent encounter

Spouse or Partner Violence, Physical, Suspected

T76.11XA Initial encounter

T76.11XD Subsequent encounter

Other Circumstances Related to Spouse or Partner Violence, Physical

Z69.11 Encounter for mental health services for victim of spouse or partner violence, physical

Z91.410 Personal history (past history) of spouse or partner violence, physical

Z69.12 Encounter for mental health services for perpetrator of spouse or partner violence, physical

Spouse or Partner Violence, Sexual

This category may be used when spouse or partner sexual violence is a focus of clinical attention. Spouse or partner sexual violence involves the use of physical force or psychological coercion to compel the partner to engage in a sexual act against his or her will, whether or not the act is completed. Also included in this category are sexual acts with an intimate partner who is unable to consent.

Spouse or Partner Violence, Sexual, Confirmed

T74.21XA Initial encounter

T74.21XD Subsequent encounter

Spouse or Partner Violence, Sexual, Suspected

T76.21XA Initial encounter

T76.21XD Subsequent encounter

Other Circumstances Related to Spouse or Partner Violence, Sexual

Z69.81 Encounter for mental health services for victim of spouse or partner violence, sexual

Z91.410 Personal history (past history) of spouse or partner violence, sexual

Z69.12 Encounter for mental health services for perpetrator of spouse or partner violence, sexual

Spouse or Partner Neglect

This category may be used when spouse or partner neglect is a focus of clinical attention. Spouse or partner neglect is any egregious act or omission by one partner that deprives a dependent partner of basic needs and thereby results, or has reasonable potential to result, in physical or psychological harm to the dependent partner. This category may be used in the context of relationships in which one partner is extremely dependent on the other partner for care or for assistance in navigating ordinary daily activities—for example, a partner who is incapable of self-care because of substantial physical, psychological/intellectual, or cultural limitations (e.g., inability to communicate with others and manage everyday activities as a result of living in a foreign culture).

Spouse or Partner Neglect, Confirmed**T74.01XA** Initial encounter**T74.01XD** Subsequent encounter

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Spouse or Partner Neglect, Suspected**T76.01XA** Initial encounter**T76.01XD** Subsequent encounter**Other Circumstances Related to Spouse or Partner Neglect****Z69.11** Encounter for mental health services for victim of spouse or partner neglect**Z91.412** Personal history (past history) of spouse or partner neglect**Z69.12** Encounter for mental health services for perpetrator of spouse or partner neglect**Spouse or Partner Abuse, Psychological**

This category may be used when spouse or partner psychological abuse is a focus of clinical attention. Spouse or partner psychological abuse encompasses nonaccidental verbal or symbolic acts by one partner that result, or have reasonable potential to result, in significant harm to the other partner. Acts of psychological abuse include berating or humiliating the victim; interrogating the victim; restricting the victim's ability to come and go freely; obstructing the victim's access to assistance (e.g., law enforcement; legal, protective, or medical resources); threatening the victim with physical harm or sexual assault; harming, or threatening to harm, people or things that the victim cares about; unwarranted restriction of the victim's access to or use of economic resources; isolating the victim from family, friends, or social support resources; stalking the victim; and trying to make the victim question his or her sanity ("gaslighting").

Spouse or Partner Abuse, Psychological, Confirmed**T74.31XA** Initial encounter**T74.31XD** Subsequent encounter**Spouse or Partner Abuse, Psychological, Suspected****T76.31XA** Initial encounter**T76.31XD** Subsequent encounter**Other Circumstances Related to Spouse or Partner Abuse, Psychological****Z69.11** Encounter for mental health services for victim of spouse or partner psychological abuse**Z91.411** Personal history (past history) of spouse or partner psychological abuse

Z69.12 Encounter for mental health services for perpetrator of spouse or partner psychological abuse

Adult Abuse by Nonspouse or Nonpartner

This category may be used when the abuse of an adult by another adult who is not an intimate partner is a focus of clinical attention. Such maltreatment may involve acts of physical, sexual, or emotional abuse. Examples of adult abuse include nonaccidental acts of physical force (e.g., pushing/shoving, scratching, slapping, throwing something that could hurt, punching, biting) that have resulted—or have reasonable potential to result—in physical harm or have caused significant fear; forced or coerced sexual acts; and verbal or symbolic acts with the potential to cause psychological harm (e.g., berating or humiliating the person; interrogating the person; restricting the person’s ability to come and go freely; obstructing the person’s access to assistance; threatening the person; harming or threatening to harm people or things that the person cares about; restricting the person’s

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access to or use of economic resources; isolating the person from family, friends, or social support resources; stalking the person; trying to make the person think that he or she is crazy). Acts for the purpose of physically protecting oneself or the other person are excluded.

Adult Physical Abuse by Nonspouse or Nonpartner, Confirmed

T74.11XA Initial encounter

T74.11XD Subsequent encounter

Adult Physical Abuse by Nonspouse or Nonpartner, Suspected

T76.11XA Initial encounter

T76.11XD Subsequent encounter

Adult Sexual Abuse by Nonspouse or Nonpartner, Confirmed

T74.21XA Initial encounter

T74.21XD Subsequent encounter

Adult Sexual Abuse by Nonspouse or Nonpartner, Suspected

T76.21XA Initial encounter

T76.21XD Subsequent encounter

Adult Psychological Abuse by Nonspouse or Nonpartner, Confirmed

T74.31XA Initial encounter

T74.31XD Subsequent encounter

Adult Psychological Abuse by Nonspouse or Nonpartner, Suspected

T76.31XA Initial encounter

T76.31XD Subsequent encounter

Other Circumstances Related to Adult Abuse by Nonspouse or Nonpartner

Z69.81 Encounter for mental health services for victim of nonspousal or nonpartner adult abuse

Z69.82 Encounter for mental health services for perpetrator of nonspousal or nonpartner adult abuse

Relational Problems

Key relationships, especially intimate adult partner relationships and parent/caregiver-child relationships, have a significant impact on the health of the individuals in these relationships. These relationships can be health promoting and protective, neutral, or detrimental to health outcomes. In the extreme, these close relationships can be associated with maltreatment or neglect, which has significant medical and psychological consequences for the affected individual. A relational problem may come to clinical attention either as the reason that the individual seeks health care or as a problem that affects the course, prognosis, or treatment of the individual's mental disorder or other medical condition.

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Parent-Child Relational Problem

Z62.820 Parent–Biological Child

Z62.821 Parent–Adopted Child

Z62.822 Parent–Foster Child

Z62.898 Other Caregiver–Child

For this category, the term *parent* is used to refer to one of the child's primary caregivers, who may be a biological, adoptive, or foster parent or may be another relative (such as a grandparent) who fulfills a parental role for the child. This category may be used when the main focus of clinical attention is to address the quality of the parent-child relationship or when the quality of the parent-child relationship is affecting the course, prognosis, or treatment of a mental disorder or other medical condition. Typically, the parent-child relational problem is associated with impaired functioning in behavioral, cognitive, or affective domains. Examples of behavioral problems include inadequate parental control, supervision, and involvement with the child; parental overprotection; excessive parental pressure; arguments that escalate to threats of physical violence; and avoidance without resolution of problems. Cognitive problems may include negative attributions of the other's intentions, hostility toward or scapegoating of the other, and unwarranted feelings of estrangement. Affective problems may include feelings of sadness, apathy, or anger about the other individual in the relationship. Clinicians should take into account the developmental needs of the child and the cultural context.

Z62.891 Sibling Relational Problem

This category may be used when the focus of clinical attention is a pattern of interaction among siblings that is associated with significant impairment in individual or family functioning or with development of symptoms in one or more of the siblings, or when a sibling relational problem is affecting the course, prognosis, or treatment of a sibling's mental disorder or other medical condition. This category may be used for either children or adults if the focus is on the sibling relationship. Siblings in this context include full, half-, step-, foster, and adopted siblings.

Z63.0 Relationship Distress With Spouse or Intimate Partner

This category may be used when the major focus of the clinical contact is to address the quality of the intimate (spouse or partner) relationship or when the quality of that relationship is affecting the course, prognosis, or treatment of a mental disorder or other medical condition. Partners can be of the same or different genders. Typically, the relationship distress is associated with impaired functioning in behavioral, cognitive, or affective domains. Examples of behavioral problems include conflict resolution difficulty, withdrawal, and overinvolvement. Cognitive problems can manifest as chronic negative attributions of the other's intentions or dismissals of the partner's positive behaviors. Affective problems would include chronic sadness, apathy, and/or anger about the other partner.

Problems Related to the Family Environment

Z62.29 Upbringing Away From Parents

This category may be used when the main focus of clinical attention pertains to issues regarding a child being raised away from the parents or when this separate upbringing affects the course, prognosis, or treatment of a mental disorder or other medical condition. The child could be one who is under state custody and placed in kin care or foster care. The child could also be one who is living in a nonparental relative's home, or with friends, but whose out-of-home placement is not mandated or sanctioned by the courts. Problems related to a child living in a group home or orphanage are also included. This category excludes issues related to Z59.3 Problem Related to Living in a Residential Institution.

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Z62.898 Child Affected by Parental Relationship Distress

This category may be used when the focus of clinical attention is the negative effects of parental relationship discord (e.g., high levels of conflict, distress, or disparagement) on a child in the family, including effects on the child's mental disorder or other medical condition.

Z63.5 Disruption of Family by Separation or Divorce

This category may be used when partners in an intimate adult couple are living apart because of relationship problems or are in the process of divorce.

Z63.8 High Expressed Emotion Level Within Family

Expressed emotion is a construct used as a qualitative measure of the "amount" of emotion—in particular, hostility, emotional overinvolvement, and criticism directed toward a family member who is an identified patient—displayed in the family environment. This category may be used when a family's high level of expressed emotion is the focus of clinical attention or is affecting the course, prognosis, or treatment of a family member's mental disorder or other medical

condition.

Educational Problems

These categories may be used when an academic or educational problem is the focus of clinical attention or has an impact on the individual's diagnosis, treatment, or prognosis. Problems to be considered include illiteracy or low-level literacy; lack of access to schooling owing to unavailability or unattainability; problems with academic performance (e.g., failing school examinations, receiving failing marks or grades) or underachievement (below what would be expected given the individual's intellectual capacity); discord with teachers, school staff, or other students; problems related to inadequate teaching; and any other problems related to education and/or literacy.

Z55.0 Illiteracy and Low-Level Literacy

Z55.1 Schooling Unavailable and Unattainable

Z55.2 Failed School Examinations

Z55.3 Underachievement in School

Z55.4 Educational Maladjustment and Discord With Teachers and Classmates

Z55.8 Problems Related to Inadequate Teaching

Z55.9 Other Problems Related to Education and Literacy

Occupational Problems

These categories may be used when an occupational problem is the focus of clinical attention or has an impact on the individual's treatment or prognosis. Areas to be considered include problems with employment or in the work environment, including problems related to current military deployment status; unemployment; recent change of job; threat of job loss; stressful work schedule; uncertainty about career choices; sexual harassment on the job; other discord with boss, supervisor, co-workers, or others in the work environment; uncongenial or hostile work environments; other physical or mental strain related to work; sexual harassment on the job; and any other problems related to employment and/or occupation.

Z56.82 Problem Related to Current Military Deployment Status

This category may be used when an occupational problem directly related to an individual's military deployment status is the focus of clinical attention or has an impact on the individual's diagnosis, treatment, or prognosis. Psychological reactions to deployment are not included in this category; such reactions would be better captured as an adjustment disorder or another mental disorder.

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Z56.0 Unemployment

Z56.1 Change of Job

- Z56.2 Threat of Job Loss**
- Z56.3 Stressful Work Schedule**
- Z56.4 Discord With Boss and Workmates**
- Z56.5 Uncongenial Work Environment**
- Z56.6 Other Physical and Mental Strain Related to Work**
- Z56.81 Sexual Harassment on the Job**
- Z56.9 Other Problem Related to Employment**

Housing Problems

Z59.01 Sheltered Homelessness

This category may be used when sheltered homelessness has an impact on an individual's treatment or prognosis. An individual is considered to be experiencing sheltered homelessness if the primary nighttime residence is a homeless shelter, a warming shelter, a domestic violence shelter, a motel, or in a temporary or transitional living situation.

Z59.02 Unsheltered Homelessness

This category may be used when unsheltered homelessness has an impact on an individual's treatment or prognosis. An individual is considered to be experiencing unsheltered homelessness if residing in a place not meant for human habitation, such as a public space (e.g., tunnel, transportation station, mall), a building not intended for residential use (e.g., abandoned structure, unused factory), a car, a cave, a cardboard box, or some other ad hoc housing situation.

Z59.1 Inadequate Housing

This category may be used when lack of adequate housing has an impact on an individual's treatment or prognosis. Examples of inadequate housing conditions include lack of heat (in cold temperatures) or electricity, infestation by insects or rodents, inadequate plumbing and toilet facilities, overcrowding, lack of adequate sleeping space, and excessive noise. It is important to consider cultural norms before assigning this category.

Z59.2 Discord With Neighbor, Lodger, or Landlord

This category may be used when discord with neighbors, lodgers, or a landlord is a focus of clinical attention or has an impact on the individual's treatment or prognosis.

Z59.3 Problem Related to Living in a Residential Institution

This category may be used when a problem (or problems) related to living in a residential institution is a focus of clinical attention or has an impact on the individual's treatment or prognosis. Psychological reactions to a change in living situation are not included in this category; such reactions would be better captured as an adjustment disorder.

Z59.9 Other Housing Problem

This category may be used when there is a problem related to housing circumstances other than as specified above.

Economic Problems

These categories may be used when an economic problem is the focus of clinical attention or has an impact on the individual's treatment or prognosis. Areas to be considered include lack of adequate food (food insecurity) or safe drinking water, extreme poverty, low income, insufficient social or health insurance or welfare support, or any other economic problems.

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Z59.41 Food Insecurity

Z58.6 Lack of Safe Drinking Water

Z59.5 Extreme Poverty

Z59.6 Low Income

Z59.7 Insufficient Social or Health Insurance or Welfare Support

This category may be used for individuals who meet eligibility criteria for social or welfare support but are not receiving such support, who receive support that is insufficient to address their needs, or who otherwise lack access to needed insurance or support programs. Examples include inability to qualify for welfare support because of lack of proper documentation or evidence of address, inability to obtain adequate health insurance because of age or a preexisting condition, and denial of support owing to excessively stringent income or other requirements.

Z59.9 Other Economic Problem

This category may be used when there is a problem related to economic circumstances other than as specified above.

Problems Related to the Social Environment

Z60.2 Problem Related to Living Alone

This category may be used when a problem associated with living alone is the focus of clinical attention or has an impact on the individual's treatment or prognosis. Examples of such problems include chronic feelings of loneliness, isolation, and lack of structure in carrying out activities of daily living (e.g., irregular meal and sleep schedules, inconsistent performance of home maintenance chores).

Z60.3 Acculturation Difficulty

This category may be used when difficulty in adjusting to a new culture (e.g., following migration) is the focus of clinical attention or has an impact on the individual's treatment or prognosis.

Z60.4 Social Exclusion or Rejection

This category may be used when there is an imbalance of social power such that there is recurrent social exclusion or rejection by others. Examples of social rejection include bullying, teasing, and intimidation by others; being targeted by others for verbal abuse and humiliation; and being purposefully excluded from the activities of peers, workmates, or others in one's social environment.

Z60.5 Target of (Perceived) Adverse Discrimination or Persecution

This category may be used when there is perceived or experienced discrimination against or persecution of the individual based on his or her membership (or perceived membership) in a specific category. Typically, such categories include gender or gender identity, race, ethnicity, religion, sexual orientation, country of origin, political beliefs, disability status, caste, social status, weight, and physical appearance.

Z60.9 Other Problem Related to Social Environment

This category may be used when there is a problem related to the individual's social environment other than as specified above.

Problems Related to Interaction With the Legal System

These categories may be used when a problem related to interaction with the legal system is the focus of clinical attention or has an impact on the individual's treatment or prognosis. Areas to be considered include conviction in criminal proceedings, imprisonment or other incarceration, problems related to release from prison, and problems related to other legal circumstances (e.g., civil litigation, child custody or support proceedings).

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Z65.0 Conviction in Criminal Proceedings Without Imprisonment**Z65.1 Imprisonment or Other Incarceration****Z65.2 Problems Related to Release From Prison****Z65.3 Problems Related to Other Legal Circumstances** (e.g., civil litigation, child custody or support proceedings)

Problems Related to Other Psychosocial, Personal, and Environmental Circumstances

Z72.9 Problem Related to Lifestyle

This category may be used when a lifestyle problem is a specific focus of treatment or directly affects the course, prognosis, or treatment of a mental disorder or other medical condition. Examples of lifestyle problems include lack of physical exercise, inappropriate diet, high-risk sexual behavior, and poor sleep hygiene. A problem that is attributable to a symptom of a mental disorder should not be coded unless that problem is a specific focus of treatment or directly affects the course, prognosis, or treatment of the individual. In such cases, both the mental disorder and the lifestyle problem should be coded.

Z64.0 Problems Related to Unwanted Pregnancy**Z64.1 Problems Related to Multiparity****Z64.4 Discord With Social Service Provider, Including Probation Officer, Case Manager, or Social Services Worker**

Z65.4 Victim of Crime

Z65.4 Victim of Terrorism or Torture

Z65.5 Exposure to Disaster, War, or Other Hostilities

Problems Related to Access to Medical and Other Health Care

These categories may be used when a problem related to access to medical or other health care is the focus of clinical attention or has an impact on the individual's treatment or prognosis.

Z75.3 Unavailability or Inaccessibility of Health Care Facilities

Z75.4 Unavailability or Inaccessibility of Other Helping Agencies

Circumstances of Personal History

Z91.49 Personal History of Psychological Trauma

Z91.82 Personal History of Military Deployment

Other Health Service Encounters for Counseling and Medical Advice

Z31.5 Genetic Counseling

This category may be used for individuals seeking genetic counseling to understand the risks of developing a mental disorder with a significant genetic component (e.g., bipolar disorder) for themselves and other family members, including their existing children, as well as the risks for their future children.

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Z70.9 Sex Counseling

This category may be used when the individual seeks counseling related to sex education, sexual behavior, sexual orientation, sexual attitudes (embarrassment, timidity), others' sexual behavior or orientation (e.g., spouse, partner, child), sexual enjoyment, or any other sex-related issue.

Z71.3 Dietary Counseling

This category may be used when the individual seeks counseling related to dietary issues like weight management.

Z71.9 Other Counseling or Consultation

This category may be used when counseling is provided or advice/consultation is sought for a problem that is not specified above or elsewhere in this chapter (e.g., counseling regarding drug abuse prevention in an adolescent).

Additional Conditions or Problems That May Be a Focus of

Clinical Attention

Z91.83 Wandering Associated With a Mental Disorder

This category may be used for individuals with a mental disorder whose desire to walk about leads to significant clinical management or safety concerns. For example, individuals with major neurocognitive or neurodevelopmental disorders may experience a restless urge to wander that places them at risk for falls and causes them to leave supervised settings without needed accompaniment. This category excludes individuals whose intent is to escape an unwanted housing situation (e.g., children who are running away from home, individuals who no longer wish to remain in the hospital) or those who walk or pace as a result of medication-induced akathisia.

Coding note: First code associated mental disorder (e.g., major neurocognitive disorder, autism spectrum disorder), then code Z91.83 wandering associated with [specific mental disorder].

Z63.4 Uncomplicated Bereavement

This category may be used when the focus of clinical attention is a normal reaction to the death of a loved one. As part of their reaction to such a loss, some grieving individuals present with symptoms characteristic of a major depressive episode—for example, feelings of sadness and associated symptoms such as insomnia, poor appetite, and weight loss. The bereaved individual typically regards the depressed mood as “normal,” although the individual may seek professional help for relief of associated symptoms such as insomnia or anorexia. The duration and expression of “normal” bereavement vary considerably among different cultural groups. Further guidance in distinguishing grief from a major depressive episode and from prolonged grief disorder is provided in their respective texts.

Z60.0 Phase of Life Problem

This category may be used when a problem adjusting to a life-cycle transition (a particular developmental phase) is the focus of clinical attention or has an impact on the individual’s treatment or prognosis. Examples of such transitions include entering or completing school, leaving parental control, getting married, starting a new career, becoming a parent, adjusting to an “empty nest” after children leave home, and retiring.

Z65.8 Religious or Spiritual Problem

This category may be used when the focus of clinical attention is a religious or spiritual problem. Examples include distressing experiences that involve loss or questioning of faith, problems associated with conversion to a new faith, or questioning of spiritual values that may not necessarily be related to an organized church or religious institution.

Z72.811 Adult Antisocial Behavior

This category may be used when the focus of clinical attention is adult antisocial behavior that is not attributable to a mental disorder (e.g., conduct disorder, antisocial personality disorder). Examples include the behavior of some professional thieves, racketeers, or dealers in illegal substances.

Z72.810 Child or Adolescent Antisocial Behavior

This category may be used when the focus of clinical attention is antisocial behavior in a child or adolescent that is not attributable to a mental disorder (e.g., intermittent explosive disorder, conduct disorder). Examples include isolated antisocial acts by children or adolescents (not a pattern of antisocial behavior).

Z91.19 Nonadherence to Medical Treatment

This category may be used when the focus of clinical attention is nonadherence to an important aspect of treatment for a mental disorder or another medical condition. Reasons for such nonadherence may include discomfort resulting from treatment (e.g., medication side effects), expense of treatment, personal value judgments or religious or cultural beliefs about the proposed treatment, age-related debility, and the presence of a mental disorder (e.g., schizophrenia, personality disorder). This category may be used only when the problem is sufficiently severe to warrant independent clinical attention and does not meet diagnostic criteria for psychological factors affecting other medical conditions.

E66.9 Overweight or Obesity

This category may be used when overweight or obesity is a focus of clinical attention.

Z76.5 Malingering

The essential feature of malingering is the intentional production of false or grossly exaggerated physical or psychological symptoms, motivated by external incentives such as avoiding military duty, avoiding work, obtaining financial compensation, evading criminal prosecution, or obtaining drugs. Under some circumstances, malingering may represent adaptive behavior—for example, feigning illness while a captive of the enemy during wartime. Malingering should be strongly considered if any combination of the following is noted:

1. Medicolegal context of presentation (e.g., the individual is referred by an attorney to the clinician for examination, or the individual self-refers while litigation or criminal charges are pending).
2. Marked discrepancy between the individual's claimed stress or disability and the objective findings and observations.
3. Lack of cooperation during the diagnostic evaluation and in complying with the prescribed treatment regimen.
4. The presence of antisocial personality disorder.

Malingering differs from factitious disorder in that the motivation for the symptom production in malingering is an external incentive, whereas in factitious disorder external incentives are absent. Malingering is differentiated from functional neurological symptom disorder (conversion disorder) and other somatic symptom-related mental disorders by the intentional production of symptoms and by the obvious external incentives associated with it. Definite evidence of feigning (such as clear evidence that loss of function is present during the examination but not at home) would suggest a diagnosis of factitious disorder if the individual's apparent aim is to assume the sick role, or malingering if it is to obtain an incentive, such as money.

R41.81 Age-Related Cognitive Decline

This category may be used when the focus of clinical attention is an objectively identified

decline in cognitive functioning consequent to the aging process that is within normal limits

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given the individual's age. Individuals with this condition may report problems remembering names or appointments or may experience difficulty in solving complex problems. This category should be considered only after it has been determined that the cognitive impairment is not better explained by a specific mental disorder or attributable to a neurological condition.

R41.83 Borderline Intellectual Functioning

This category may be used when an individual's borderline intellectual functioning is the focus of clinical attention or has an impact on the individual's treatment or prognosis. Differentiating borderline intellectual functioning and mild intellectual developmental disorder (intellectual disability) requires careful assessment of intellectual and adaptive functions and their discrepancies, particularly in the presence of co-occurring mental disorders that may affect patient compliance with standardized testing procedures (e.g., schizophrenia or attention-deficit/hyperactivity disorder, with severe impulsivity).

SECTION III

Emerging Measures and Models

Assessment Measures

Cross-Cutting Symptom Measures

DSM-5 Self-Rated Level 1 Cross-Cutting Symptom Measure—Adult

Parent/Guardian-Rated DSM-5 Level 1 Cross-Cutting Symptom Measure—Child
Age 6–17

Clinician-Rated Dimensions of Psychosis Symptom Severity

World Health Organization Disability Assessment Schedule 2.0 (WHODAS 2.0)

Culture and Psychiatric Diagnosis

Key Terms

Cultural Formulation

Core Cultural Formulation Interview (CFI)

Cultural Formulation Interview (CFI)—Informant Version

Cultural Concepts of Distress

Alternative DSM-5 Model for Personality Disorders

Conditions for Further Study

Attenuated Psychosis Syndrome

Depressive Episodes With Short-Duration Hypomania

Caffeine Use Disorder

Internet Gaming Disorder

Neurobehavioral Disorder Associated With Prenatal Alcohol Exposure

Suicidal Behavior Disorder

Nonsuicidal Self-Injury Disorder

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This section contains tools and techniques to enhance clinical practice, understand the cultural context of mental disorders, and facilitate further study of proposed emerging diagnoses. The inclusion of this material represents a dynamic DSM-5 that will evolve with advances in the field.

Among the tools in Section III, Assessment Measures provides a Level 1 cross-cutting self/informant-rated measure that serves as a review of systems across mental disorders. A clinician-rated symptom severity scale for schizophrenia and other psychotic disorders also is provided, as well as the World Health Organization Disability Assessment Schedule, Version 2 (WHODAS 2.0). Severity measures for symptoms identified by the Level 1 cross-cutting self/informant-rated measure are available online (www.psychiatry.org/dsm5) and may be used to explore significant responses to the Level 1 screen.

A comprehensive review of the cultural context of mental disorders and the Cultural Formulation Interview (CFI) for clinical use are provided in the chapter “Culture and Psychiatric Diagnosis.” Both clinician and informant versions of the CFI are available online (www.psychiatry.org/dsm5). The chapter also contains a glossary of examples of cultural concepts of distress.

The Alternative DSM-5 Model for Personality Disorders provides an alternative to the extant personality disorders classification in Section II. This hybrid dimensional-categorical model defines personality disorder in terms of impairments in personality functioning and pathological personality traits.

Conditions for Further Study includes proposed criteria sets and descriptive text for new conditions that are the focus of active research, such as attenuated psychosis syndrome and caffeine use disorder.

Assessment Measures

A growing body of scientific evidence favors dimensional concepts in the diagnosis of mental disorders. Limitations of a categorical approach to diagnosis include the failure to find zones of rarity between diagnoses (i.e., delineation of mental disorders from one another by natural boundaries), need for intermediate categories like schizoaffective disorder, high rates of comorbidity, need for frequent use of other or unspecified diagnoses, relative lack of utility in furthering identification of unique antecedent validators for most mental disorders, and lack of treatment specificity for the various diagnostic categories.

From both clinical and research perspectives, there is a need for a more dimensional approach that can be combined with DSM's set of categorical diagnoses to better capture the heterogeneity in the presentation of various mental and substance use disorders. Such an approach allows clinicians or others to better communicate particular variation of features that apply to presentations that meet criteria for a disorder. Such features include differential severity of individual symptoms (including symptoms that are part of the diagnostic features as well as those that are associated with the disorder) as measured by intensity, duration, and impact on functioning. This combined approach also allows clinicians or others to identify conditions that do not meet criteria for a disorder but are severe and disabling and in need of treatment.

It is expected that as the understanding of basic disease mechanisms for mental and substance use disorders based on pathophysiology, neurocircuitry, and gene-environment interactions increases, more objective measures of psychopathology will be incorporated into the diagnostic criteria sets to enhance their accuracy. Until such time, a dimensional approach depending primarily on an individual's subjective reports of symptom experiences along with the clinician's interpretation is highlighted by current psychiatric evaluation guidelines as an important step in enhancing diagnostic practice.

Cross-cutting symptom measures, modeled on general medicine's review of systems, can serve as an approach for reviewing critical psychopathological domains across age groups and diagnoses. The general medical review of systems—a list of questions arranged by organ systems—is crucial to detecting signs and symptoms of dysfunction and disease with which the individual may or may not present that can facilitate diagnosis and treatment. A similar review of various mental systems (or domains), which is the goal of the cross-cutting symptom measures, can aid in a more comprehensive mental status assessment of individuals at the initial evaluation. The review of mental systems can systematically draw attention to signs and symptoms of other domains of mental health and functioning that may be important to the individual's care. The cross-cutting measures have two levels of inquiry: Level 1 uses 1 to 3 questions for each of 13 symptom domains for adults (self-rated) and 12 domains for children (ages 6–17, parent rated) and adolescents (child rated, ages 11–17) to identify emerging signs and symptoms. Level 2 questions provide a more in-depth assessment of certain domains (e.g., depression, anxiety, mania, anger, irritability, somatic symptoms). These measures are developed to be administered

both at initial interview and at follow-up visits. Thus, use of these measures can form key aspects of measurement-based care, the process by which standardized assessment tools are

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administered and results used to track individuals' progress over time to guide a more precise plan of care. Use of these measures ultimately aims to inform measurement-based care by identifying areas of emerging symptoms and concerns as well as supporting ongoing symptom monitoring, treatment adjustment, and outcomes critical to the provision of quality care for individuals with mental and substance use disorders. As a result, these cross-cutting symptom measures have been identified as important components of psychiatric diagnostic assessment in clinical practice guidelines.

Severity measures are disorder-specific, corresponding closely to the criteria that constitute the disorder definition. They may be administered to individuals who have received a diagnosis or who have a clinically significant syndrome that falls short of meeting full criteria for a diagnosis (e.g., use of the Clinician-Rated Dimensions of Psychosis Symptom Severity in individuals whose symptoms meet criteria for schizophrenia). Some of the assessments are self-rated, while others are rated by the clinician based on observation of the individual. As with the cross-cutting symptom measures, these measures can be administered both at initial interview and over time to track the severity of the individual's disorder and response to treatment. These assessments help operationalize symptom frequency, intensity, or duration; overall symptom severity; or symptom type (e.g., depression, anxiety, sleep disturbance) for many, though not all, DSM-5 diagnoses (e.g., generalized anxiety disorder, social anxiety disorder, psychotic disorders, posttraumatic stress disorder, autism spectrum disorder, and social (pragmatic) communication disorder). Data obtained from use of these disorder-specific measures can assist with diagnosis and inform symptom monitoring and treatment planning.

The World Health Organization Disability Assessment Schedule, Version 2.0 (WHODAS 2.0) was developed by the World Health Organization to assess an individual's ability to perform activities in six areas: understanding and communicating; getting around; self-care; getting along with people; life activities (e.g., household, work/school); and participation in society. This version of the scale is self-administered and was developed for individuals with any medical condition, not just mental disorders. It corresponds to concepts contained in the WHO International Classification of Functioning, Disability and Health. This assessment can also be used over time to track changes in an individual's level of functioning. Assessment of functioning is a key aspect of psychiatric diagnostic assessment given that most DSM-5 criteria sets include a requirement that the disturbance causes clinically significant distress or impairment in functioning. Individuals with mental disorders are more likely to have severe impairment in functioning (i.e., communicating or understanding; getting along with others; carrying out daily activities at work, home, or school; participating in social activities) compared to individuals with chronic medical conditions. In addition, many individuals seek help for mental disorders because of the direct impact of their disorders on functional impairment across multiple domains and settings. Functional impairment may impact prognosis across diagnoses and, if residual functional impairment remains after symptoms subside, can lead to recurrence or relapse for conditions such as major depressive disorder and anxiety disorders.

This chapter focuses on the DSM-5 Level 1 Cross-Cutting Symptom Measure (adult self-

rated and parent/guardian versions); the Clinician-Rated Dimensions of Psychosis Symptom Severity; and the WHODAS 2.0. Clinician instructions, scoring information, and interpretation guidelines are included for each. Description of the child-rated version is not included in print given the overall similarity in items, scoring, and clinician instructions and guidelines with the parent/guardian-rated version. These measures, including the child-rated version, and additional dimensional assessments, such as those for diagnostic severity, can be found online at www.psychiatry.org/dsm5.

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Cross-Cutting Symptom Measures

Level 1 Cross-Cutting Symptom Measure

The DSM-5 Level 1 Cross-Cutting Symptom Measure is a self- or informant-rated measure that assesses domains that are important across psychiatric diagnoses. It is intended to help clinicians identify additional areas of inquiry that may have significant impact on the individual's treatment and prognosis. In addition, the measure may be used to track changes in the individual's symptom presentation over time.

The adult version of the measure consists of 23 questions that assess 13 psychiatric domains, including depression, anger, mania, anxiety, somatic symptoms, suicidal ideation, psychosis, sleep problems, memory, repetitive thoughts and behaviors, dissociation, personality functioning, and substance use (Table 1). Each domain consists of one to three questions. Each item inquires about how much (or how often) the individual has been bothered by the specific symptom during the past 2 weeks. If the individual is of impaired capacity and unable to complete the form (e.g., an individual with major neurocognitive disorder), a knowledgeable adult informant may complete this measure.

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TABLE 1 Adult DSM-5 Self-Rated Level 1 Cross-Cutting Symptom Measure: 13 domains, thresholds for further inquiry, and associated DSM-5 Level 2 measures

Domain	Domain name	Threshold to guide further inquiry	DSM-5 Level 2 Cross-Cutting Symptom Measure ^a
I.	Depression	Mild or greater	Level 2—Depression—Adult (PROMIS Emotional Distress—Short Form)
II.	Anger	Mild or greater	Level 2—Anger—Adult (PROMIS Emotional Distress—Anger—Short Form)
III.	Mania	Mild or greater	Level 2—Mania—Adult (Altman Self-Rating Mania Scale [ASRM])
IV.	Anxiety	Mild or greater	Level 2—Anxiety—Adult (PROMIS Emotional Distress—Anxiety—Short Form)
V.	Somatic symptoms	Mild or greater	Level 2—Somatic Symptom—Adult (Patient Health Questionnaire–15 [PHQ-15] Somatic Symptom Severity Scale)

VI.	Suicidal ideation	Slight or greater	None
VII.	Psychosis	Slight or greater	None
VIII.	Sleep problems	Mild or greater	Level 2—Sleep Disturbance—Adult (PROMIS Sleep Disturbance—Short Form)
IX.	Memory	Mild or greater	None
X.	Repetitive thoughts and behaviors	Mild or greater	Level 2—Repetitive Thoughts and Behaviors—Adult (Florida Obsessive-Compulsive Inventory [FOCI] Severity Scale)
XI.	Dissociation	Mild or greater	None
XII.	Personality functioning	Mild or greater	None
XIII.	Substance use	Slight or greater	Level 2—Substance Use—Adult (adapted from the NIDA-Modified ASSIST)

Note. NIDA = National Institute on Drug Abuse.

^aAvailable at www.psychiatry.org/dsm5.

The measure was found to be clinically useful and to have good reliability in the DSM-5 Field Trials that were conducted in adult clinical samples across the United States and in Canada. In the DSM-5 Field Trials, in which the individual's symptom ratings were shared with the clinician before meeting, individuals reported that the results from the measure helped facilitate communication during the clinical encounter. Similarly, clinicians in both major academic-medical research institutions as well as routine clinical practice settings found the measures clinically useful and feasible for integration into everyday clinical care as well as specialty clinical settings. In addition to results from the DSM-5 Field Trials, several studies have evaluated the psychometric properties of the adult self-rated version of the cross-cutting symptom measure in a variety of populations. For example, findings from a large study of non-treatment-seeking college students across the United States demonstrated acceptable internal consistency and internal validity.

The parent/guardian-rated version of the measure (for children ages 6–17) consists of 25 questions that assess 12 psychiatric domains, including depression, anger, irritability, mania, anxiety, somatic symptoms, inattention, suicidal ideation/attempt, psychosis, sleep disturbance, repetitive thoughts and behaviors, and substance use (Table 2). Each item asks the parent or guardian to rate how much (or how often) his or her child has been bothered by the specific psychiatric symptom during the past 2 weeks. The measure was also found to be clinically useful and to have good reliability in the DSM-5 Field Trials that were conducted in pediatric clinical samples across the United States. For children ages 11–17, along with the parent/guardian rating of the child's symptoms, the clinician may consider having the child complete the child-rated version of the measure. The child-rated version of the measure can be found online at www.psychiatry.org/dsm5.

TABLE 2 Parent/guardian-rated DSM-5 Level 1 Cross-Cutting Symptom Measure for child age 6–17: 12 domains, thresholds for further inquiry, and associated Level 2 measures

Threshold to guide	DSM-5 Level 2 Cross-Cutting Symptom
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